

AGENDA

What we will cover today



01

Where the close loses time

P. 3

02

Target operating model

P. 7

03

Phased roadmap

P. 12

04

Business case

P. 18

05

Team and next steps

P. 22

How to use this deck

NARRATIVE

This deck is a library, not a story told start to finish. Each section answers one buyer question for Northfield Health System, from why change now to what happens next. Pull only the slides that fit the buyer and the moment.

HOW IT IS ORGANIZED

01 Context

Why the buyer acts now

02 Economics

What modernization costs and returns

03 Approach

How the work gets delivered

04 Proof

Results from comparable programs

05 Next steps

Decisions and first steps

HOW TO USE IT

1. Start with the buyer and the reason to act.
2. Pick the sections that match the conversation.
3. Choose depth: one slide, or the full set.
4. Close with proof and one clear next step.

INTERNAL USE ONLY: REMOVE THIS GUIDE AND EVERY SECTION GUIDE BEFORE SHARING THE DECK EXTERNALLY.

WHAT WE HEARD

“

We do not need faster spreadsheets. We need to stop finding problems on day nine.

CORPORATE CONTROLLER • NORTHFIELD HEALTH SYSTEM



WHAT WE HEARD

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We do not need faster spreadsheets. We need to stop finding problems on day nine.

CORPORATE CONTROLLER • NORTHFIELD HEALTH SYSTEM



Four design choices shape the new close

WHY IT MATTERS

A close that runs on rules, not reminders

Close takes 12 days because work waits for people, not because it is hard. We redesign the sequence so entries are checked as they post and exceptions reach an owner on day one.

RECOMMENDED

01 Standard reconciliations

One template and one owner for every balance-sheet account, closed during the month.

02 Automated matching

Intercompany balances match overnight, so eleven entities stop trading spreadsheets.

03 Earlier review

Controllers review entries as they post, not in a single push on day nine.

Close governance

TO BE CONFIRMED



WHAT WE HEARD



**We do not need faster spreadsheets.
We need to stop finding problems on
day nine.**

CORPORATE CONTROLLER • NORTHFIELD HEALTH SYSTEM

WHY IT MATTERS

Day nine is when controllers first see most entries. By then the books are drafted, so every finding reopens work that was already signed off.

WHAT WE DID WITH IT

We moved review to the start of the close and set rules that check entries as they post.

One quote, four reasons behind it

“

We do not need faster spreadsheets. We need to stop finding problems on day nine.

CORPORATE CONTROLLER • NORTHFIELD
HEALTH SYSTEM

01 Day nine

Controllers first see most entries after the books are drafted.

02 Rework

Each late finding reopens work that was already signed off.

03 Spreadsheets

Eleven entities reconcile by email and spreadsheet.

04 Ownership

Tasks have no named owner or due time.

One quote, four reasons behind it

01 Day nine

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We do not need faster spreadsheets. We need to stop finding problems on day nine.

CORPORATE CONTROLLER • NORTHFIELD
HEALTH SYSTEM

Four shifts take the close from 12 days to 5

TODAY

Entries reviewed on day 9

Intercompany matched by email

Controllers review every entry

Close takes 12 days



WITH THE NEW MODEL

Rules check entries as they post

92% AUTO-CHECKED

Matched automatically, daily

11 ENTITIES

Controllers review exceptions

-70% REVIEW TIME

Close takes 5 days

-7 DAYS

Source: West Monroe close assessment, 14 business units, Aug 2026.

TARGET OPERATING MODEL

From a 12-day close to a 5-day close

TODAY

Entries reviewed on day 9

Intercompany matched by email

Controllers review every entry

Close takes 12 days



WITH THE NEW MODEL

Rules check entries as they post

92% AUTO-CHECKED

Matched automatically, daily

11 ENTITIES

Controllers review trends and exceptions

-70% REVIEW TIME

Close takes 5 days

-7 DAYS

Source: West Monroe analysis of the FY26 baseline.

Frameworks are emerging for AI-powered delivery

FRAMEWORK LAYER	PUBLIC EXAMPLES	WHAT THEY EMPHASIZE	MATURITY
01 Product outcomes Why we build	Outcome-based operating model; continuous discovery	Frames outcomes with customer evidence.	Mainstream
02 AI product quality Whether to trust it	Human-AI interaction guidelines; trust patterns	Trust, feedback and failure recovery.	Growing
03 Product methods How we discover	Reusable discovery skills; context as code	Repeatable methods and shared product context.	Early
04 Specs and handoff How we hand off	Spec-driven development kits; change proposals	Reviewable specs, plans, tasks and changes.	Growing
05 Delivery flow How teams ship	Cross-role planning and QA playbooks	Cross-role planning, review and QA.	Early
Workshop application	Use today's learnings to upgrade your delivery operating system		

Vendor B is the strongest fit for Northfield

●○○○ Limited

●●○○ Partial

●●●○ Strong

●●●● Leading

	VENDOR A LOGO	RECOMMENDED VENDOR B LOGO	VENDOR C LOGO
NORTHFIELD NEED	VENDOR A	VENDOR B	VENDOR C
Core workflow coverage Intake, scheduling and referrals	●●●○	●●●●	●●○○
Integration with ERP and data Finance, supply and data feeds	●●○○	●●●●	●●●○
Security and compliance Access control, audit and privacy	●●●●	●●●○	●●●●
Admin and configuration effort Setup, tuning and ongoing upkeep	●●●○	●●●○	●●○○
Delivery partner ecosystem Implementers, skills and capacity	●●○○	●●●●	●●●○

PRIORITIZATION

Quick wins first: high value, low effort

AIM HERE

Quick wins come first

Three items pay back inside wave 1.

PLAN CAREFULLY

Big bets need a sponsor

High value, but the most effort.

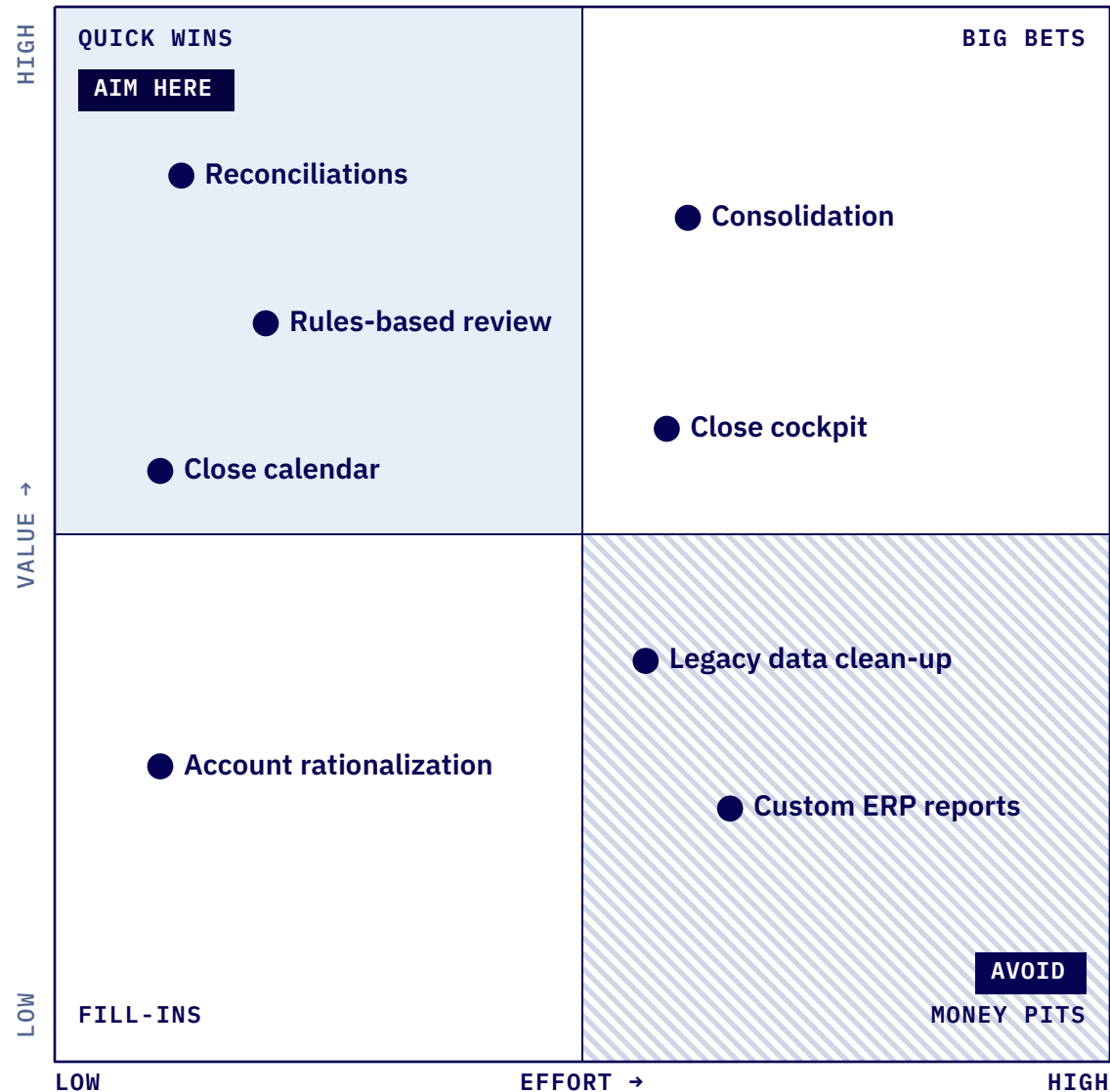
AVOID

Skip the money pits

Cost outruns return on both items.



Source: Northfield finance workshops, Sep 2026.



PRIORITIZATION

Start with quick wins, not money pits

AIM HERE

Quick wins come first

Three items pay back inside wave 1.

PLAN CAREFULLY

Big bets need a sponsor

High value, but the most effort.

AVOID

Skip the money pits

Cost outruns return on both items.

Source: Northfield finance workshops, Sep 2026.

RISK

Out of the red zone by wave 3

STAY OUT

Labs sit in the red zone

Rules replace manual review.

STAY OUT

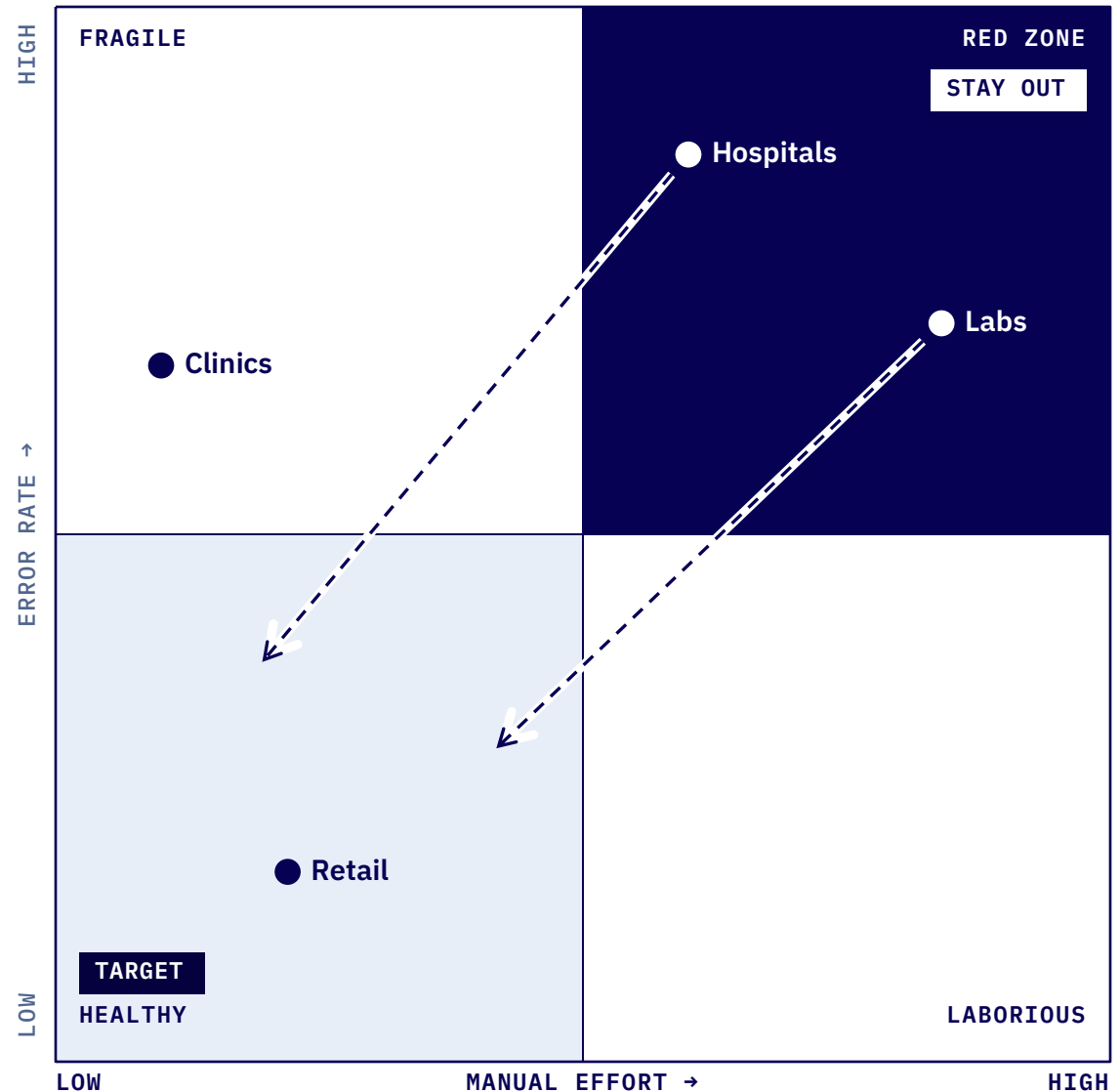
Hospitals follow next

Errors fall with earlier checks.

TARGET

Healthy by wave 3

Clinics and retail hold steady.



Source: Entity error review, 14 units, Aug 2026.

Three of five workstreams are on track

STATUS BY MONTH				
WORKSTREAM	JUL	AUG	SEP	OCT
Process design	On track	On track	On track	On track
Data migration	On track	On track	At risk	At risk
Controls	On track	On track	On track	On track
Change and training	On track	On track	On track	At risk
Close cockpit	On track	On track	On track	On track

NEXT STEPS

- Reset the Wave 2 data freeze with Finance IT
- Run the second training cohort in October
- Confirm the cutover go or no-go on 13 Nov

Source: Northfield program office status report, 30 Sep 2026.

MILESTONES

Wave 1 go-live 12 AUG

Five entities live.

Rules signed off 20 SEP

Controller approved.

Acceptance testing 30 OCT

Wave 2 entities.

Cutover decision 13 NOV

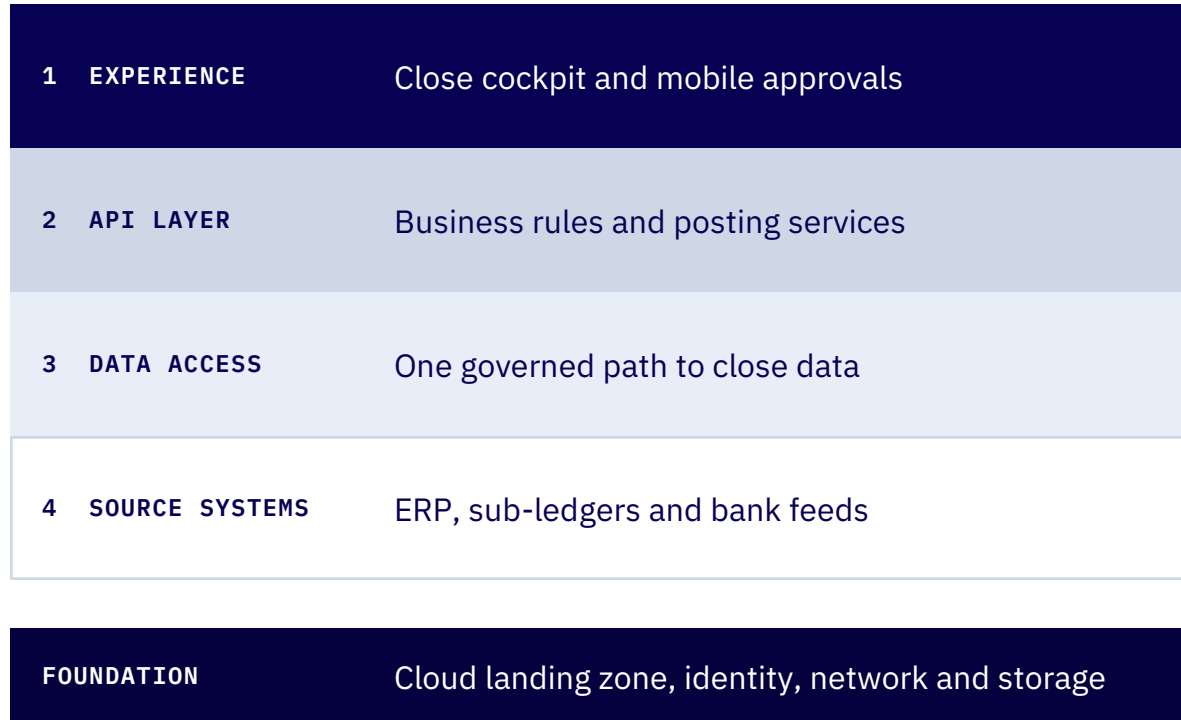
Go or no-go.

One layered platform replaces the old close tools

CLOSE PLATFORM

Build once

Each layer serves every close process, so one moves without a rebuild.



ACROSS LAYERS 1-4

Shared controls

- Security
- Governance
- Observability
- Audit trail

Source: West Monroe reference architecture.

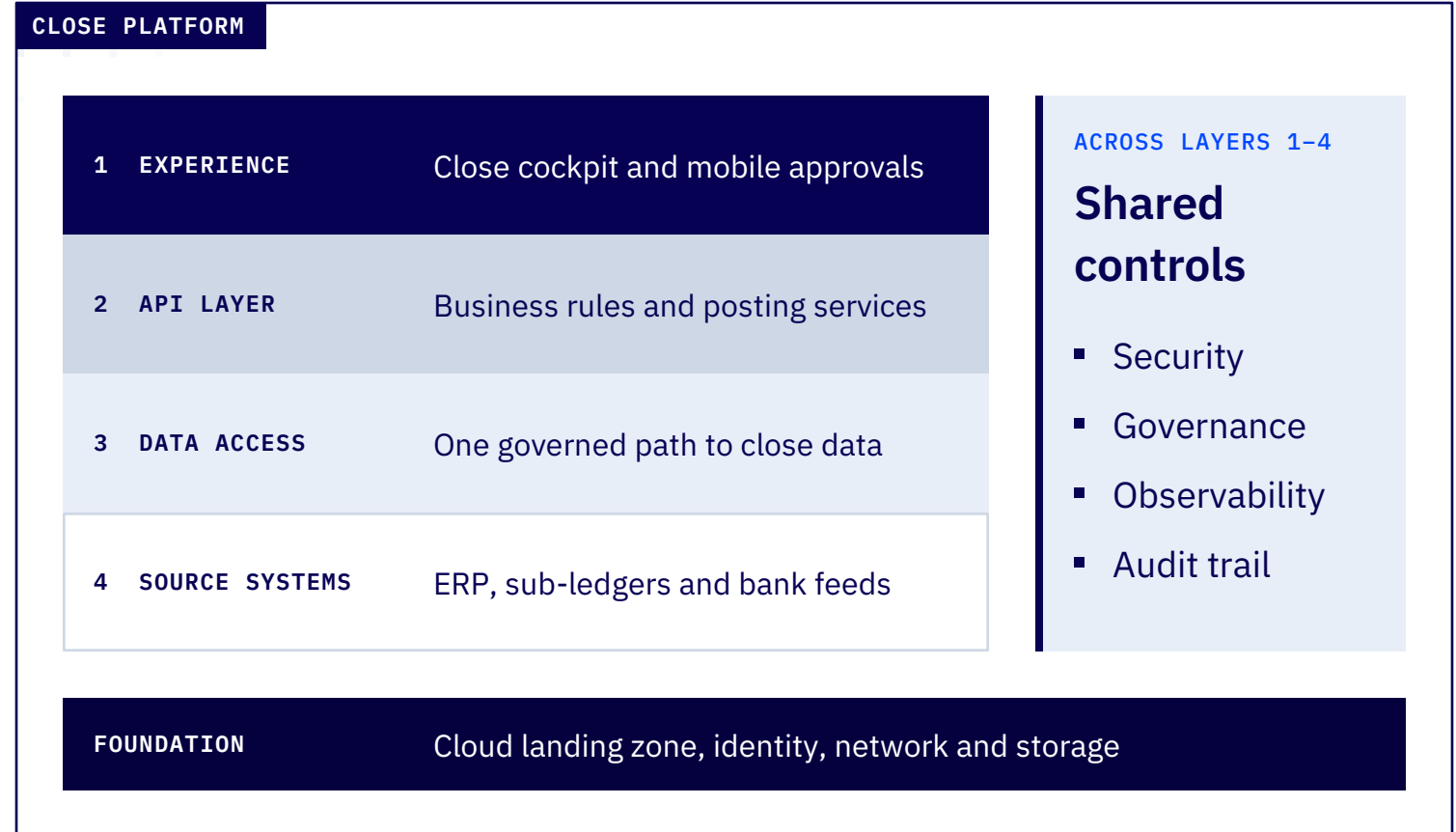
DESIGN PRINCIPLE

Build once

Each layer serves every close process, so one moves without a rebuild.

SOLUTION ARCHITECTURE

One platform replaces the old tools



Source: West Monroe reference architecture.

One layered platform replaces the old close tools

CLOSE PLATFORM

Build once

Each layer serves every close process, so one moves without a rebuild.

1	EXPERIENCE	Close cockpit and mobile approvals
2	API LAYER	Business rules and posting services
3	DATA ACCESS	One governed path to close data
4	SOURCE SYSTEMS	ERP, sub-ledgers and bank feeds

FOUNDATION Cloud landing zone, identity, network and storage

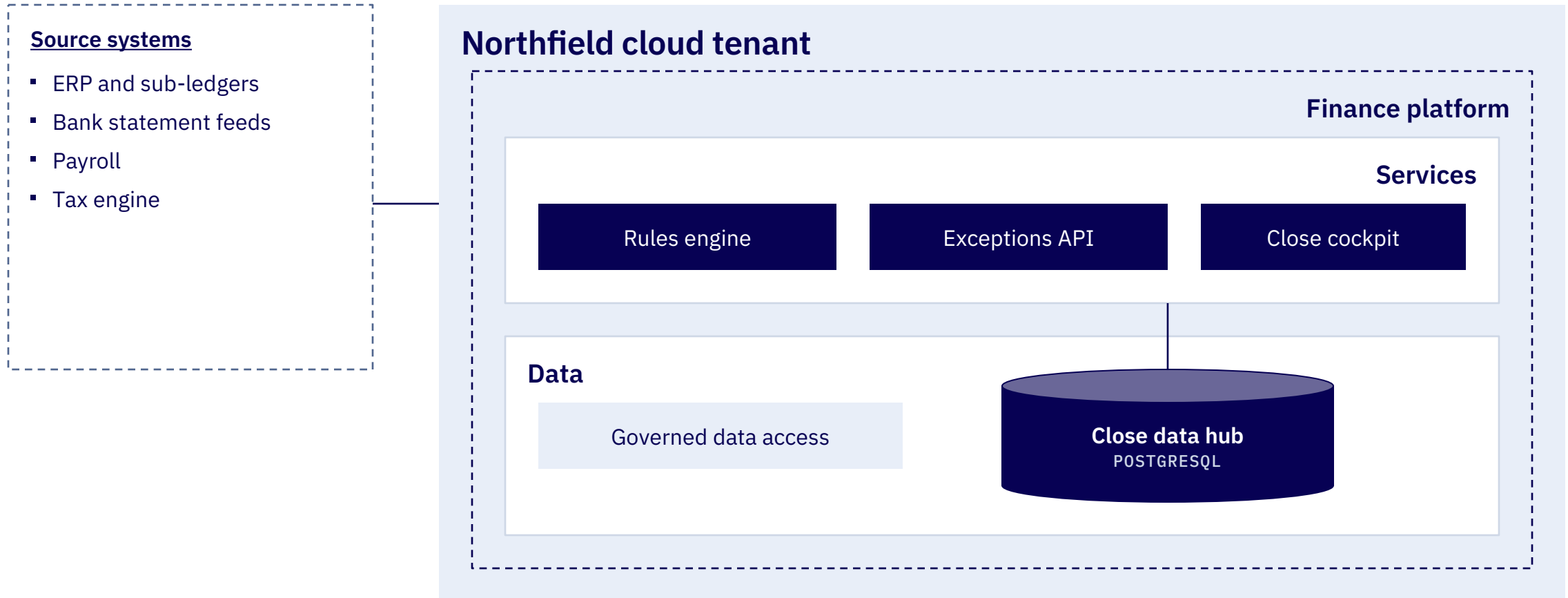
ACROSS LAYERS 1-4

Shared controls

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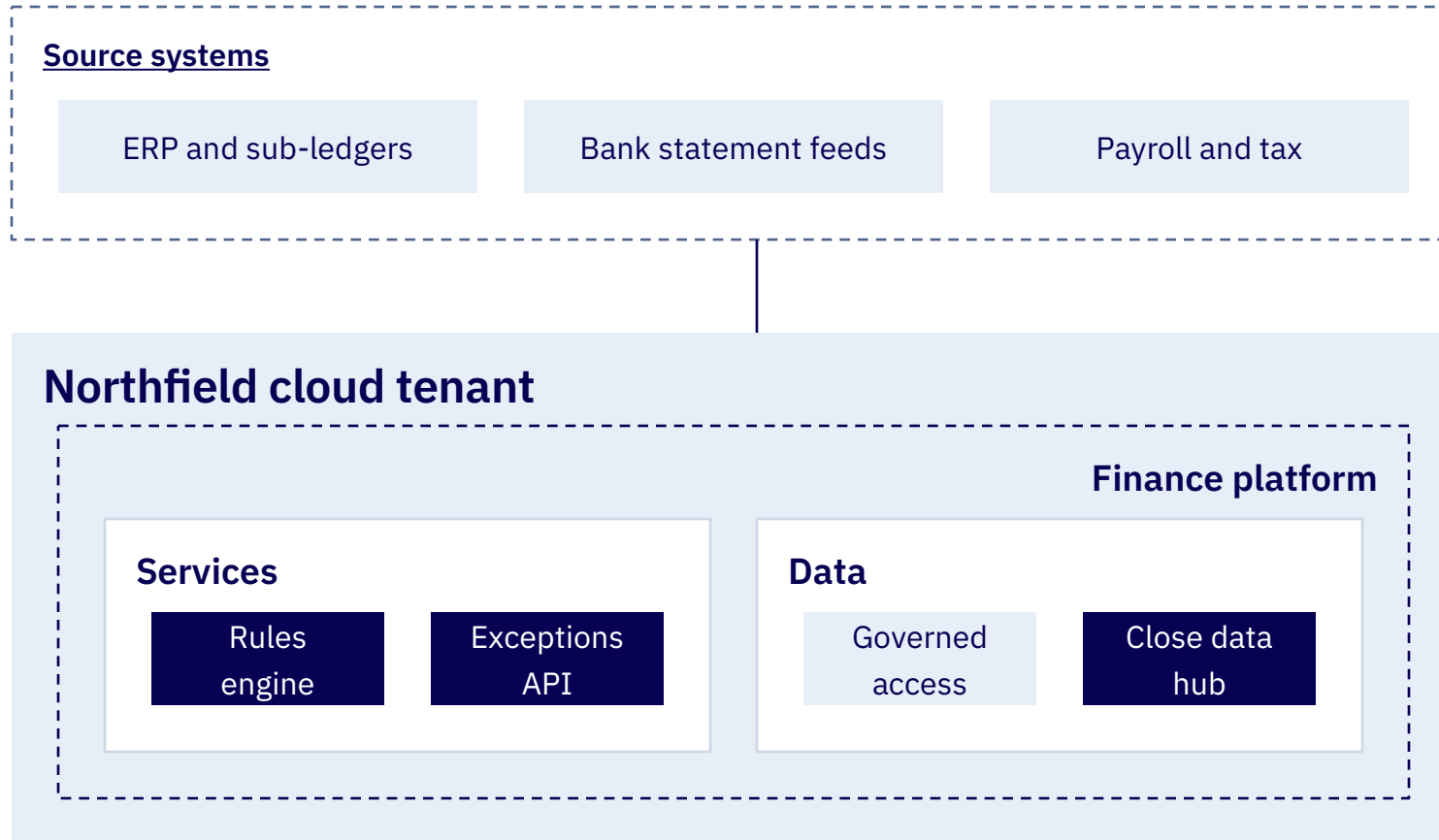
Source: West Monroe reference architecture.

Everything runs in one governed Northfield tenant



Source: West Monroe reference architecture.

One governed tenant runs everything



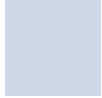
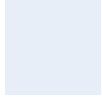
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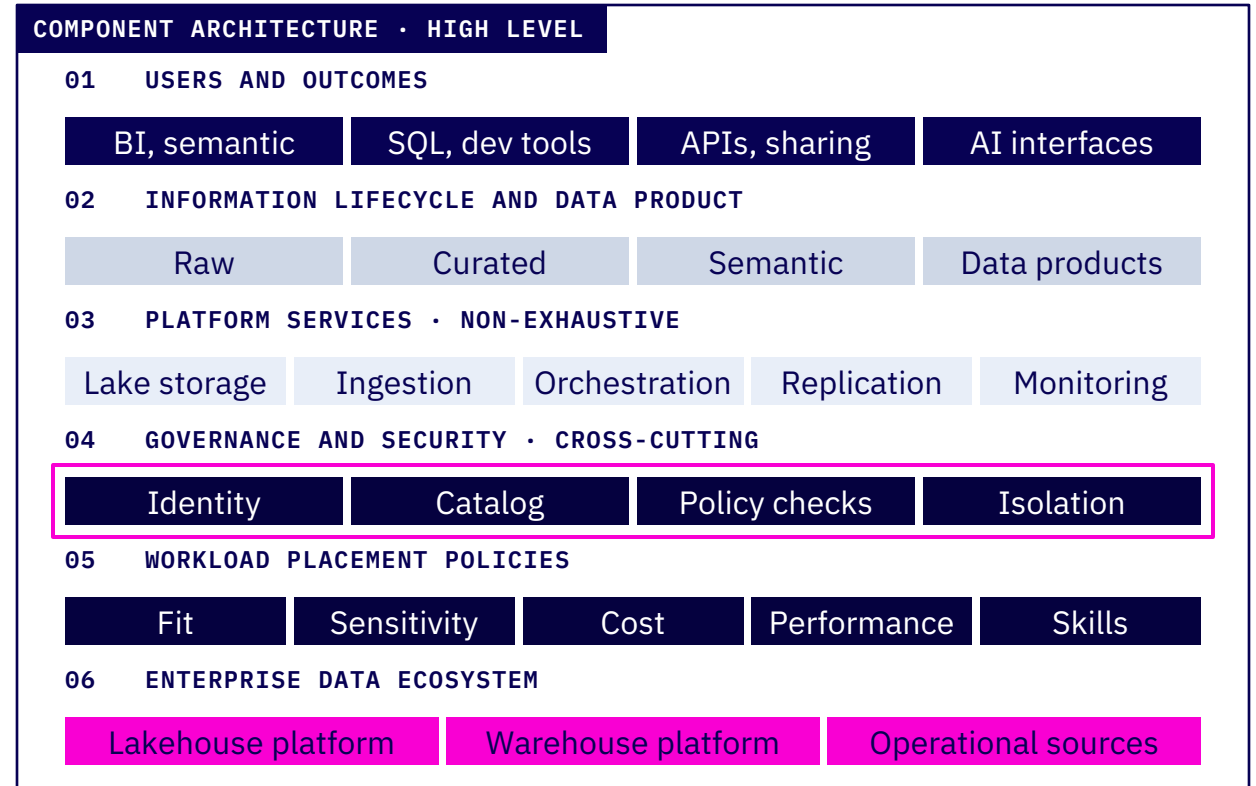
WHAT CHANGES

One tenant, one data path

Every source lands once, and every service reads from the same governed hub.

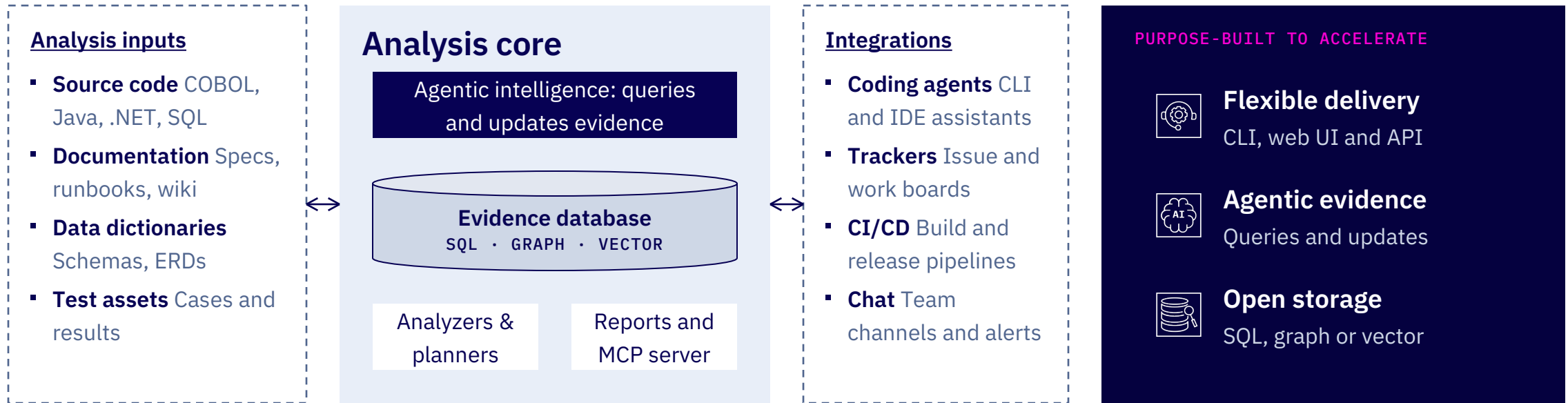
Six layers connect users to governed data products through reusable services

- 
Users and outcomes
 BI, developer, sharing and AI experiences
- 
Information lifecycle
 Raw to curated, semantic and governed products
- 
Platform services
 Reusable processing, interoperability, operations
- 
Governance and placement
 Embedded controls and clear placement criteria
- 
Enterprise data ecosystem
 Existing platforms, connected not consolidated



Source: West Monroe reference architecture for Northfield Health System.

AI and **code analysis** guide modernization that also improves the product itself



VALUE DELIVERED

12 months

to modernize, from 3–5 years

10+

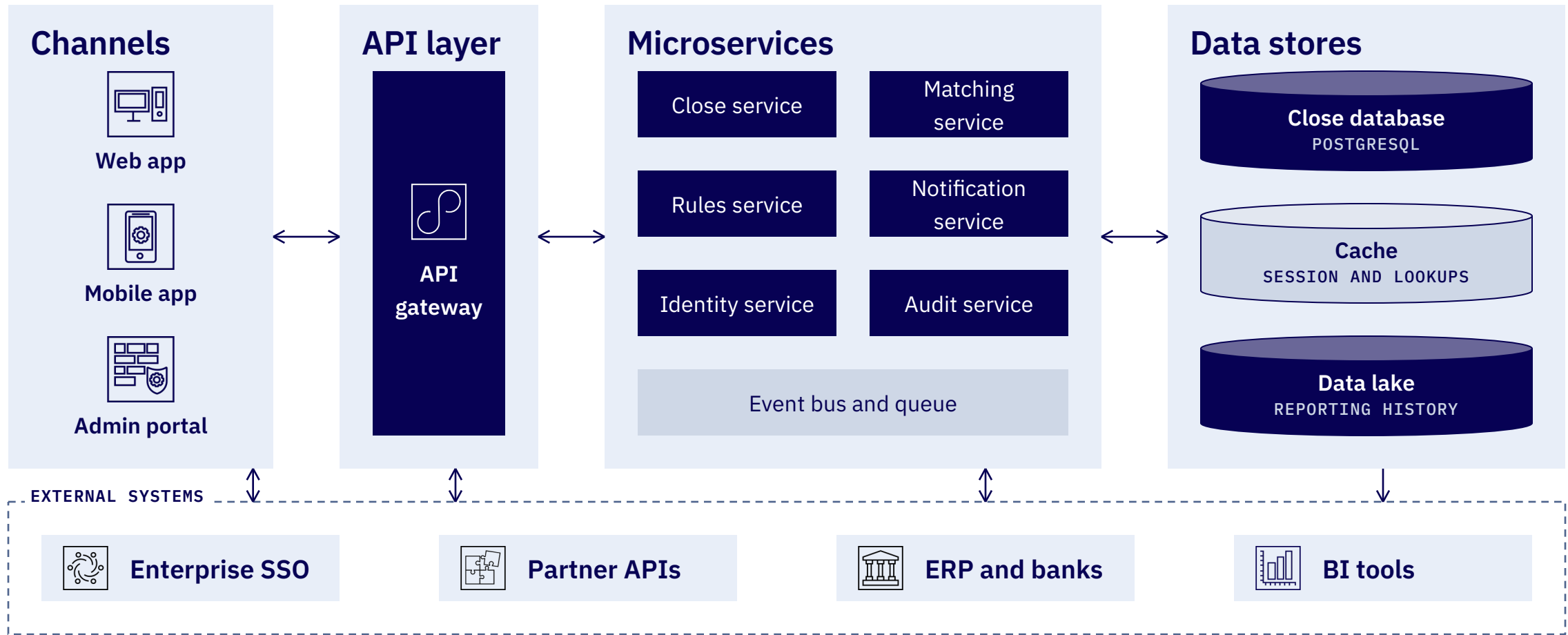
pre-built AI-guided workflows

Up to 80%

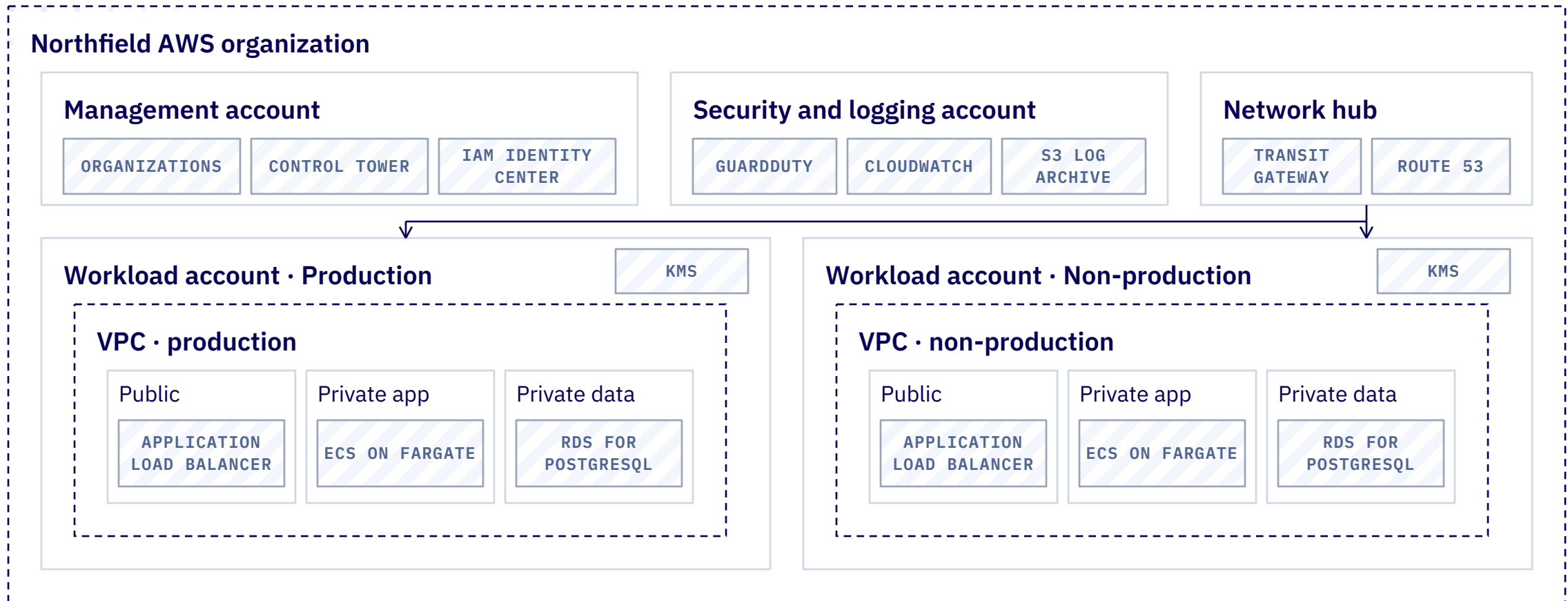
savings in re-engineering effort

Source: West Monroe delivery experience on comparable modernization programs.

Small services behind one gateway contain change

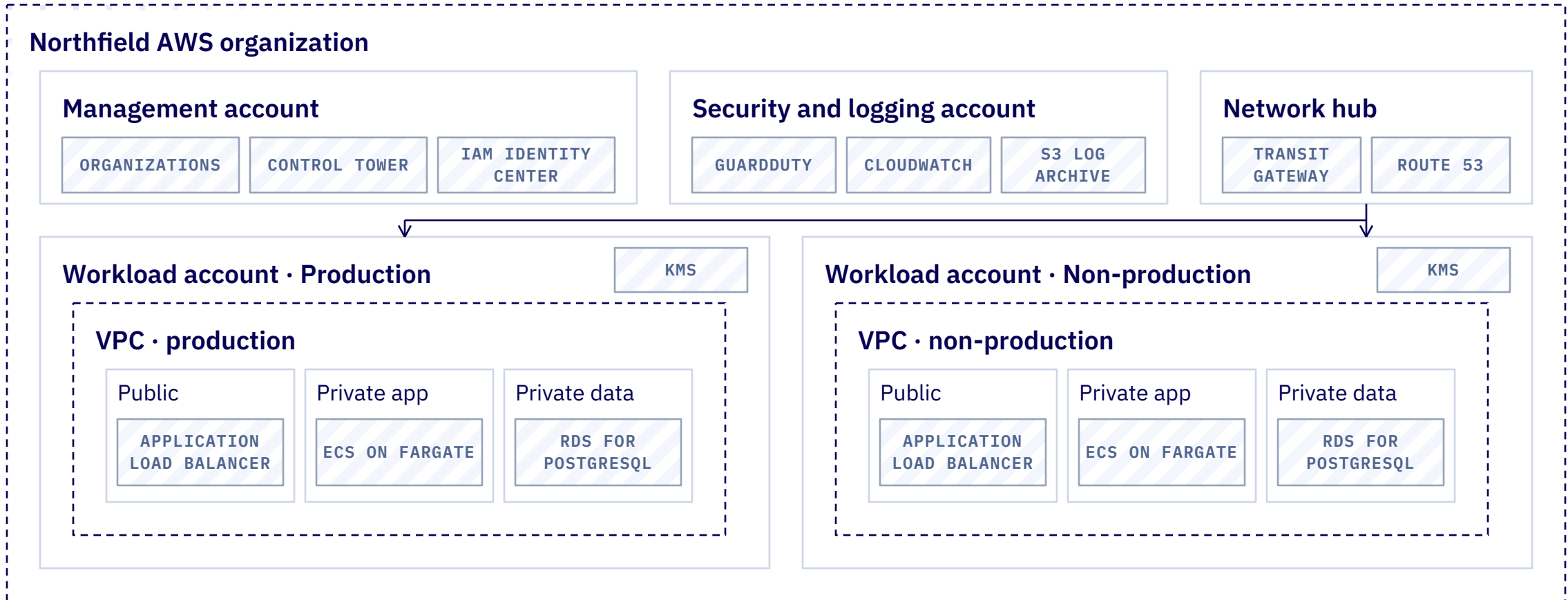


Workloads land in a governed multi-account foundation



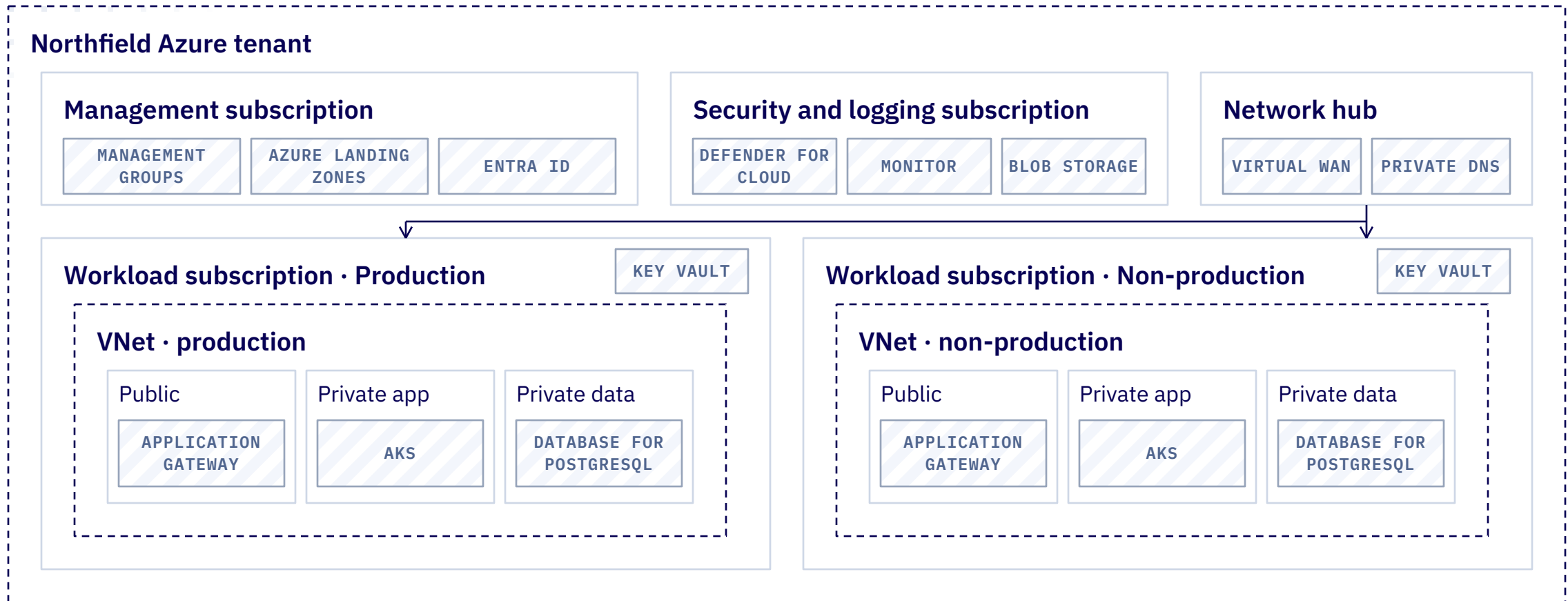
Source: Sample AWS services; logos and services swap per provider, and an Azure or GCP variant keeps the same structure.

Workloads land in a governed multi-account foundation



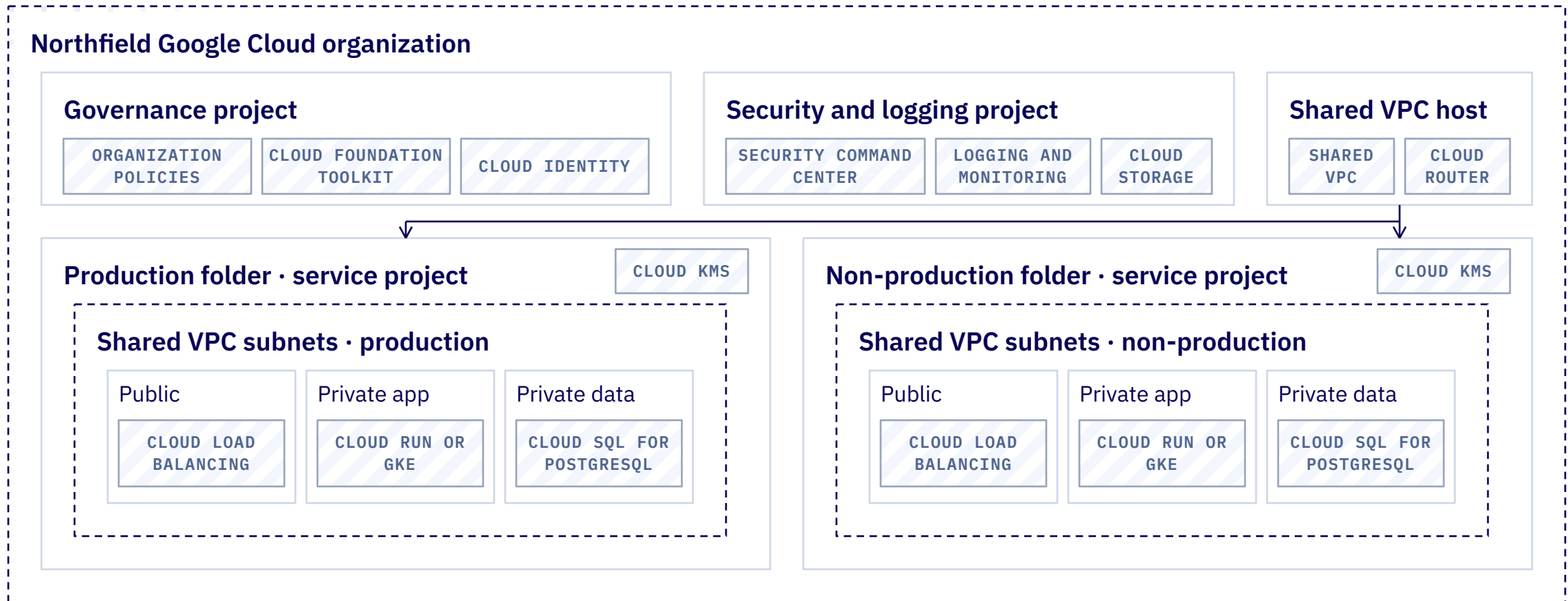
Source: Sample AWS services; logos and services swap per provider, and an Azure or GCP variant keeps the same structure.

Workloads land in a governed subscription foundation



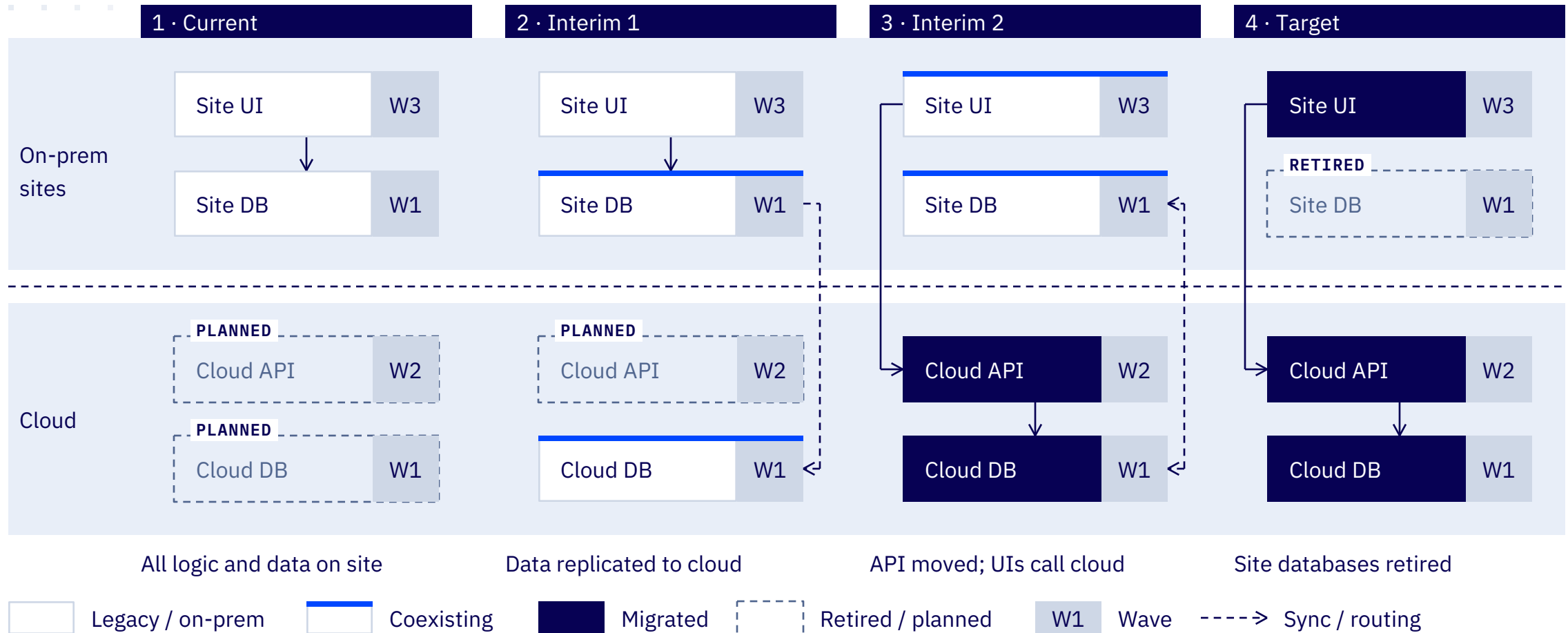
Source: Sample Azure services; logos and services swap per provider, and an AWS or GCP variant keeps the same structure.

Workloads land in a governed project foundation

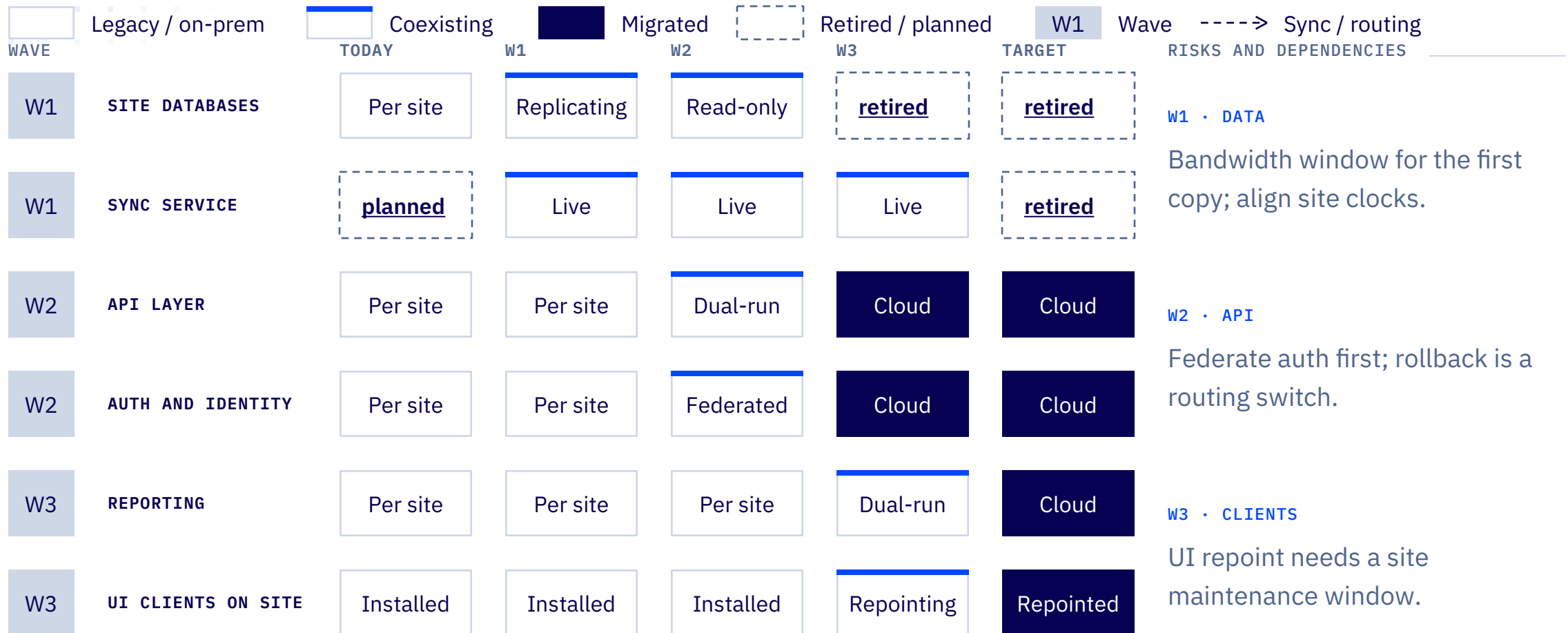


Source: Sample Google Cloud services; logos and services swap per provider, and the structure stays the same for AWS or Azure.

Four steps take each site from local to cloud



Three waves move data, then API, then clients



Three waves move data, then API, then clients

WAVE		TODAY	W1	W2	W3	TARGET	RISKS AND DEPENDENCIES
W1	SITE DATABASES	Per site	Replicating	Read-only	<u>retired</u>	<u>retired</u>	W1 • DATA Bandwidth window for the first copy; align site clocks.
W1	SYNC SERVICE	<u>planned</u>	Live	Live	Live	<u>retired</u>	W2 • API Federate auth first; rollback is a routing switch.
W2	API LAYER	Per site	Per site	Dual-run	Cloud	Cloud	W3 • CLIENTS UI repoint needs a site maintenance window.
W2	AUTH AND IDENTITY	Per site	Per site	Federated	Cloud	Cloud	
W3	REPORTING	Per site	Per site	Per site	Dual-run	Cloud	
W3	UI CLIENTS ON SITE	Installed	Installed	Installed	Repointing	Repointed	

Legacy

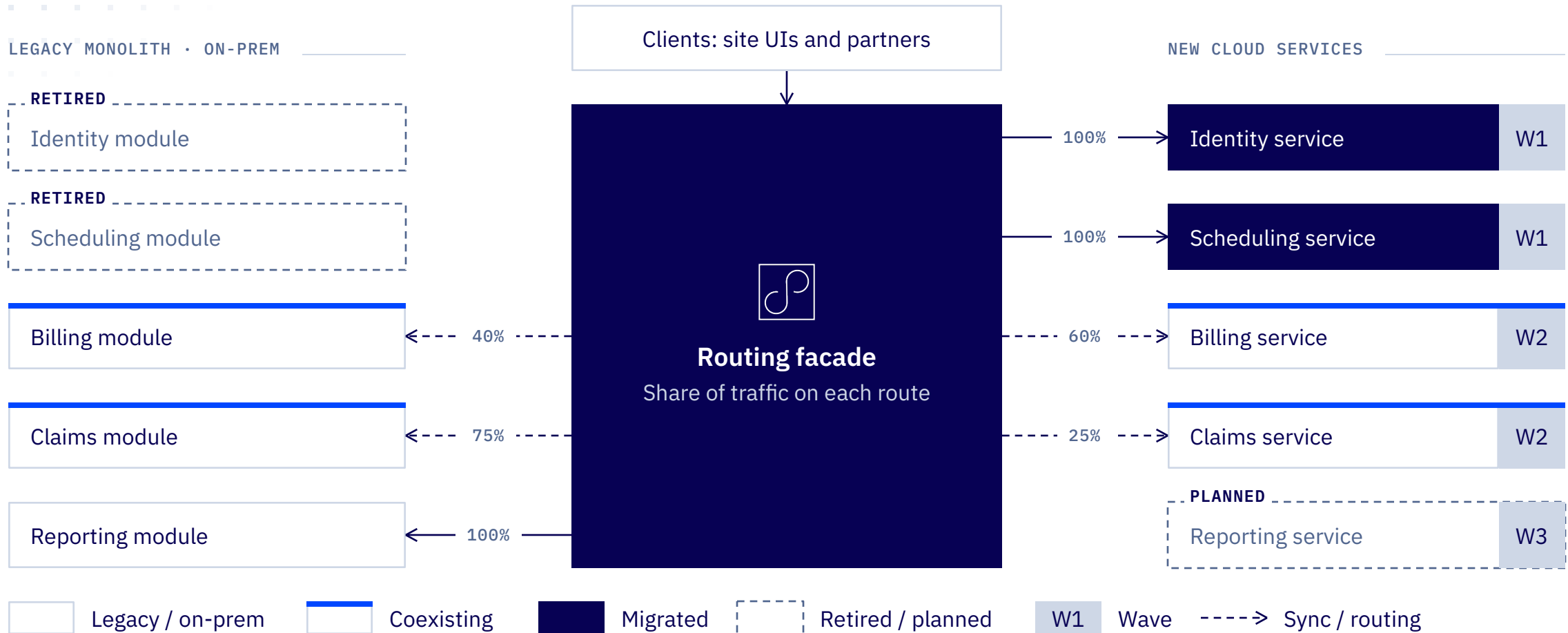
Coexisting

Migrated

Retired

Source: Sample migration plan; waves and states are illustrative and confirmed with each site before cutover.

A routing facade shifts traffic service by service



TRANSITION ARCHITECTURE

A routing facade shifts traffic service by service

Each percentage is the share of live traffic the facade sends down that route.

STATE KEY

 Legacy / on-prem

 Coexisting

 Migrated

 Retired / planned

 W1 Wave

----> Sync / routing

LEGACY • ON-PREM

RETIRED

Identity module

RETIRED

Scheduling module

Billing module

Claims module

Reporting module

Clients: site UIs, partners

Routing facade

Traffic share per route

NEW CLOUD SERVICES

Identity service

W1

Scheduling service

W1

Billing service

W2

Claims service

W2

PLANNED

Reporting service

W3

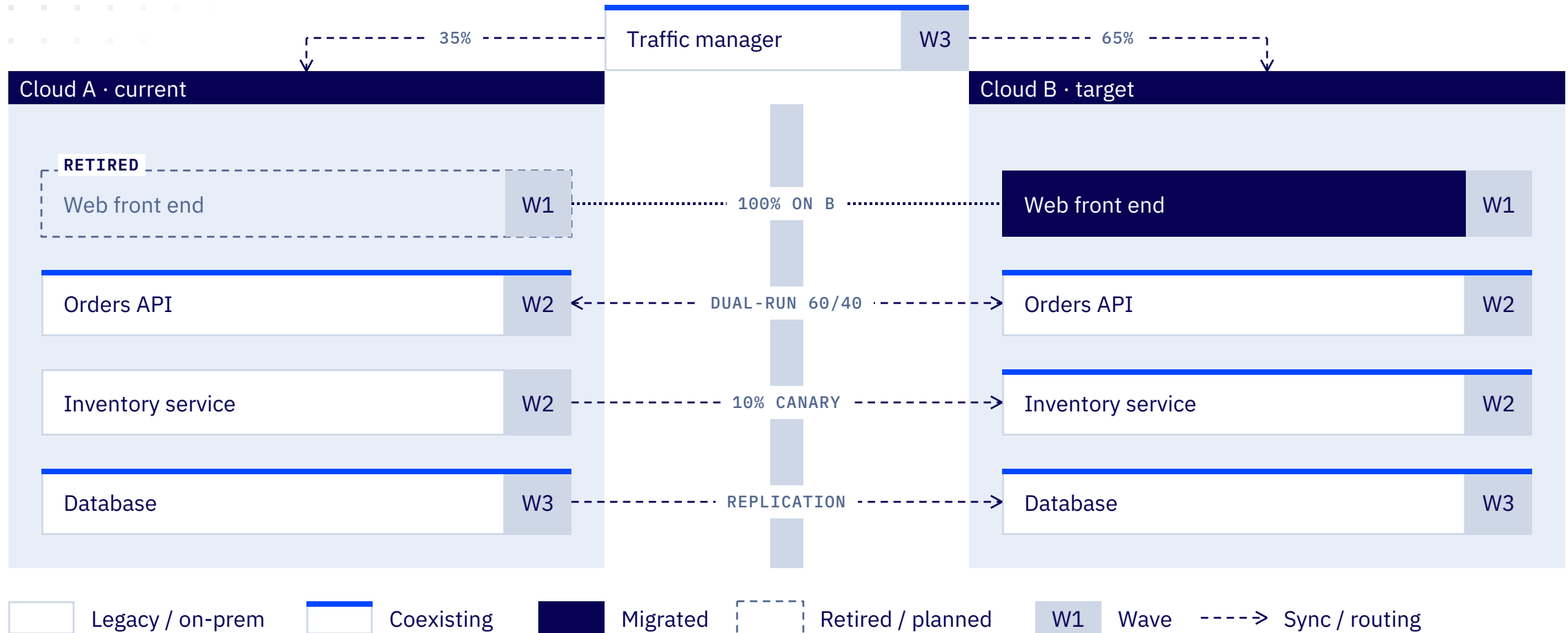
Four cohorts cut over, each behind a go/no-go gate



■ Coexisting (replicate and dual-run)
 □ On legacy
 ■ Migrated (cut over, site DB retired)
 — Gate

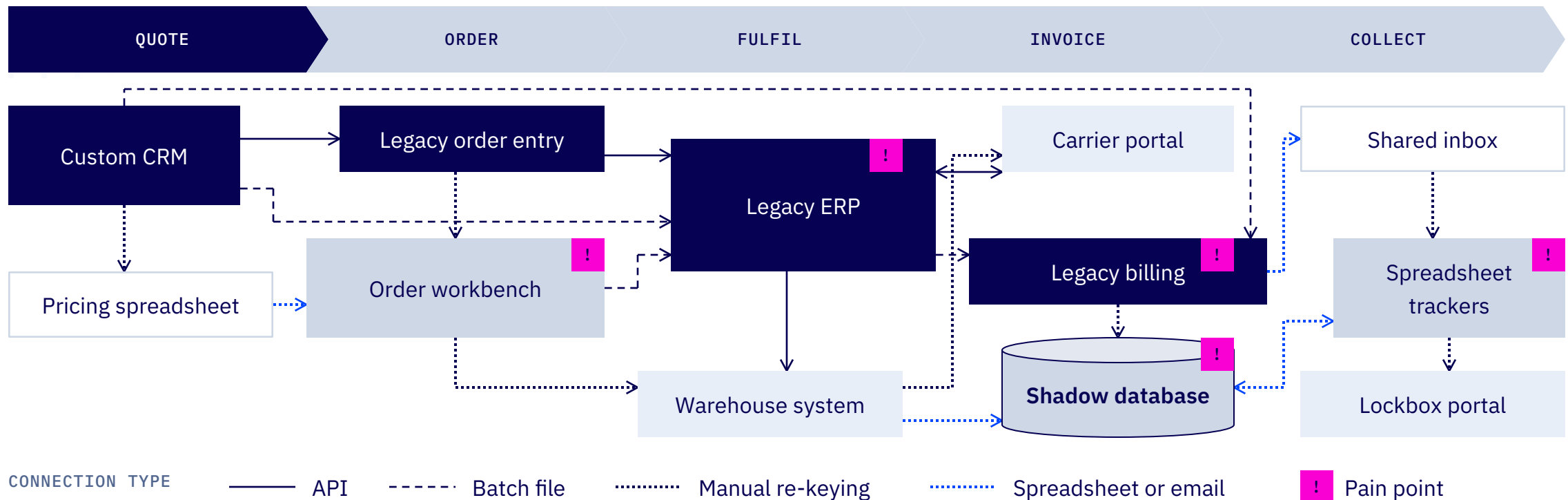
Go/no-go: data reconciles within 0.1 percent and no severity-1 defects for five days. Rollback: repoint the site UI to its local API; the site database stays read-only for 30 days.

Services move from Cloud A to Cloud B in waves



CURRENT STATE

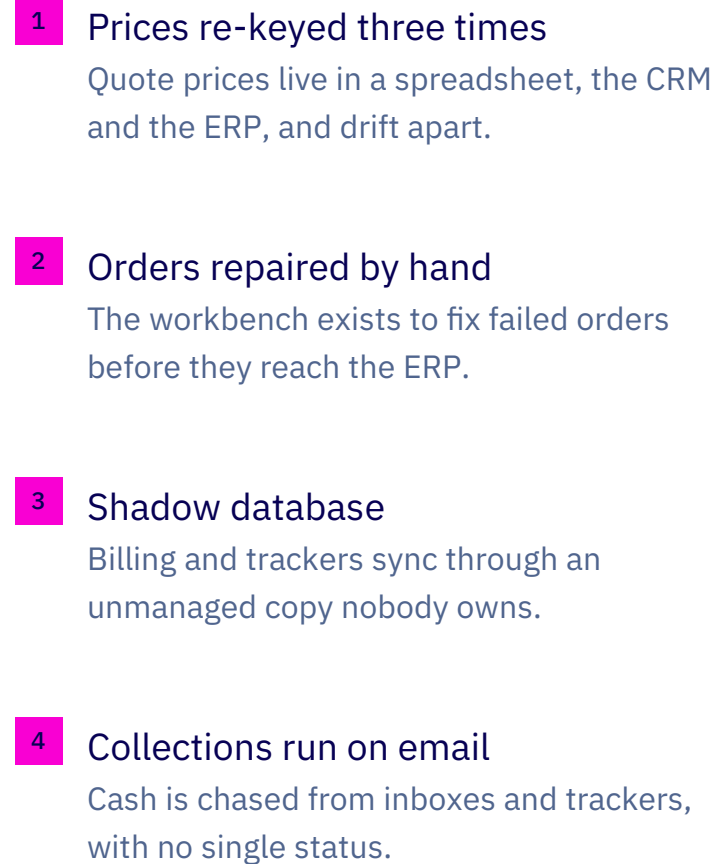
Quote to cash today crosses 12 systems



COST OF THE MESS	9 manual hand-offs per order	6 days from shipment to invoice	3 versions of customer data
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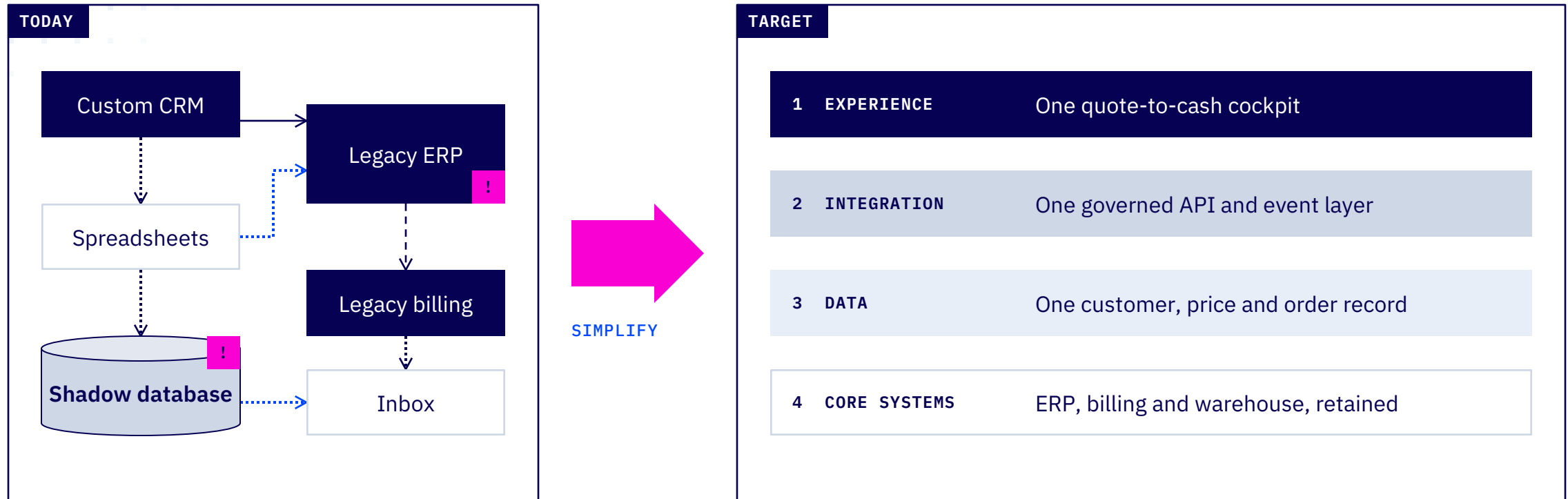
Source: Illustrative current state. Sample figures for layout only; replace with current firm data before use.

Four hotspots drive most of the rework



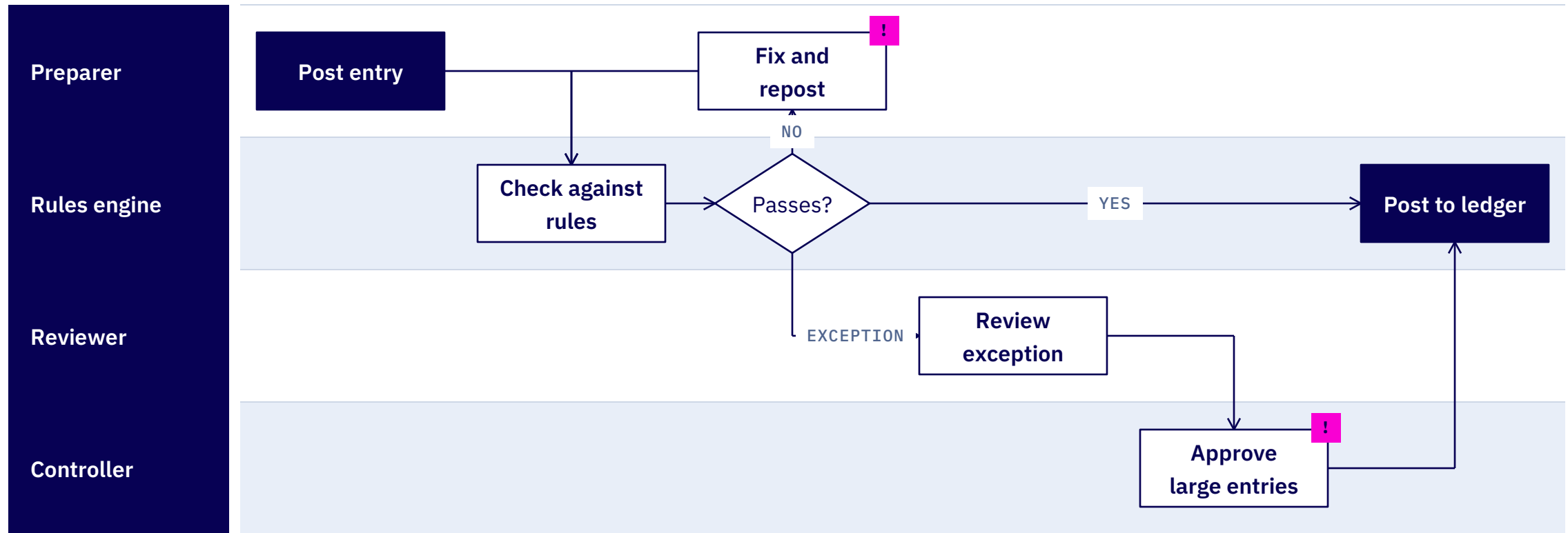
— API - - - - - Batch file Manual re-keying
 Spreadsheet or email ! Pain point

From a tangled landscape to one connected platform



Replace point-to-point links and re-keying with one governed integration layer.

Journal review hands off four times before posting

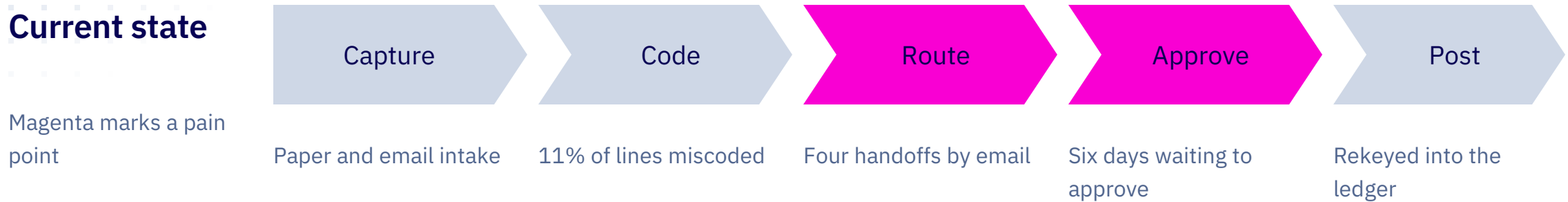


! PAIN POINT RAISED IN INTERVIEWS

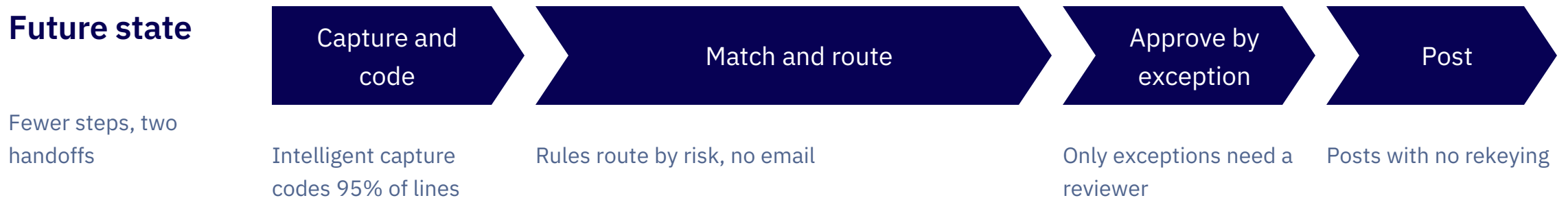
Source: Northfield close interviews, Sep 2026.

Redesign cuts invoice handling from six handoffs to two

Current state



Future state

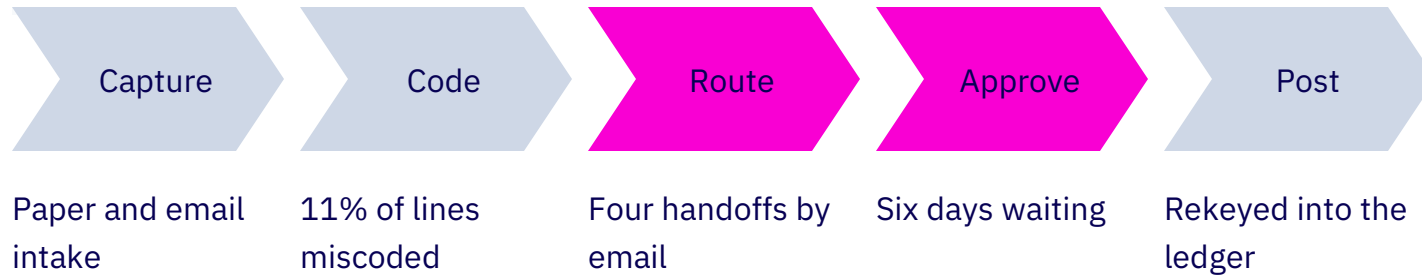


Cycle time 14 days to 5 days · Handoffs 6 to 2 · Rekeying removed

Source: Northfield Health System invoice sample, Q3 FY26.

Redesign cuts handoffs from six to two

Current state: magenta marks a pain point



Future state: fewer steps, two handoffs



Source: Northfield Health System invoice sample, Q3 FY26.

RESULT

14 days down to 5

Handoffs fall from six to two, and no invoice is rekeyed.

Persistent, agent-ready context can compress the path from prototype to production

CONTEXT CARRIED FORWARD

Evidence database

Source code, components, data and controls, with confidence.

Business process map

Steps, rules, exceptions and users linked to technology.

Decision trail

Approved outcomes, design choices and open questions.

Agent-ready context layer

■	PROCESS	business activity
■	RULE	what must hold true
■	COMPONENT	where behavior lives
■	REQUIREMENT	what must change
■	TEST	how it is proven

BUILD WORK ACCELERATED

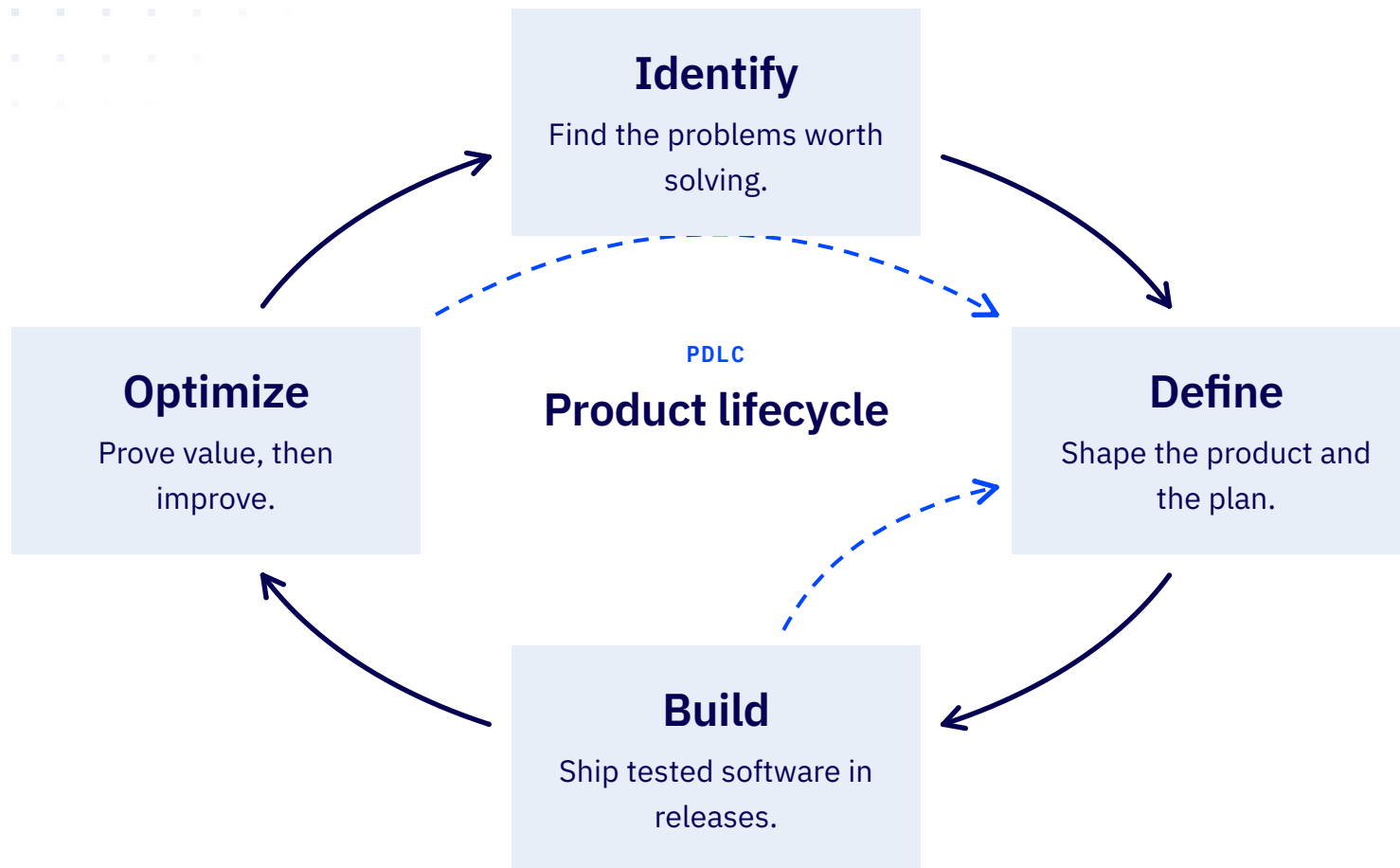
- 1 Prototype** **FASTER FEEDBACK**
Options grounded in current behavior
- 2 Specify** **LESS REDISCOVERY**
Traceable stories and edge cases
- 3 Build** **FIRST-PASS FIT**
Relevant rules and design decisions
- 4 Test** **EARLIER DEFECTS**
Known states turned into tests



THE SHIFT

Retrieve the right context for the next task instead of rediscovering it, with review and testing still authoritative.

The lifecycle is a loop: every phase can send work back



BUILD TO DEFINE

Findings reshape scope

Testing and clinician feedback change what we build next.

OPTIMIZE TO DEFINE

Live data reopens design

Adoption and usage evidence change the solution.

ANY PHASE

New problems enter anywhere

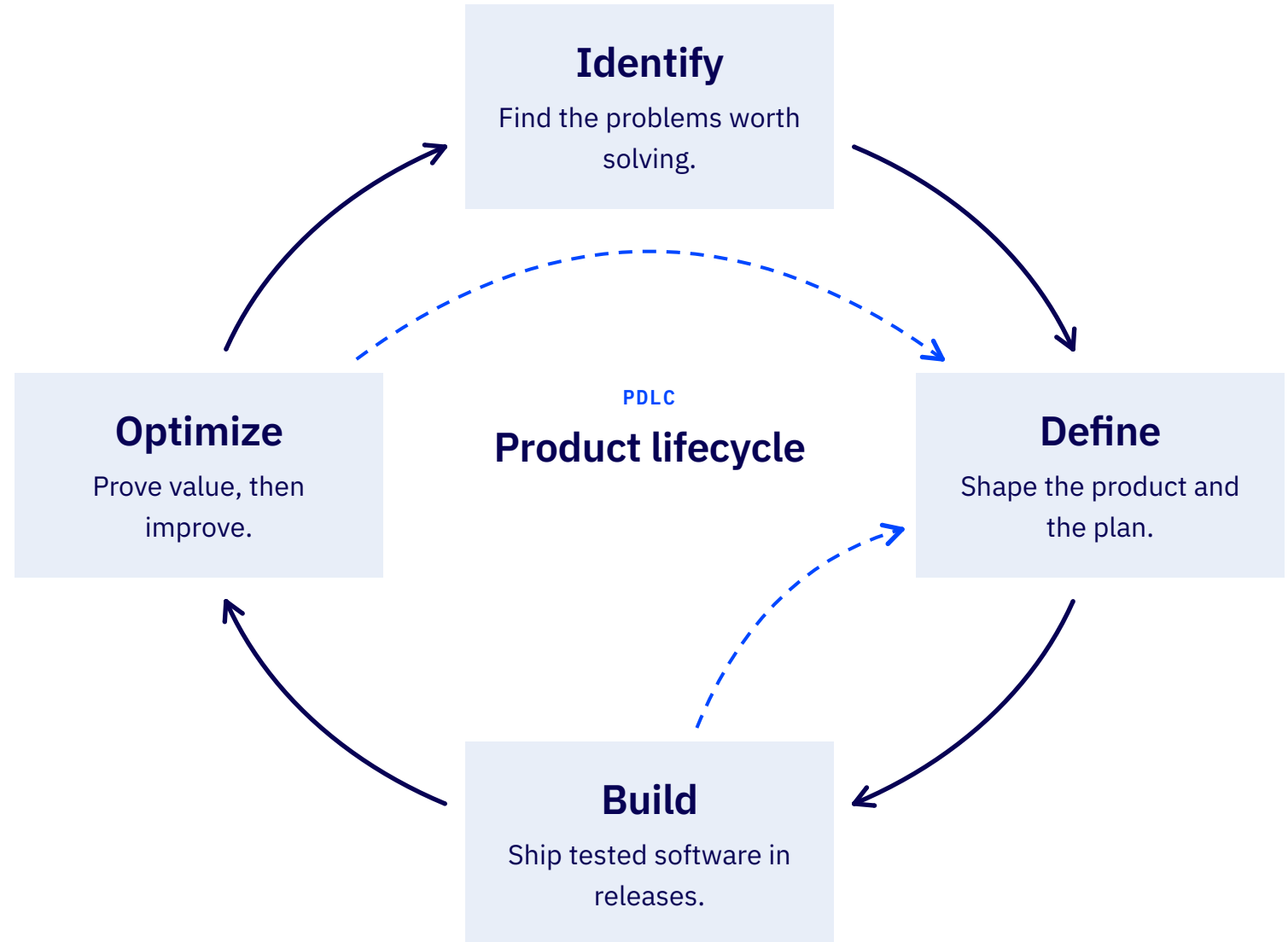
The patient app backlog is never closed.

Phases loop back as evidence arrives

- **Build to Define.** Test findings reshape scope.
- **Optimize to Define.** Live data reopens the design.
- **Any phase.** New problems enter at any time.

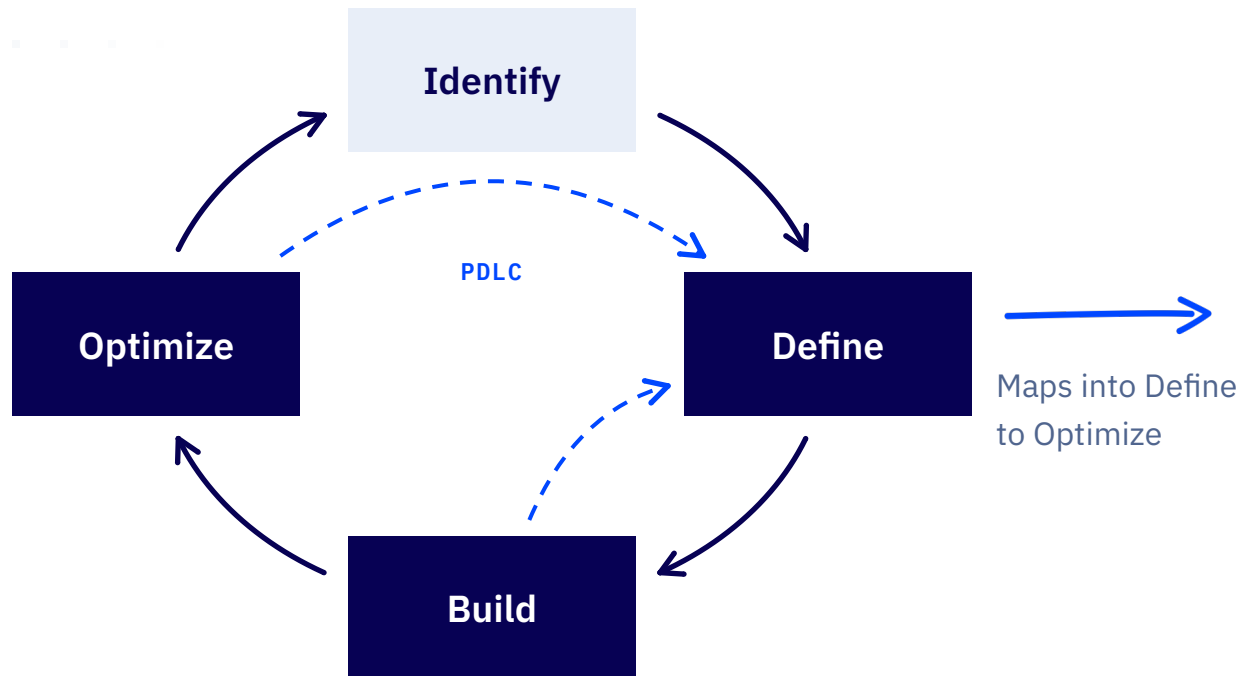
READING THE CYCLE

Solid arrows are the plan; dashed arrows are rework.



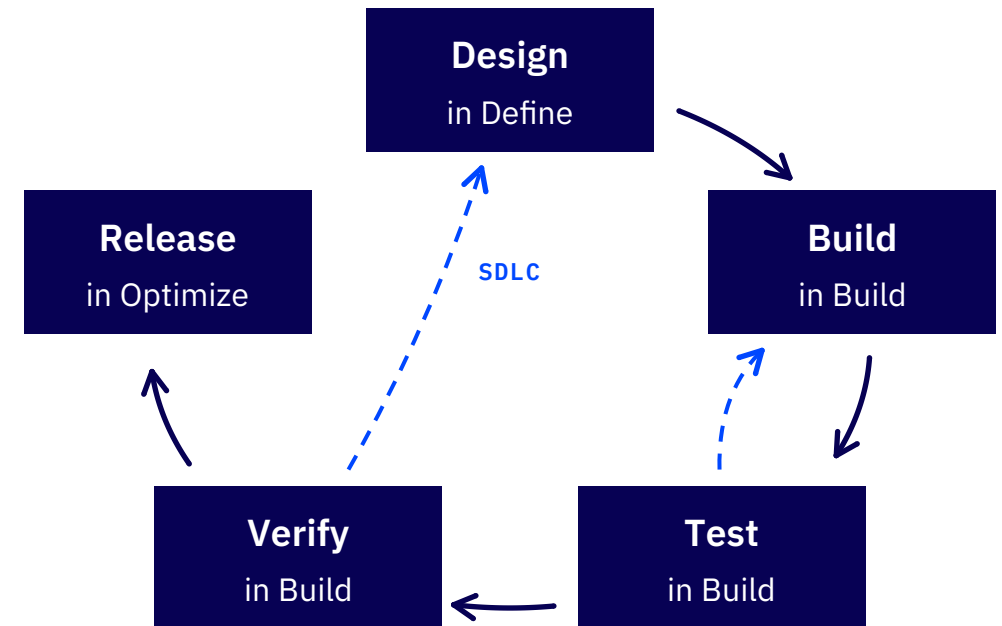
The SDLC is the slice of the PDLC from Define to Optimize

PDLC: THE SUPERSET



Four phases that loop back. Dark phases are where the SDLC runs.

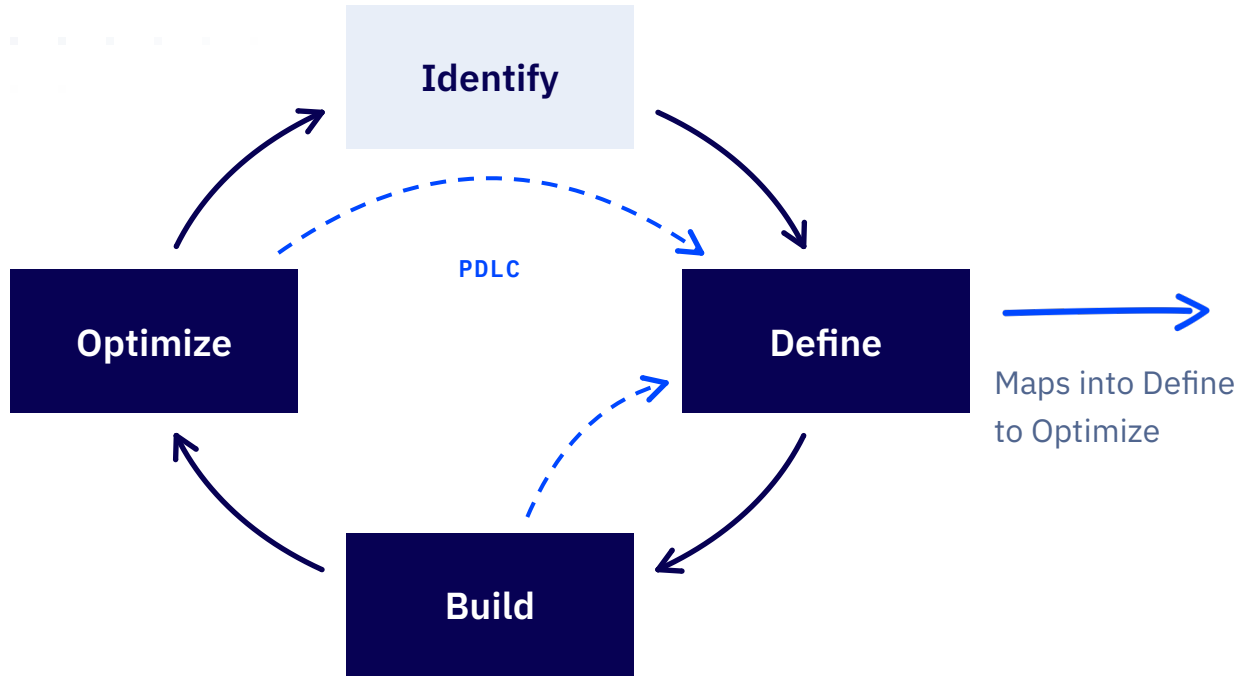
SDLC: A SUBSET



Five engineering steps that loop back on defects and gaps. Tags show the PDLC phase.

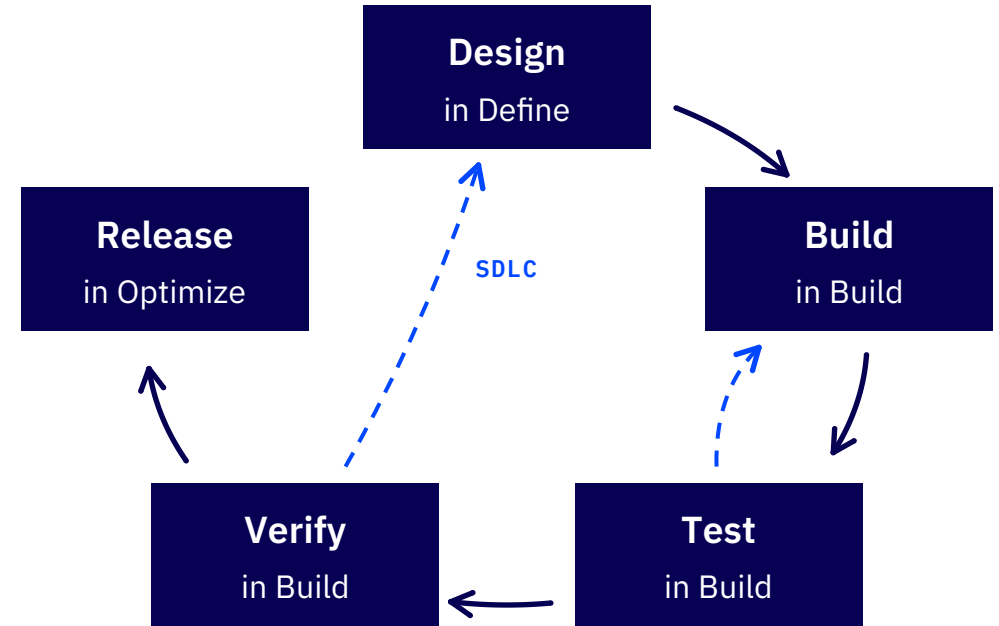
The SDLC is the slice of the PDLC from Define to Optimize

PDLC: THE SUPERSET



Four phases that loop back. Dark phases are where the SDLC runs.

SDLC: A SUBSET



Five engineering steps that loop back on defects and gaps. Tags show the PDLC phase.

HOW TO READ IT

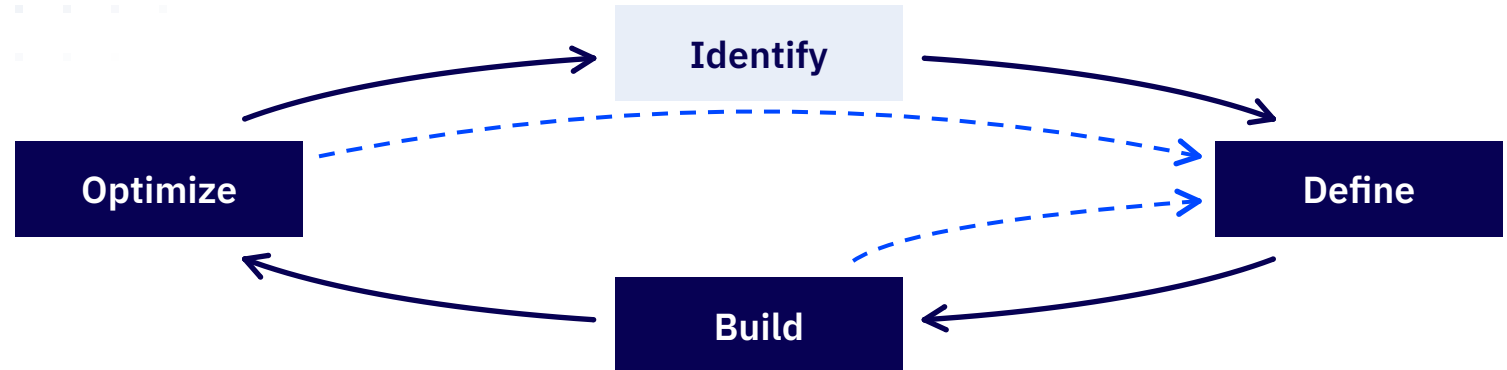
PDLC is the superset; the SDLC is a subset.

The SDLC runs from Define to Optimize. Dark phases are where it applies; both loop back.

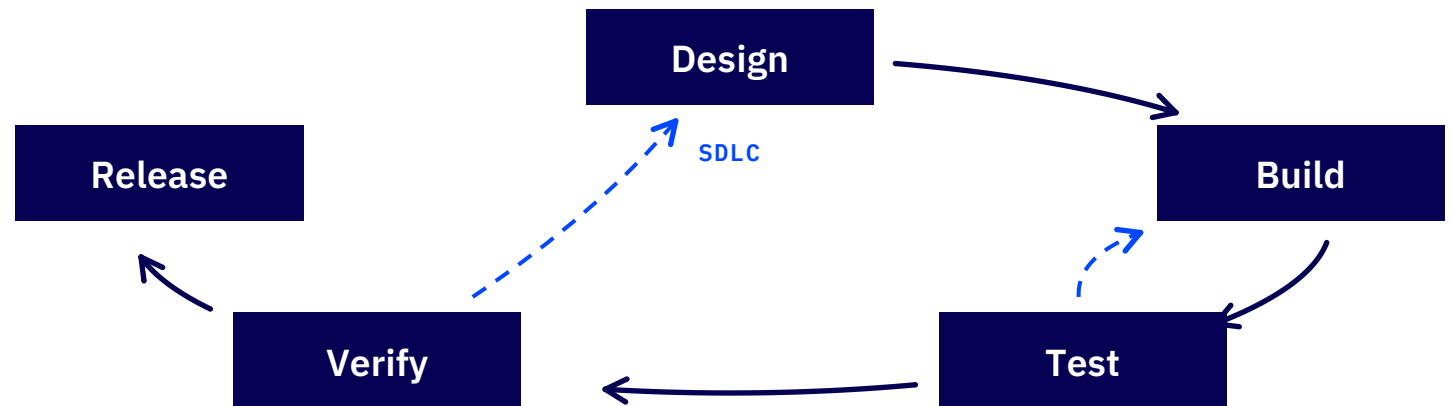
PRODUCT LIFECYCLE

Where the SDLC sits inside the PDLC

PDLC: THE SUPERSET, FOUR PHASES THAT LOOP BACK

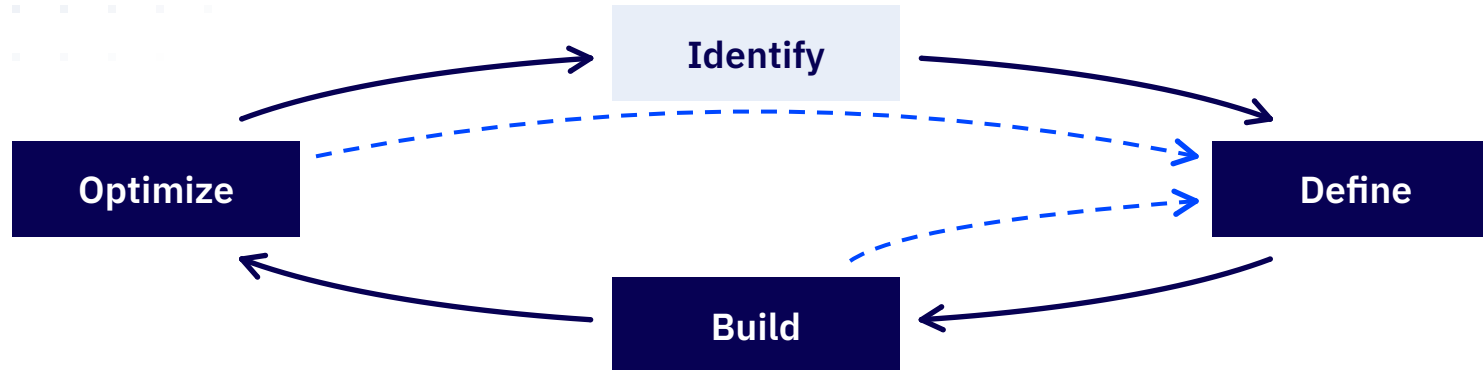


SDLC: A SUBSET, DEFINE TO OPTIMIZE

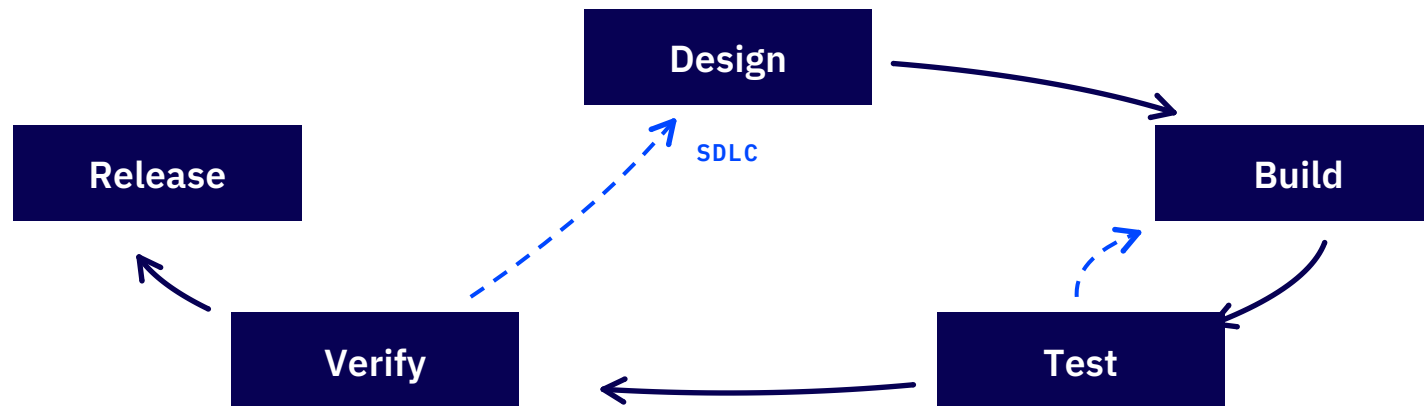


Where the SDLC sits inside the PDLC

PDLC: THE SUPERSET, FOUR PHASES THAT LOOP BACK



SDLC: A SUBSET, DEFINE TO OPTIMIZE

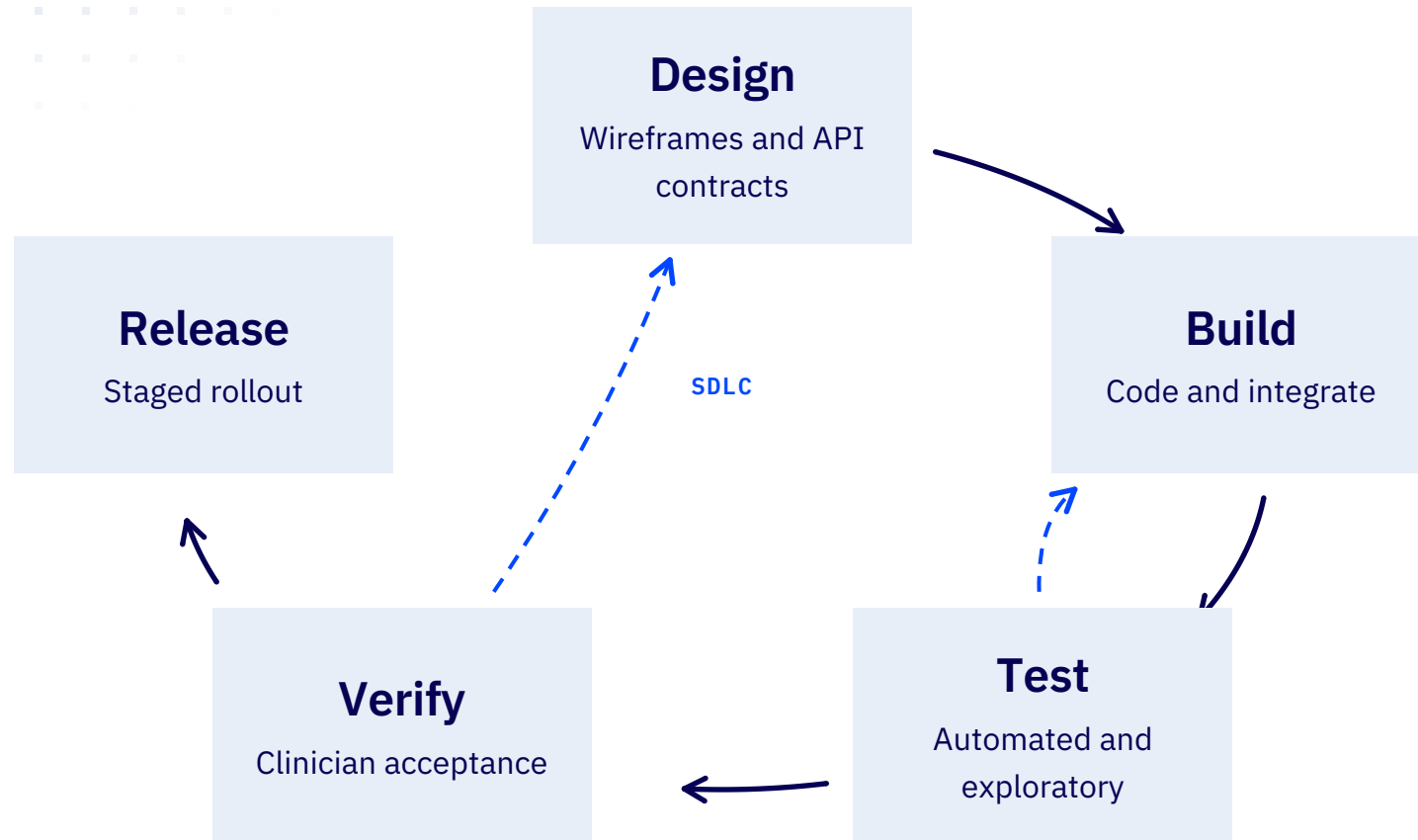


HOW TO READ IT

PDLC is the superset; the SDLC is a subset.

The SDLC runs from Define to Optimize. Dark phases are where it applies; both loop back.

Five steps, and two ways work loops back



TEST TO BUILD

Failed tests return work

Defects go back to the sprint, not to a later phase.

VERIFY TO DESIGN

Gaps reopen the design

Workflow gaps change the design before release.

RELEASE

Hand-off to Optimize

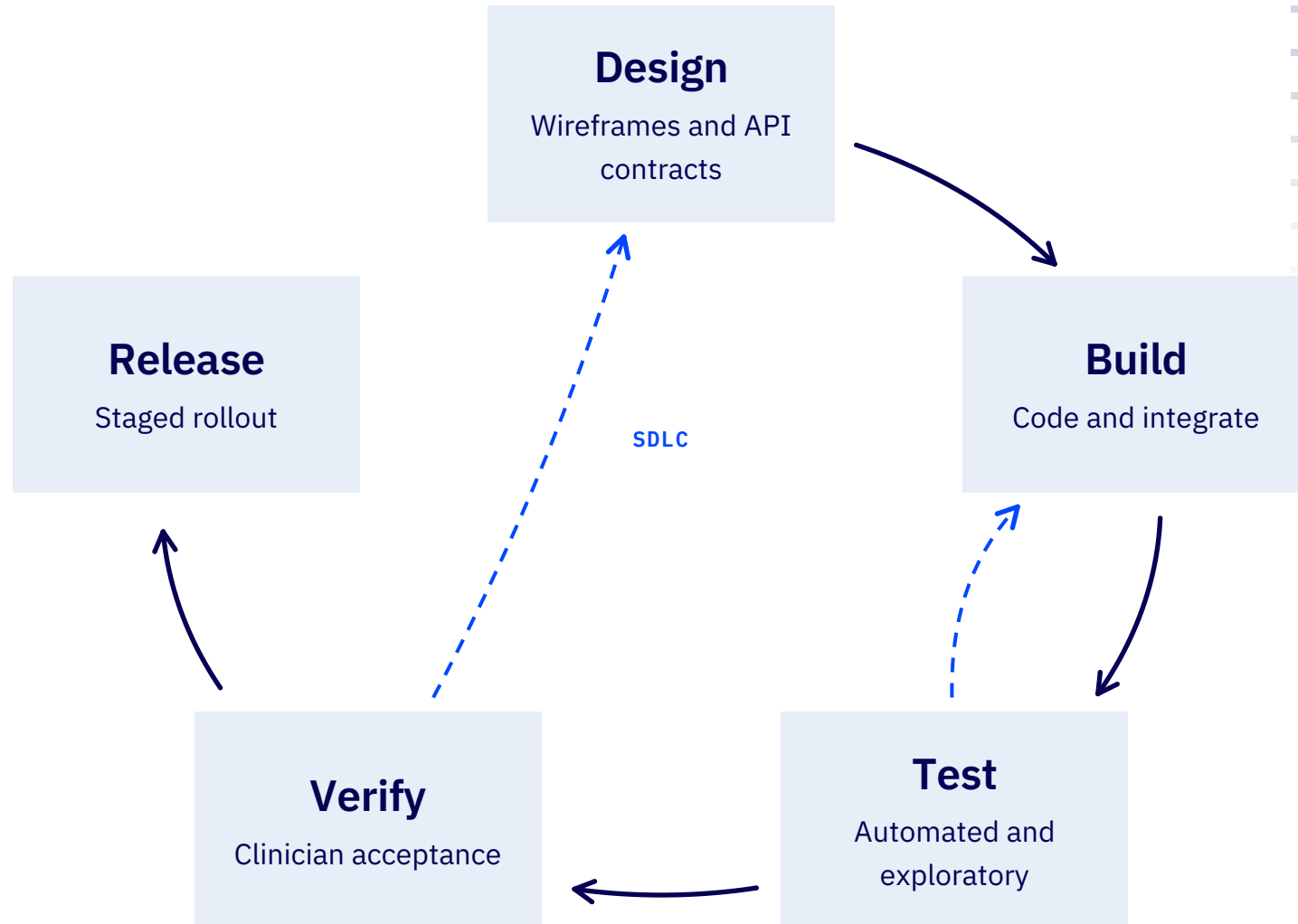
Live usage data starts the next PDLC loop.

Defects and gaps loop back inside the SDLC

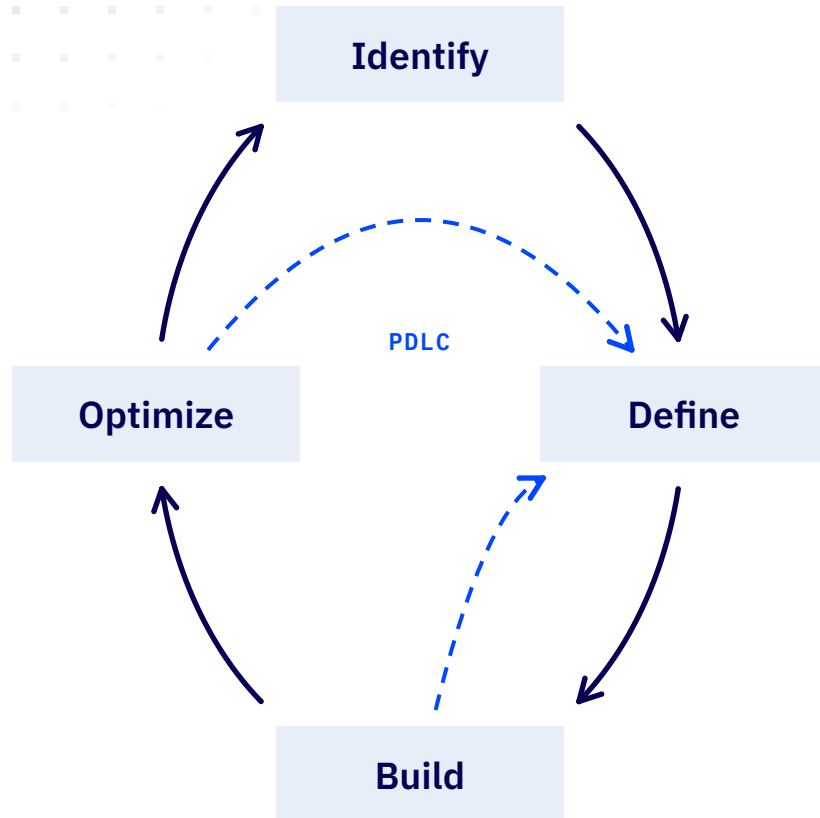
- **Test to Build.** Defects return to the sprint.
- **Verify to Design.** Clinician gaps reopen the design.

HAND-OFF

Release feeds Optimize, where live usage data starts the next PDLC loop.



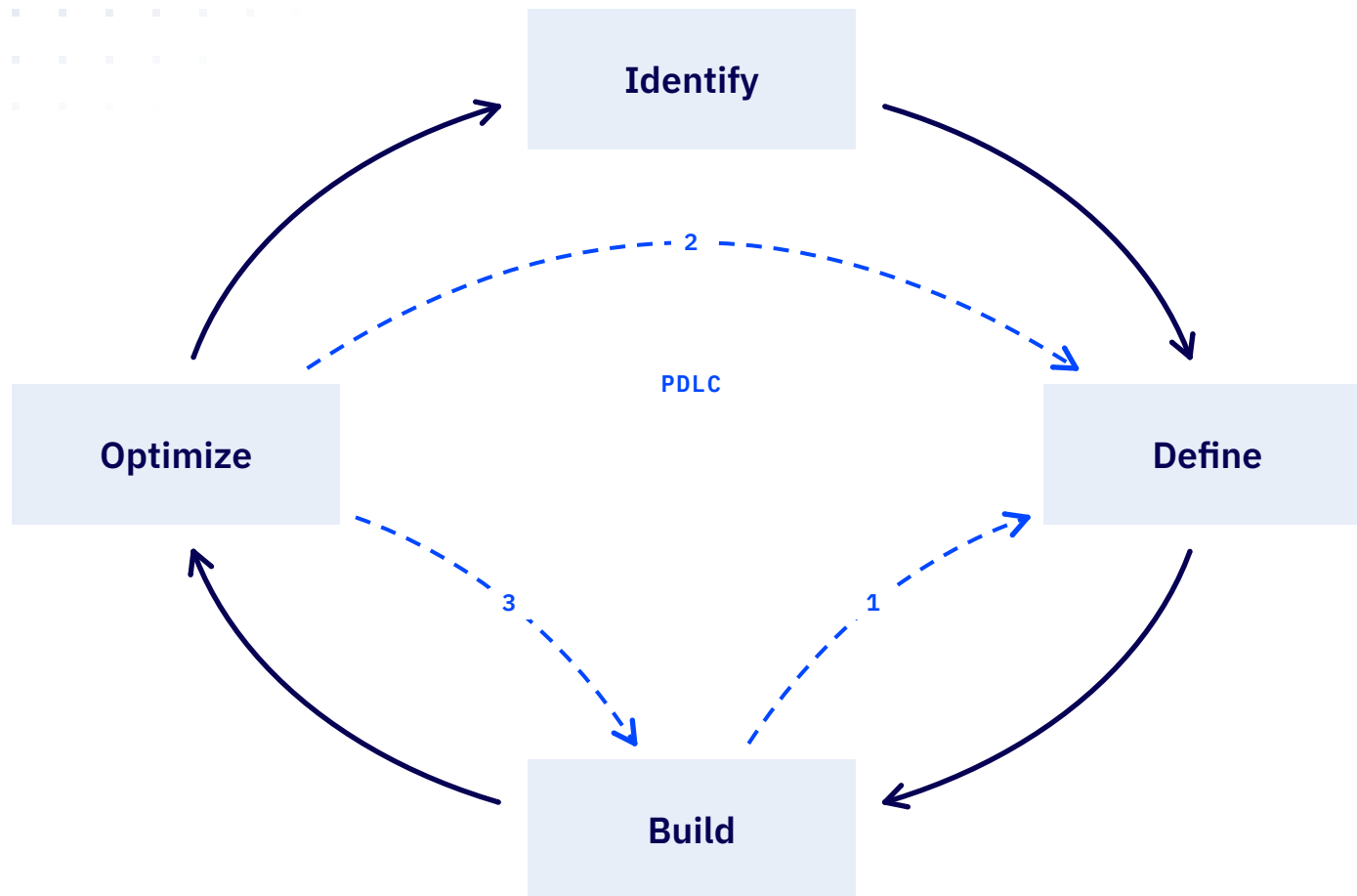
What we see and do in each phase



PHASE	WHAT WE OBSERVE	WHAT WE DO
Identify	38% of patients abandon booking at step 3	Interview patients and staff; rank the bets
Define	Three booking journeys overlap	Shape one journey; plan releases and risks
Build	Eight-week cycle; manual regression tests	Two-week sprints; automate tests
Optimize	Adoption plateaus with no loop to the backlog	Track adoption; run experiments

Source: Northfield Health System lifecycle review, Sep 2026.

Three loop-backs keep the product honest



01 Build back to Define

Test findings change scope before release.

02 Optimize back to Define

Adoption data reopens the booking design.

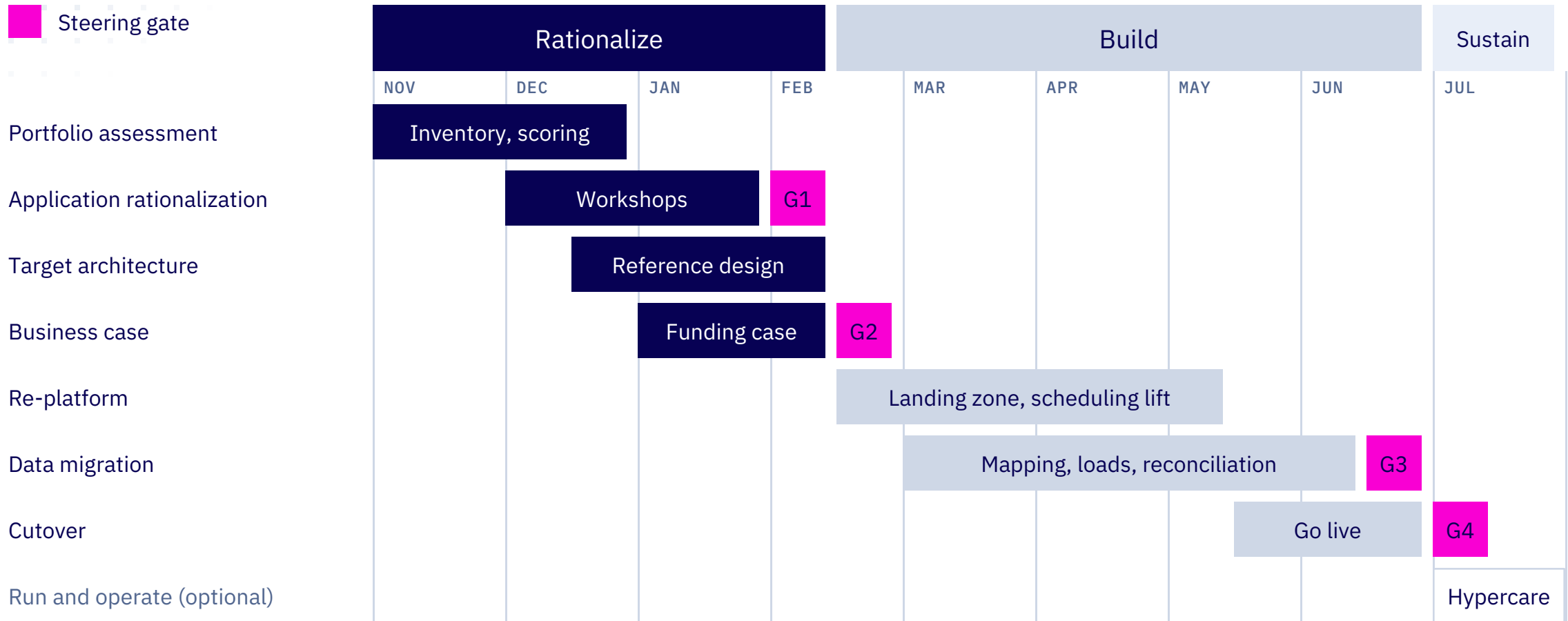
03 Optimize back to Build

Live defects return to the next sprint.

RULE OF THUMB

Give every loop-back an owner and a review cadence, or it will not happen.

Nine months from portfolio view to Northfield cutover



Four stages take the practice from ad hoc to optimized

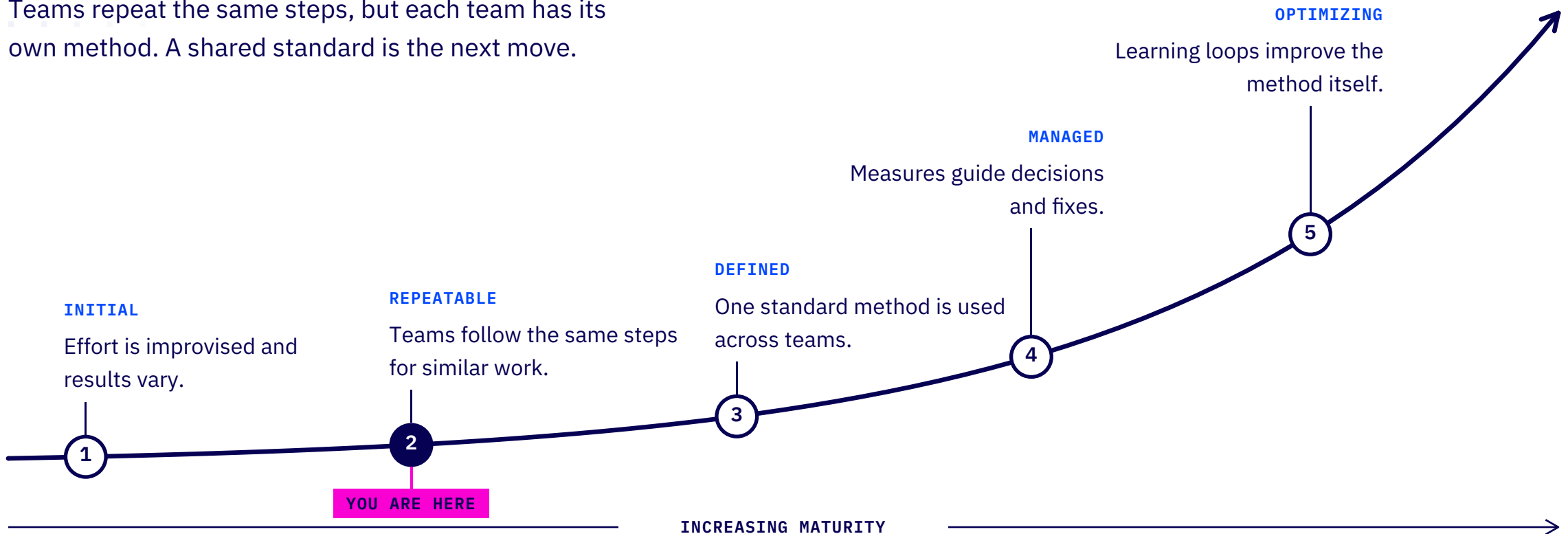
Each stage builds on the last. Early stages take effort and show little; the climb steepens once practice is shared.



CURRENT STATE

Northfield sits at stage two of five

Teams repeat the same steps, but each team has its own method. A shared standard is the next move.



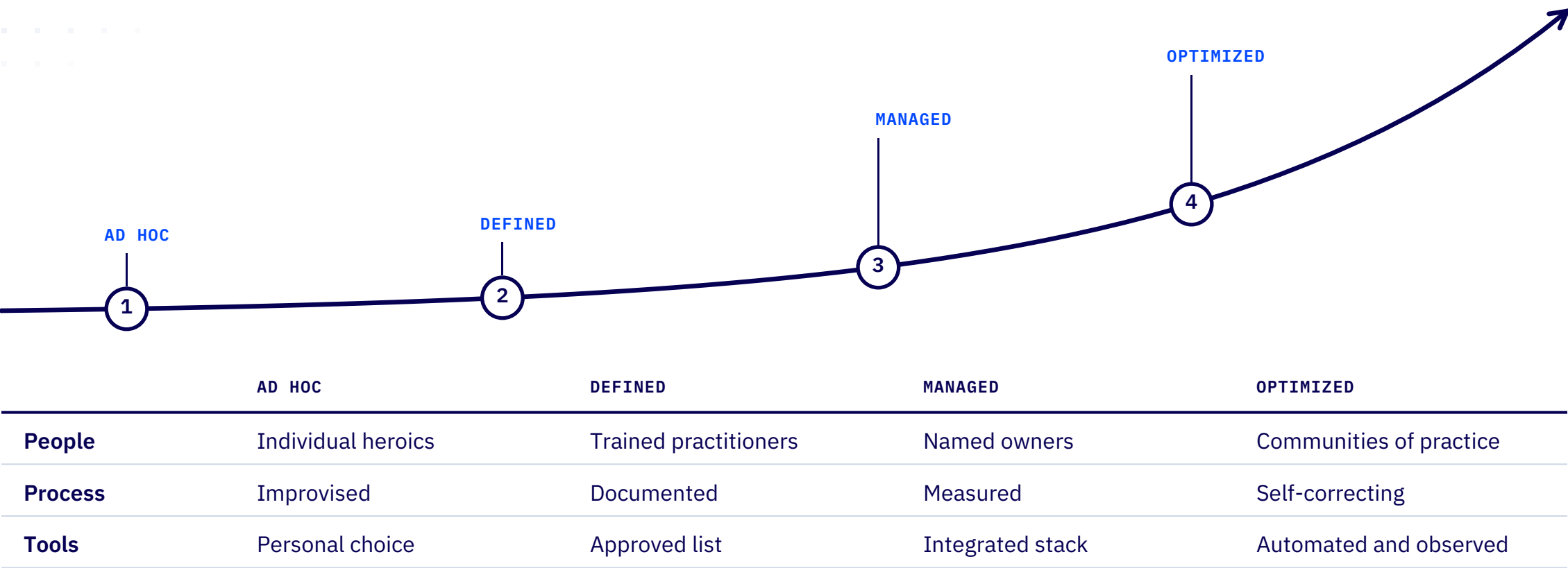
The filled marker is where Northfield is today.

Four stages take the practice from ad hoc to optimized

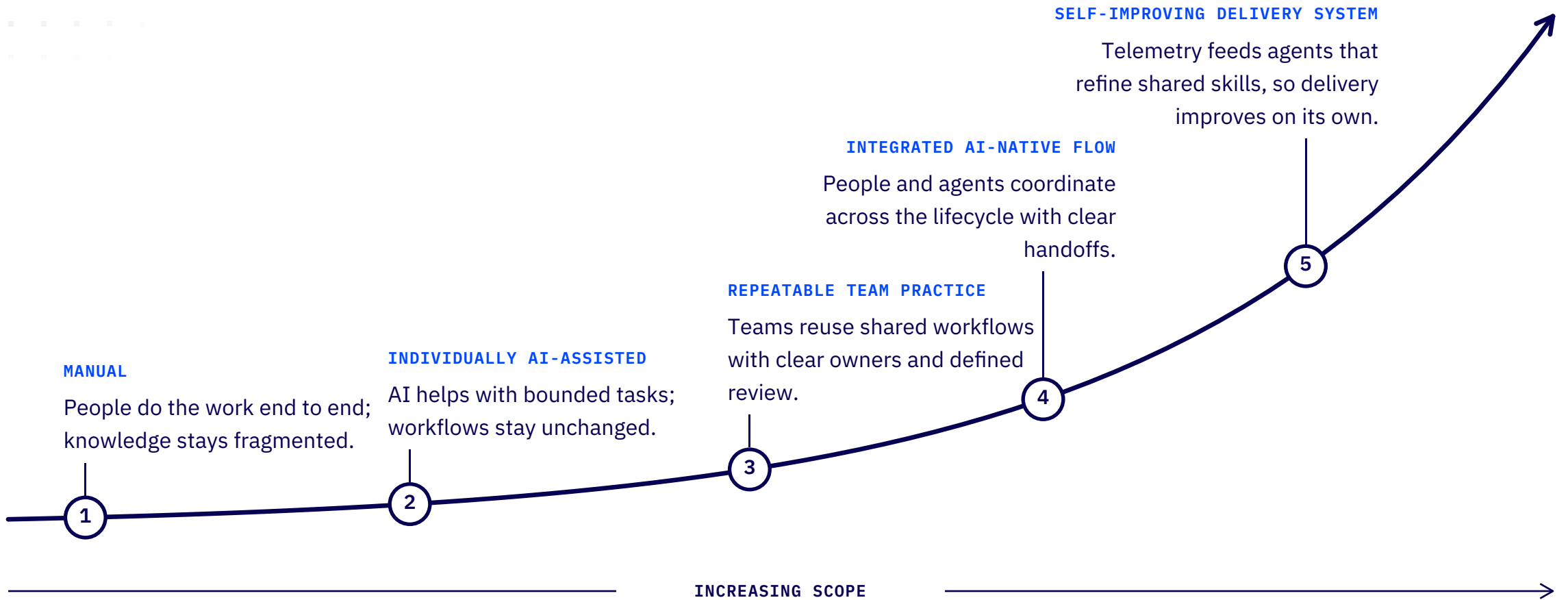
Each stage builds on the last. Early stages take effort and show little; the climb steepens once practice is shared.



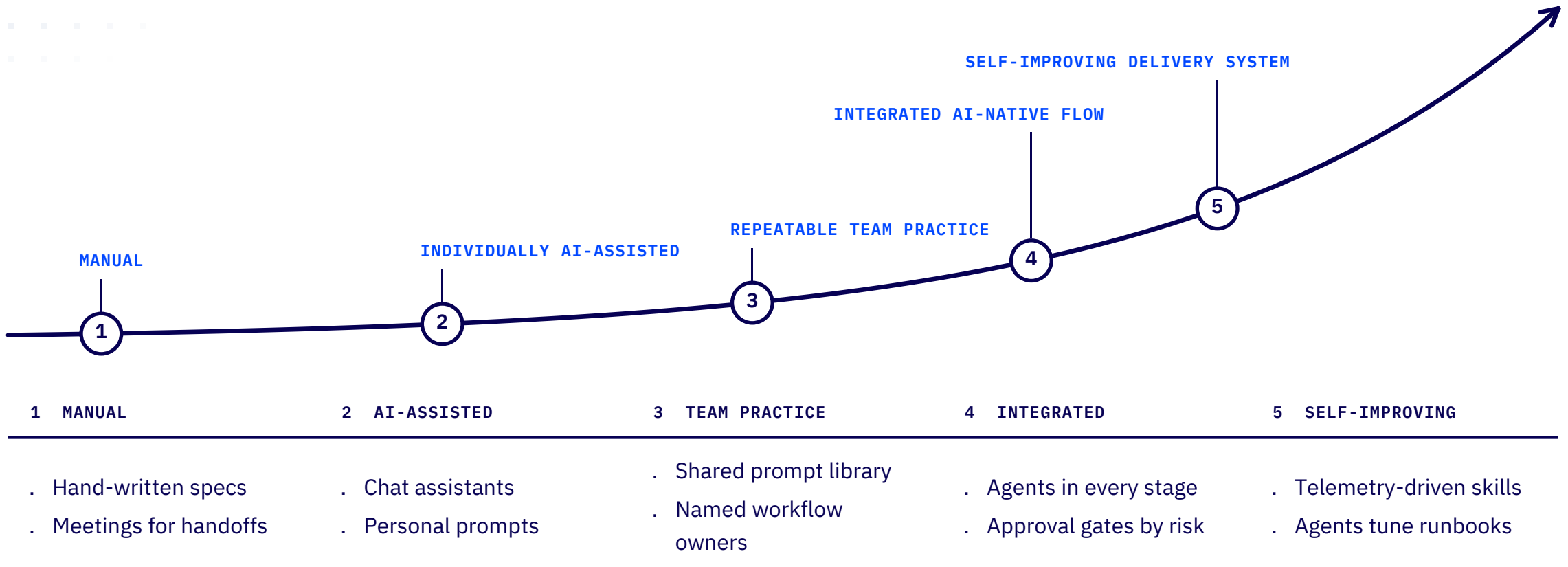
What changes at each stage



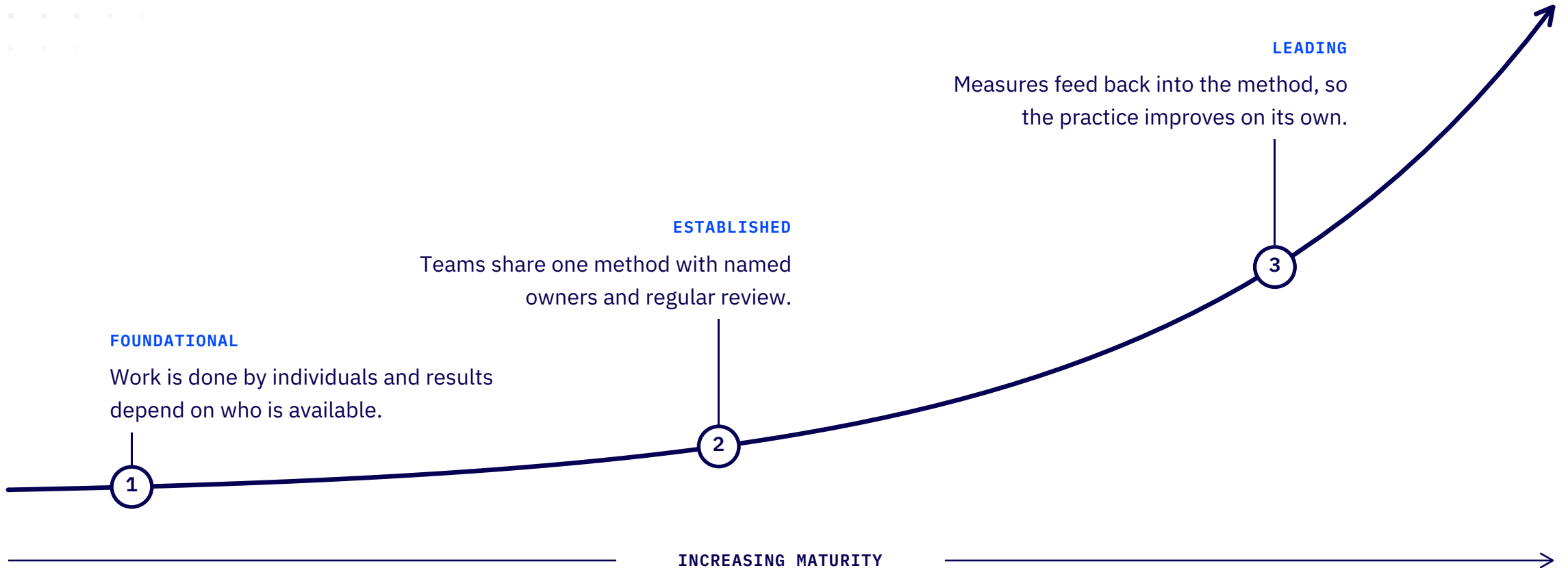
Delivery matures in five stages



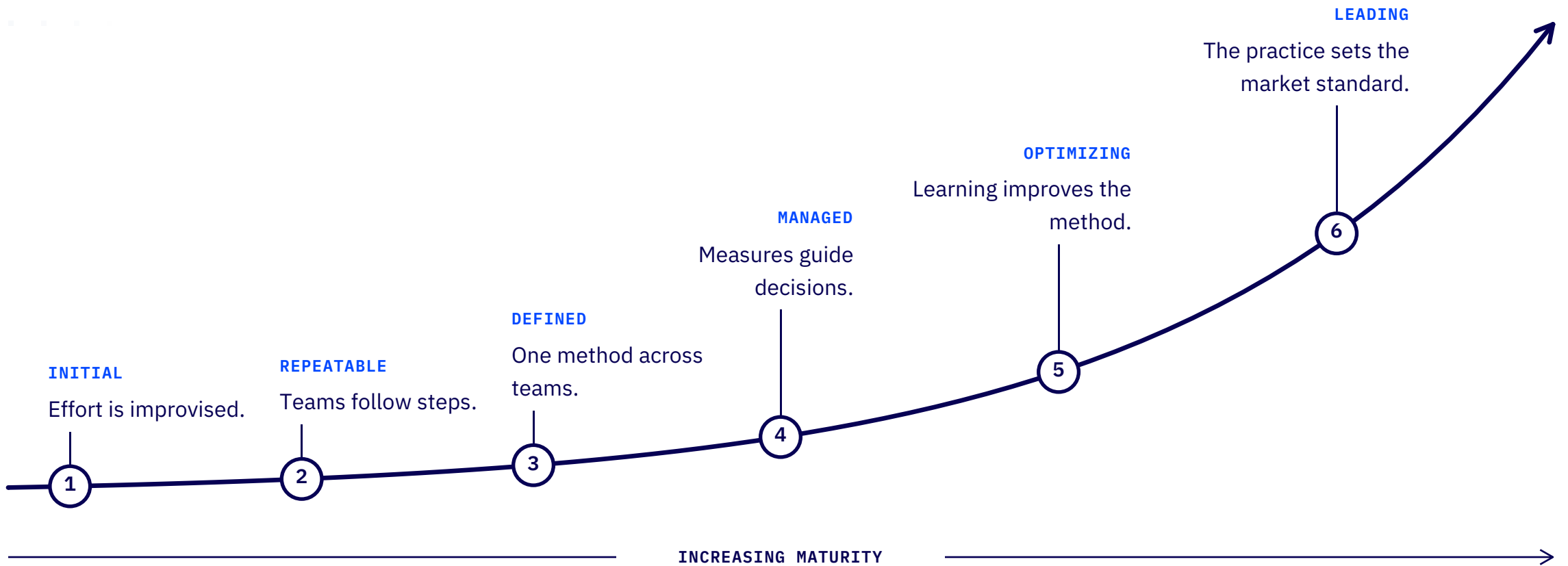
Each stage adds capabilities the last one lacked



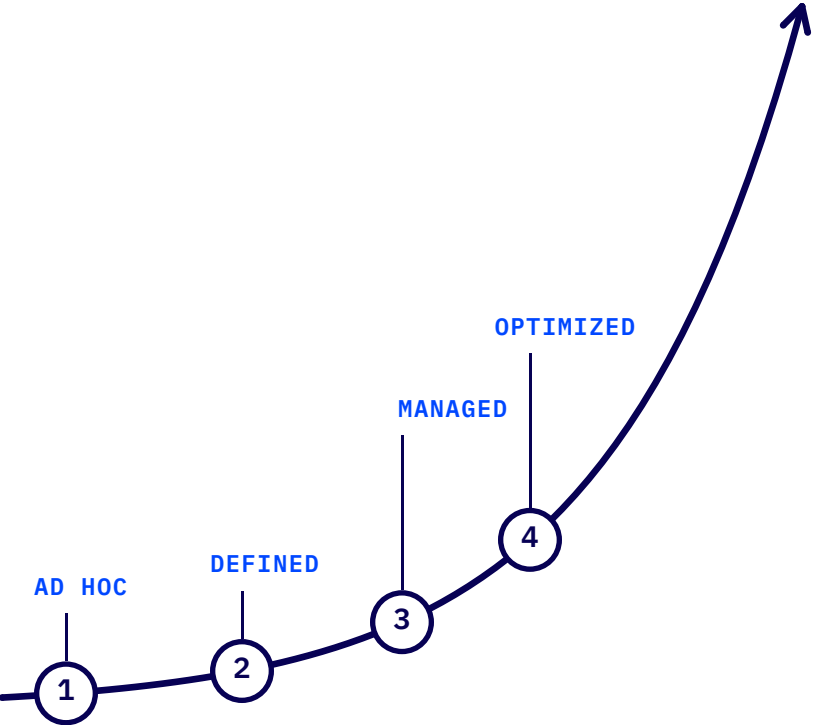
Three stages take a practice from habit to advantage



Six stages describe the full climb

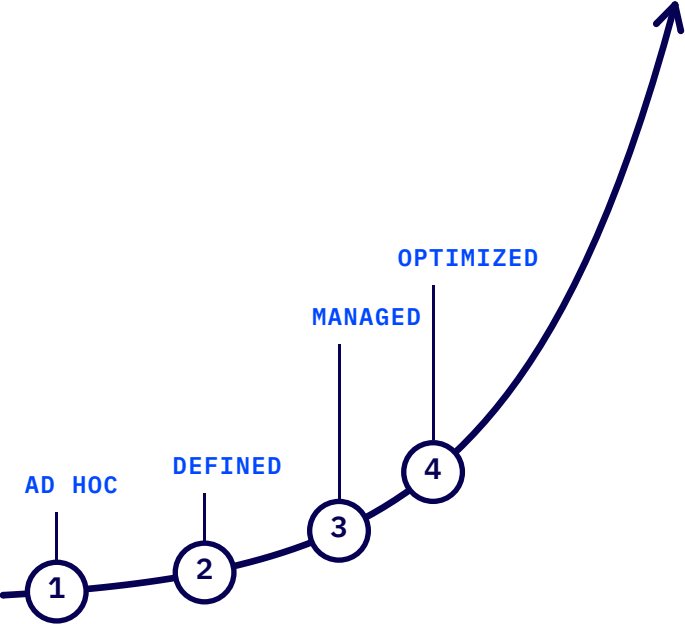


What people, process and tools look like at each stage



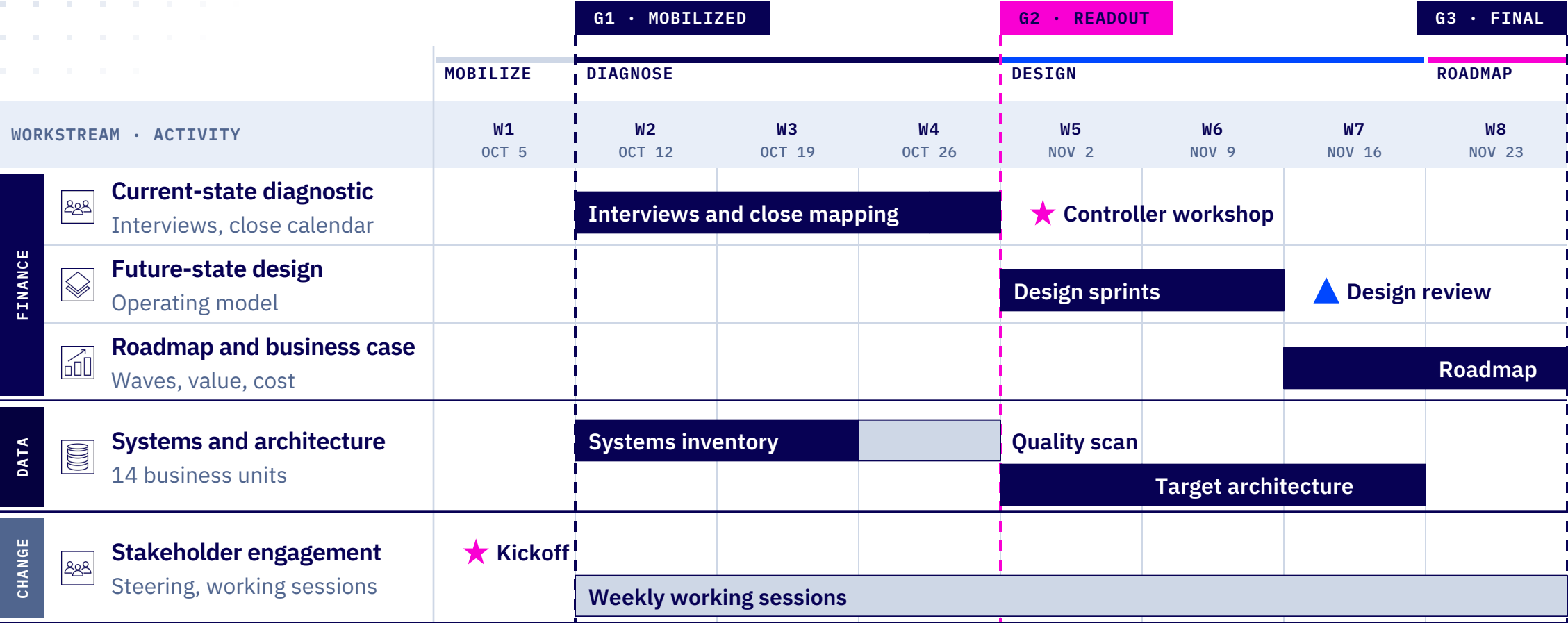
	PEOPLE	PROCESS	TOOLS
Ad hoc	Individual heroics	Improvised	Personal choice
Defined	Trained practitioners	Documented steps	Approved list
Managed	Named owners	Measured results	Integrated stack
Optimized	Communities of practice	Self-correcting	Automated and observed

What people, process and tools look like at each stage



	PEOPLE	PROCESS	TOOLS
Ad hoc	Individual heroics	Improvised, varies by person	Personal choice of tools
Defined	Trained practitioners	Documented, repeatable steps	Approved list of tools
Managed	Named owners per practice	Measured against targets	Integrated, shared stack
Optimized	Communities of practice	Self-correcting through review	Automated and observed

Eight weeks from kickoff to a signed-off roadmap



■ Core activity □ Supporting activity ▨ Hatched: in progress, soft start or tentative ★ Workshop ▲ Review — Gate

APPROACH

Four phases take the close from 12 days to 5

PHASE 1

1 WK

Mobilize

Team, data access and plan in place.

ACTIVITIES

- Kickoff and interviews
- Data access

DELIVERABLES

- Project plan

PHASE 2

3 WKS

Diagnose

Know where the 12 days go.

ACTIVITIES

- Close calendar mapping
- Delay ranking

DELIVERABLES

- Diagnostic readout

PHASE 3

3 WKS

Design

A five-day close, designed.

ACTIVITIES

- Rules and routing
- Operating model

DELIVERABLES

- Target design

PHASE 4

1 WK

Roadmap

Waves 1–3 planned and funded.

ACTIVITIES

- Wave sequencing
- Business case

DELIVERABLES

- \$4.2M business case

G1 • MOBILIZED

G2 • READOUT

G3 • SIGN-OFF

G4 • FINAL

AI sequences each cutover wave and watches coexistence; people hold every go or no-go decision

1 Sequence

Find viable capability slices, expose hidden dependencies and simulate fallback paths.

2 Prepare

Profile and map data; draft test scenarios, runbooks, communications and support.

3 Monitor

Compare transactions and outcomes, flag exceptions and trace anomalies to dependencies.

ILLUSTRATIVE PHASED CUTOVER

	CURRENT STATE	WAVE 1	WAVE 2	WAVE 3
CLOSE AND CONSOLIDATION	Legacy	Coexistence	Modern	Modern
INTERCOMPANY AND PAYABLES	Legacy	Legacy	Coexistence	Modern
PLANNING AND REPORTING	Legacy	Legacy	Legacy	Coexistence

Source: Illustrative. The migration unit is a business-viable capability slice, not a single technical component.

APPROACH

Four phases to a 5-day close

PHASE 1

1 WK

Mobilize

Team, data access and plan in place.

ACTIVITIES

- Kickoff and interviews
- Data access

DELIVERABLES

- Project plan

PHASE 2

3 WKS

Diagnose

Know where the 12 days go.

ACTIVITIES

- Close calendar mapping
- Delay ranking

DELIVERABLES

- Diagnostic readout

PHASE 3

3 WKS

Design

A five-day close, designed.

ACTIVITIES

- Rules and routing
- Operating model

DELIVERABLES

- Target design

PHASE 4

1 WK

Roadmap

Waves 1–3 planned and funded.

ACTIVITIES

- Wave sequencing
- Business case

DELIVERABLES

- \$4.2M business case

G1 • MOBILIZED

G2 • READOUT

G3 • SIGN-OFF

G4 • FINAL

OUR COMMITMENT

8 weeks

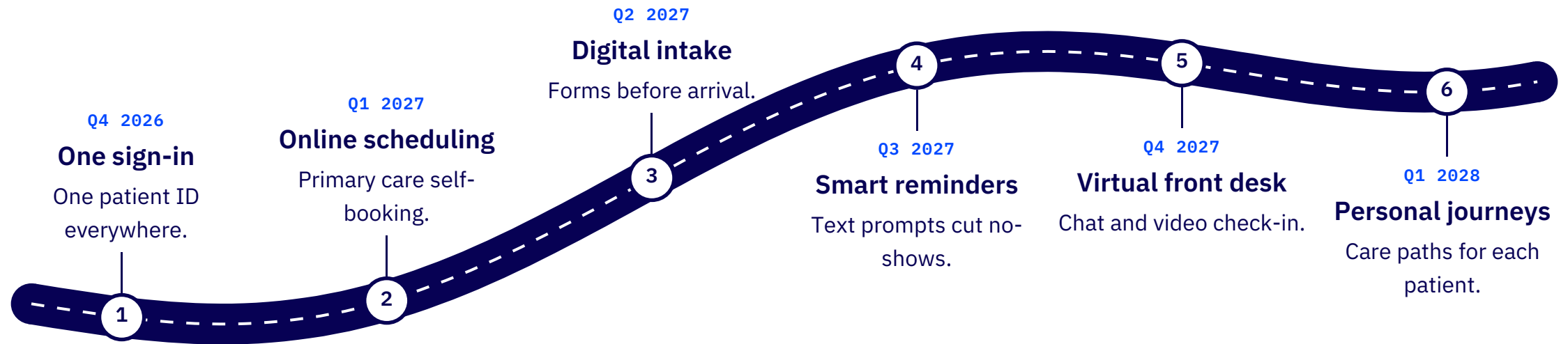
from kickoff to a funded roadmap

WHAT YOU CAN HOLD US TO

A five-day close designed with your team, and a business case your board can approve at Gate 4.

The digital front door ships in six steps over 18 months

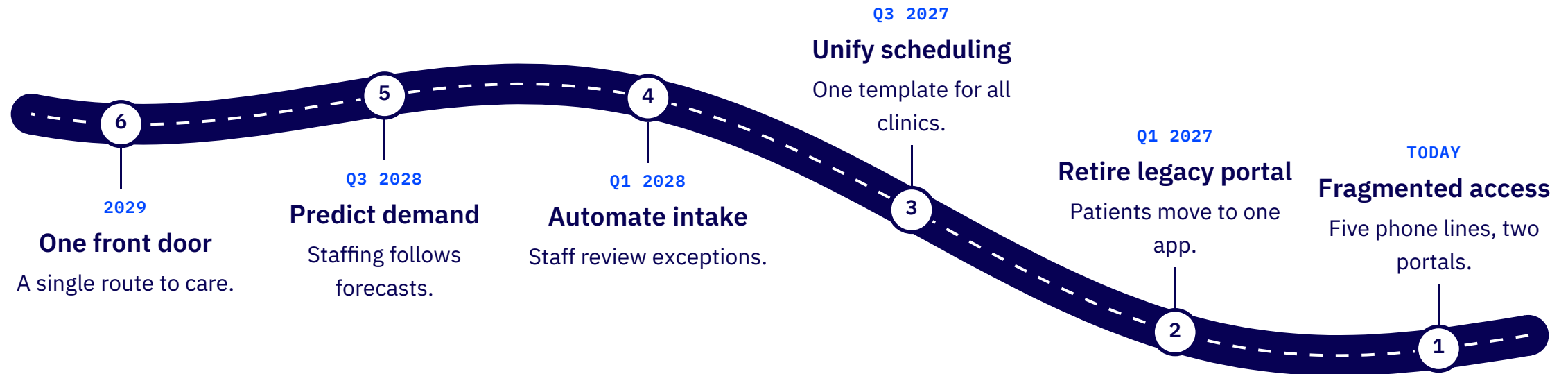
Each release removes a reason for patients to phone the access center.



Source: Northfield Health System digital front door program, Sep 2026.

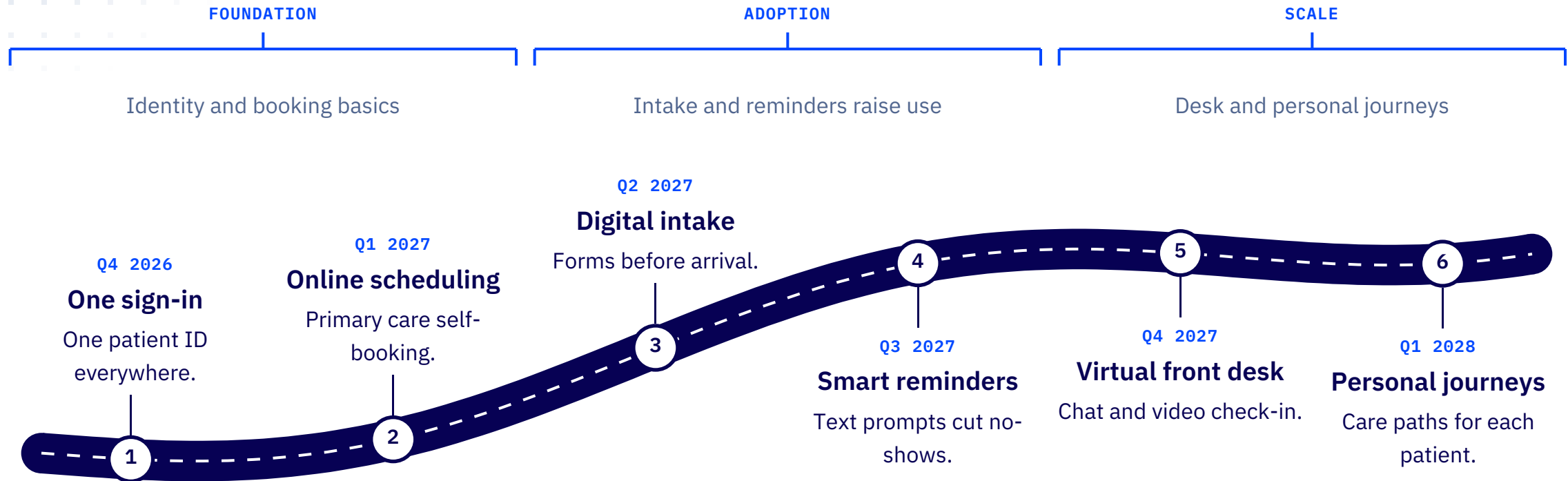
Six checkpoints lead back to one front door

Each checkpoint retires one more phone line, portal or manual step.



Source: Northfield Health System digital front door program, Sep 2026.

Three phases take sign-in to personal journeys



Source: Northfield Health System digital front door program, Sep 2026.

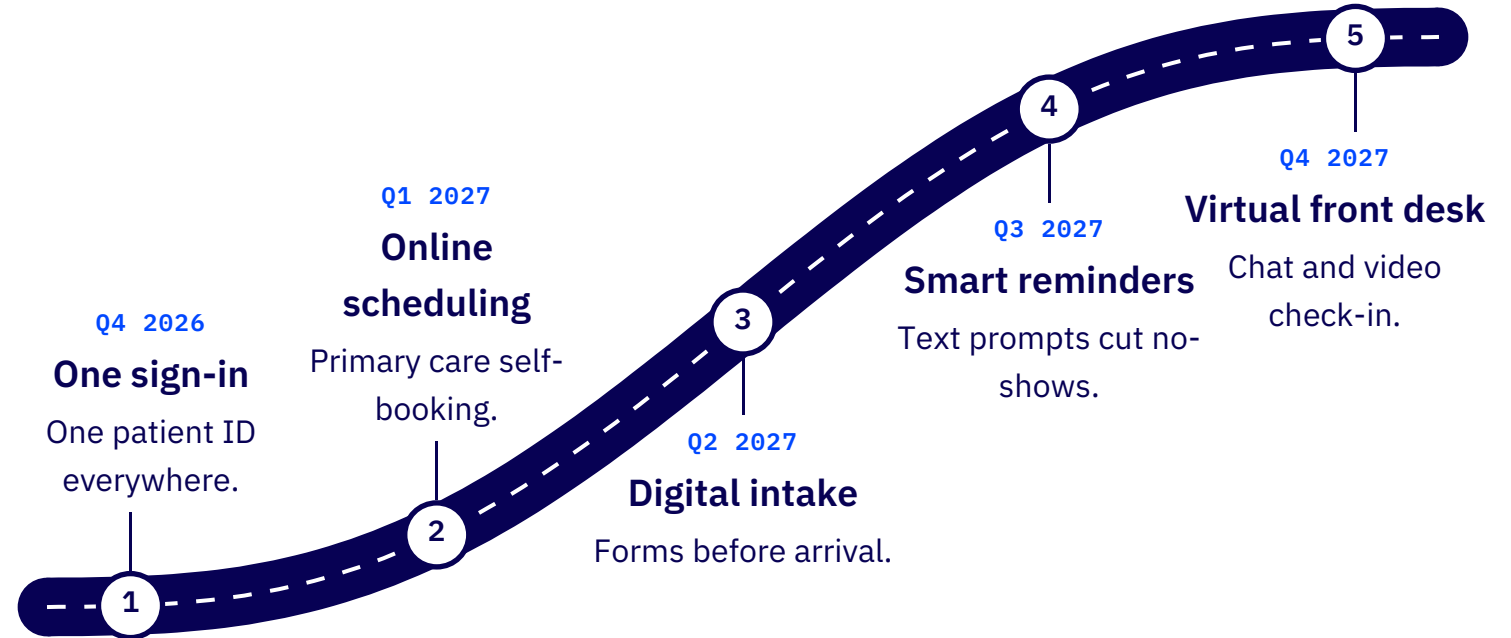
Five releases make the front door real

5

releases in 12 months

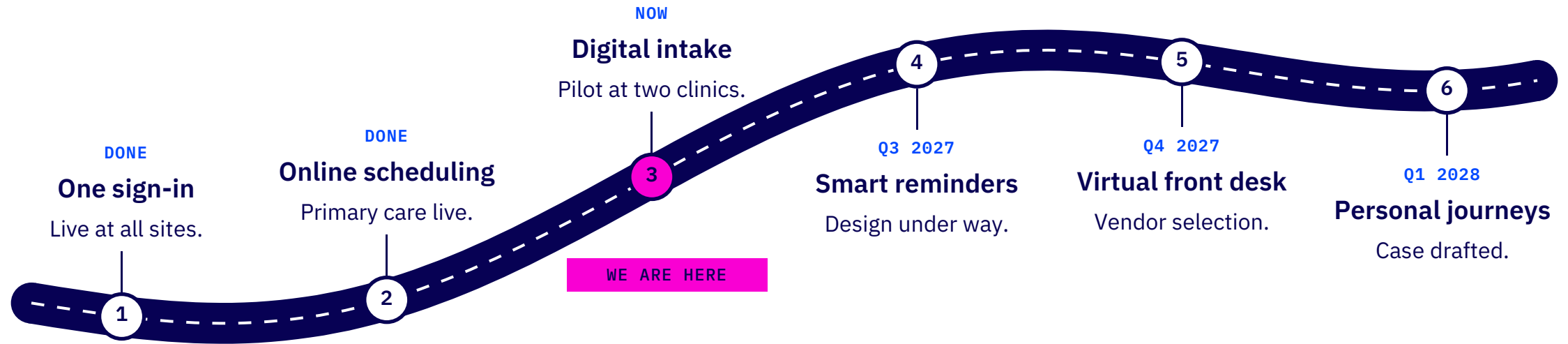
Each release removes a reason for patients to phone the access center.

- **Early.** Sign-in and booking build the habit.
- **Later.** Reminders and chat raise completion.



Source: Northfield digital front door program.

Digital intake is next: we are two milestones in



Source: Northfield Health System digital front door program, Sep 2026.

Three horizons take the patient app from booking to care

Now	Next	Later
NEXT 90 DAYS	FOLLOWING TWO QUARTERS	BEYOND SIX MONTHS
Remove friction from booking	Extend care between visits	Widen access and reach
Self-scheduling for primary care	Care plans in the app	Proxy access for caregivers
Waitlist and cancellations	Remote monitoring pilot	Community programs
Secure messaging triage	Cost estimates before visits	Predictive no-show outreach
Pay online and statements	Clinician console redesign	Multilingual experience

WHY THIS ORDER

Patients give up at the front door before they reach care.

Fix booking first, extend care between visits next, then widen who the app reaches.

RULE OF THUMB

Each horizon names the problem, not the feature list.

PRODUCT ROADMAP

One patient problem per horizon

Now NEXT 90 DAYS

Patients give up on booking before they reach a visit

Measure: bookings completed without a call

Ships: self-scheduling, waitlist, messaging triage

Next TWO QUARTERS

Patients lose the thread between visits

Measure: care-plan tasks completed in the app

Ships: care plans, remote monitoring pilot, cost estimates

Later SIX MONTHS AND BEYOND

Families and caregivers are locked out of the app

Measure: share of patients with a proxy or language preference

Ships: proxy access, multilingual booking

Horizons are planning commitments, not delivery dates.

The roadmap in one view

Now	Next	Later
Self-scheduling	Care plans	Caregiver proxy
Waitlist	Remote monitoring	Multilingual booking
Messaging triage	Cost estimates	No-show outreach
Pay online	Pre-visit forms	Kiosk check-in
Payment plans	Console redesign	Data re-platform

IN BRIEF

Front door first, then care.

NOW

Remove friction from booking and billing.

NEXT

Extend care between visits.

LATER

Widen access to caregivers and communities.

Releases land lane by lane across three horizons

PRODUCT AREA	Now · 90 days	Next · two quarters	Later · six months+
Access	Self-scheduling	Waitlist and cancellations	Multilingual booking
Care	Messaging triage	Care plans in the app Remote monitoring pilot	Caregiver proxy access
Billing	Pay online Statements and plans	Cost estimates	Insurance card capture
Platform	EHR hardening	Event stream Observability	Data store re-platform

Dark bars are committed, mid-blue planned, outlined exploratory. Magenta marks the launch commitment.

Confidence falls as the horizon lengthens

Now

Committed: funded and staffed

RELEASE	CONFIDENCE
Self-scheduling	● ● ● ●
Waitlist	● ● ● ●
Messaging triage	● ● ● ●
Pay online	● ● ● ●

Next

Probable: scoped, not yet funded

RELEASE	CONFIDENCE
Care plans	● ● ● ●
Remote monitoring	● ● ● ●
Cost estimates	● ● ● ●
Console redesign	● ● ● ●

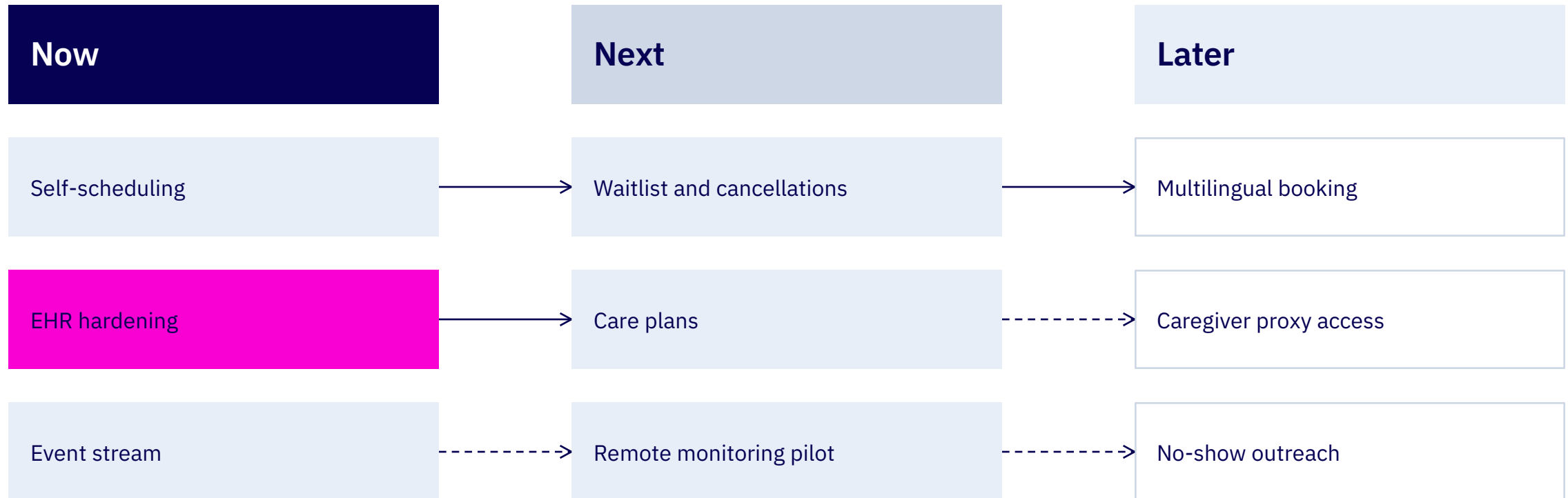
Later

Exploratory: needs discovery

RELEASE	CONFIDENCE
Caregiver proxy	● ● ● ●
Multilingual booking	● ● ● ●
No-show outreach	● ● ● ●
Data re-platform	● ● ● ●

Each Next and Later release is re-scored at the quarterly product council.

EHR hardening gates the later horizons



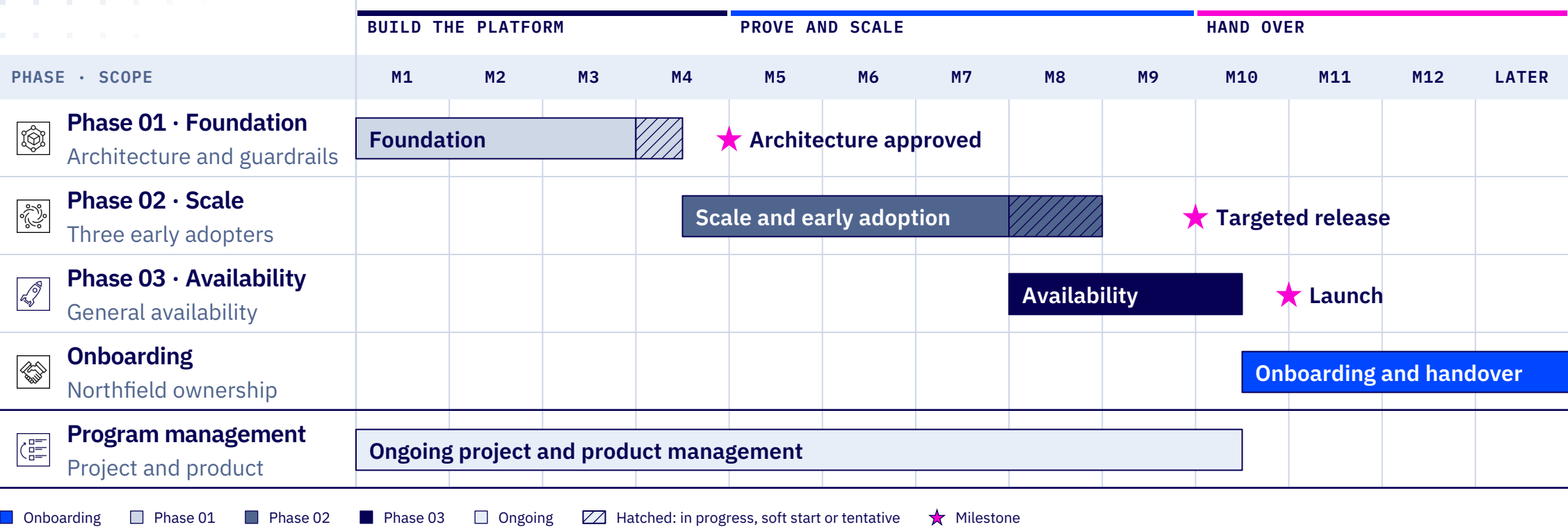
READING THE ARROWS

Solid arrows are hard dependencies; dashed are soft.

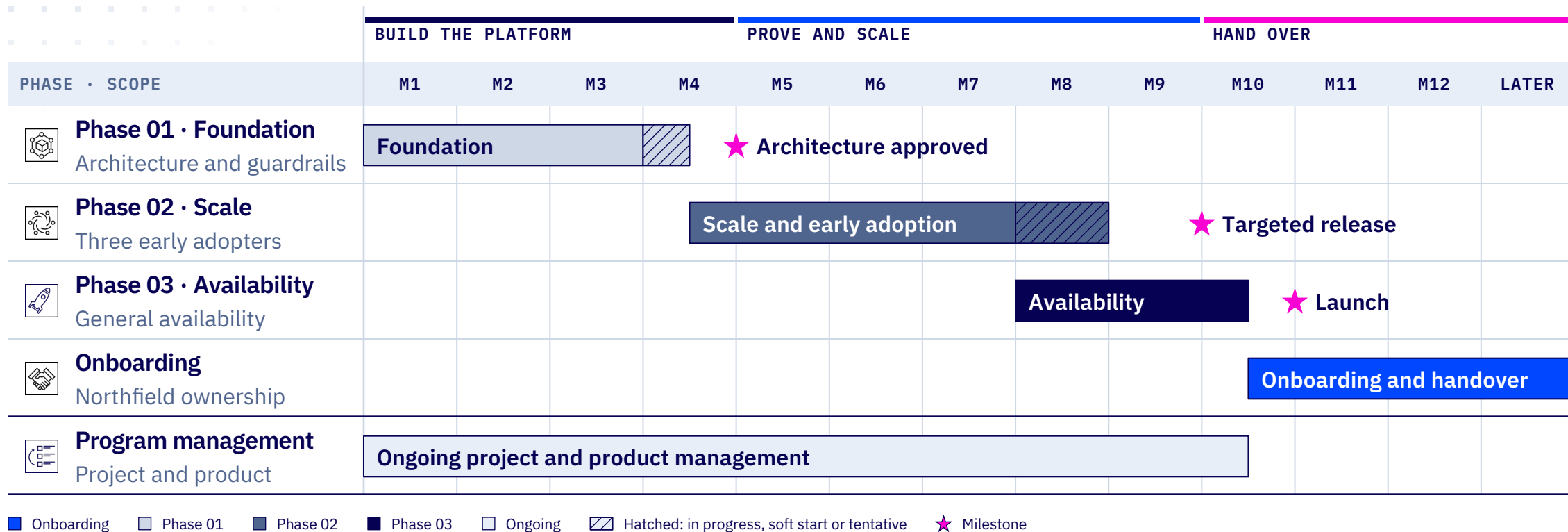
CRITICAL PATH

EHR hardening gates care plans, and a slip there moves caregiver access with it.

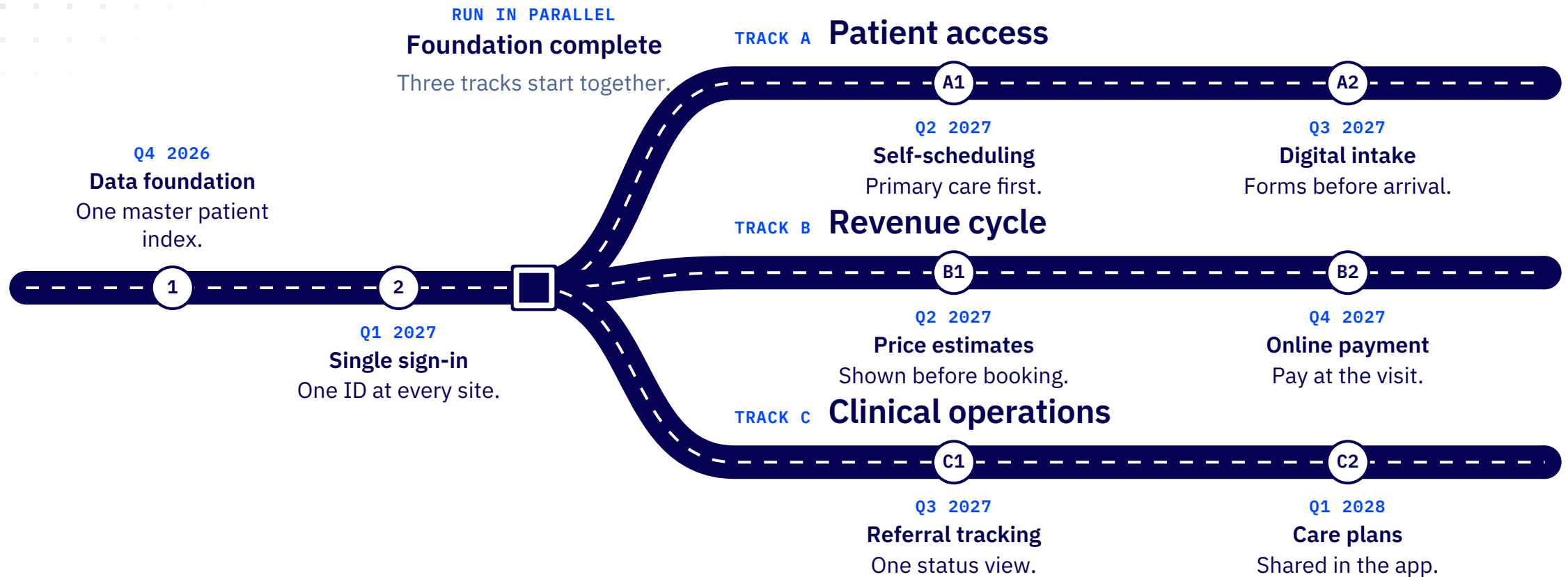
Nine months from build to Northfield ownership



Nine months from build to Northfield ownership

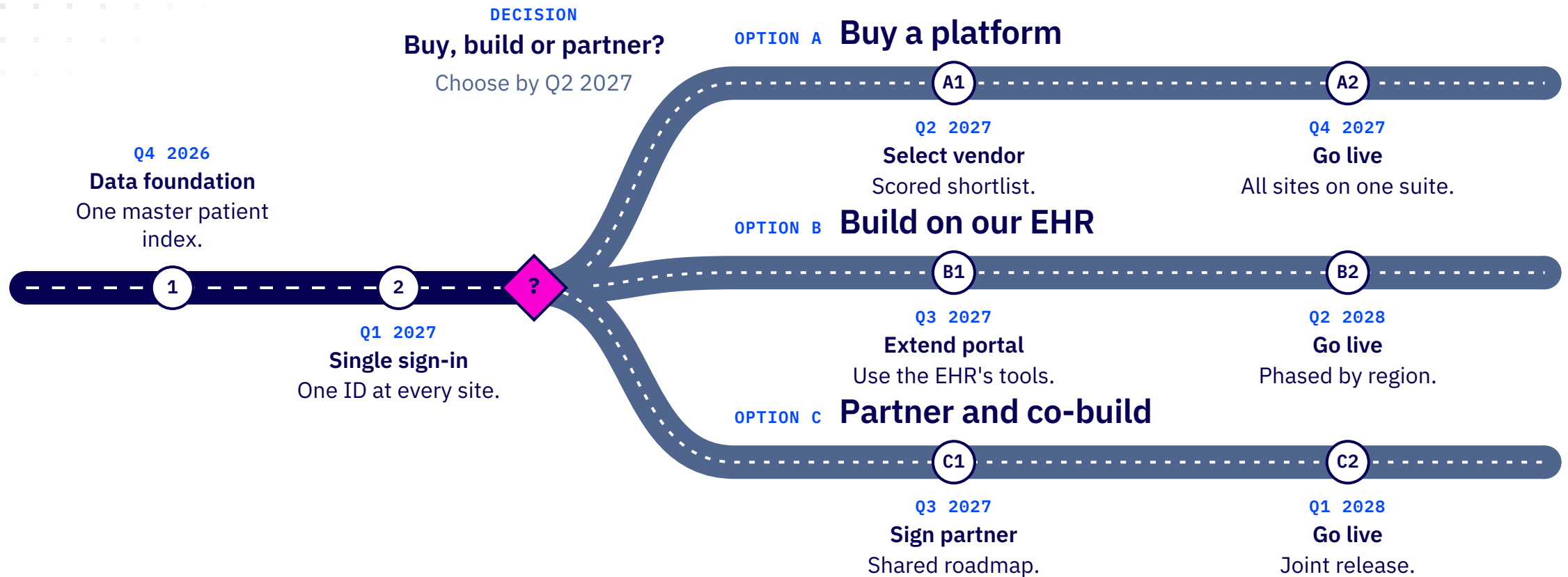


One foundation, then three tracks run in parallel



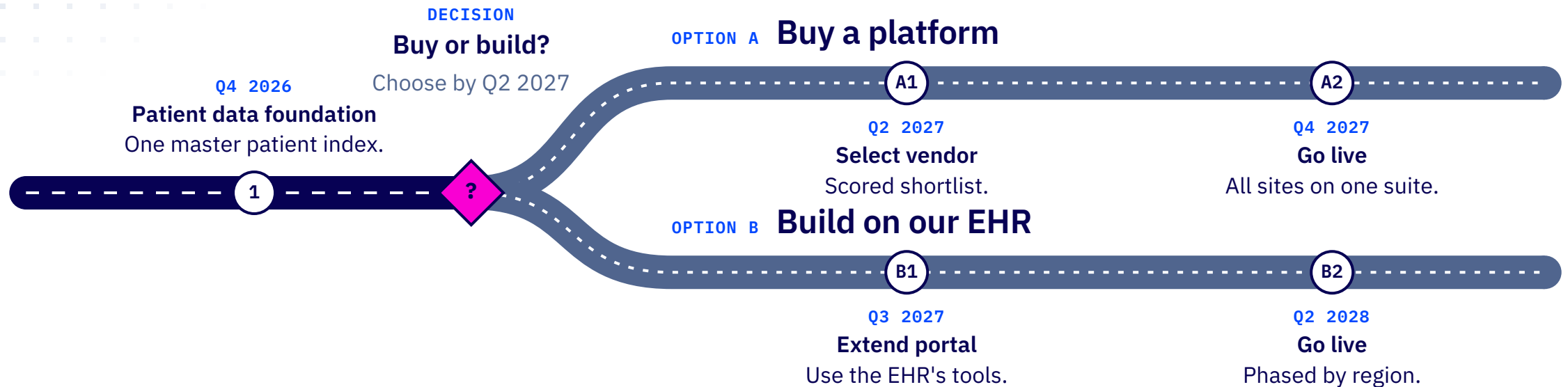
Source: Northfield Health System digital front door program.

You must choose: buy, build or partner



Source: Northfield Health System digital front door program.

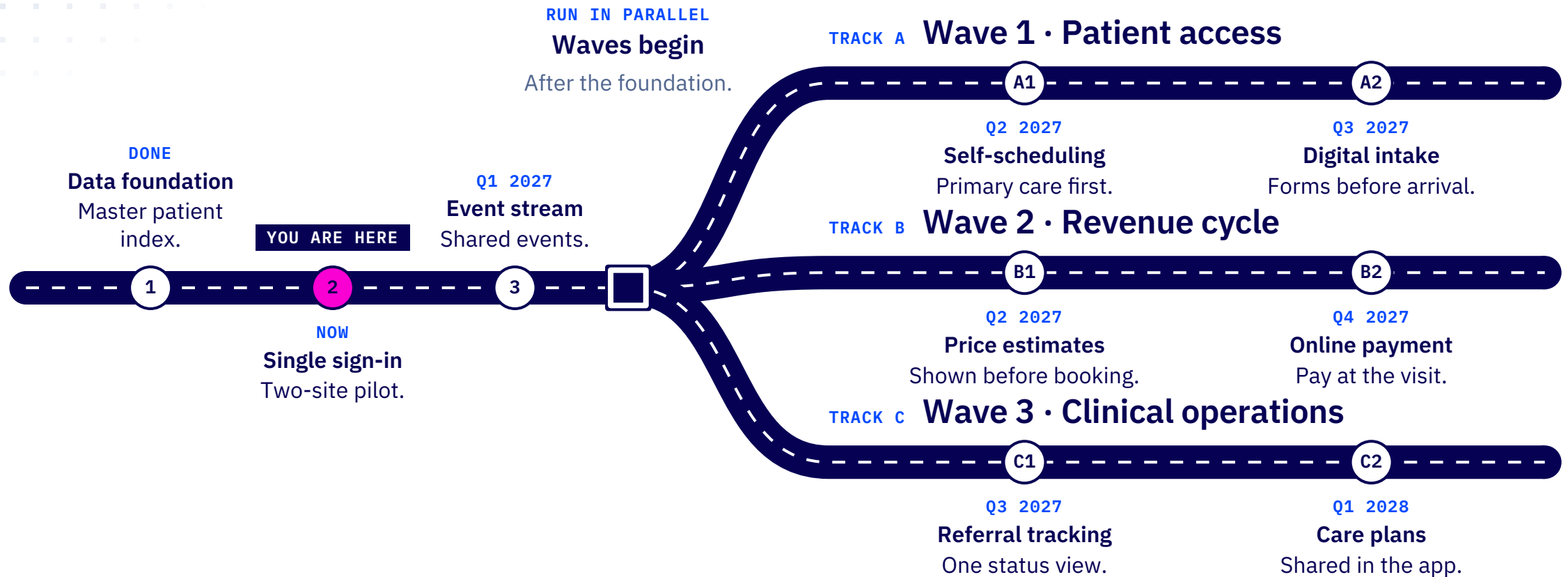
The choice turns on time and fit, not cost



CRITERION	A BUY A PLATFORM	B BUILD ON OUR EHR
Cost to Northfield	Medium	Strong
Time to every site	Strong	Weak
Delivery risk	Strong	Medium
Fit with clinical workflow	Medium	Strong

Source: Northfield Health System digital front door program.

The foundation is half built; the waves follow



Source: Northfield Health System digital front door program.

Three objectives, six initiatives, one owner each

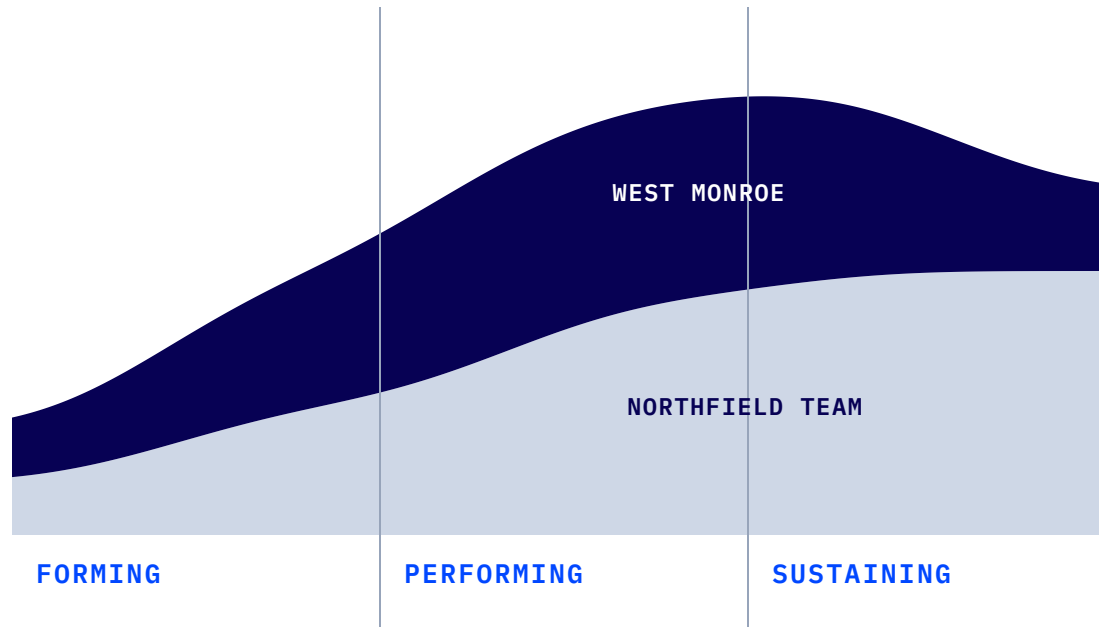
	INITIATIVE	WHAT IT DELIVERS	OWNER	START	LIVE	HORIZON
KNOW PATIENT	Master patient index	One record per patient across six hospitals.	Chief data officer	Q4 26	Q1 27	NOW
	Single sign-in	One patient and staff identity everywhere.	Chief digital officer	Q4 26	Q2 27	NOW
OPEN THE DOOR	Online scheduling	Primary care self-booking, then specialty.	Chief patient access officer	Q1 27	Q3 27	NEXT
	Digital intake	Forms, insurance and consent before arrival.	Director of registration	Q2 27	Q4 27	NEXT
PAY AND TRACK	Price estimates	A reliable price shown before the patient books.	VP revenue cycle	Q3 27	Q1 28	LATER
	Referral tracking	One status view from order to appointment.	Chief nursing informatics officer	Q4 27	Q2 28	LATER

Source: Northfield Health System digital front door program.

Agents let a **smaller team** run the same platform

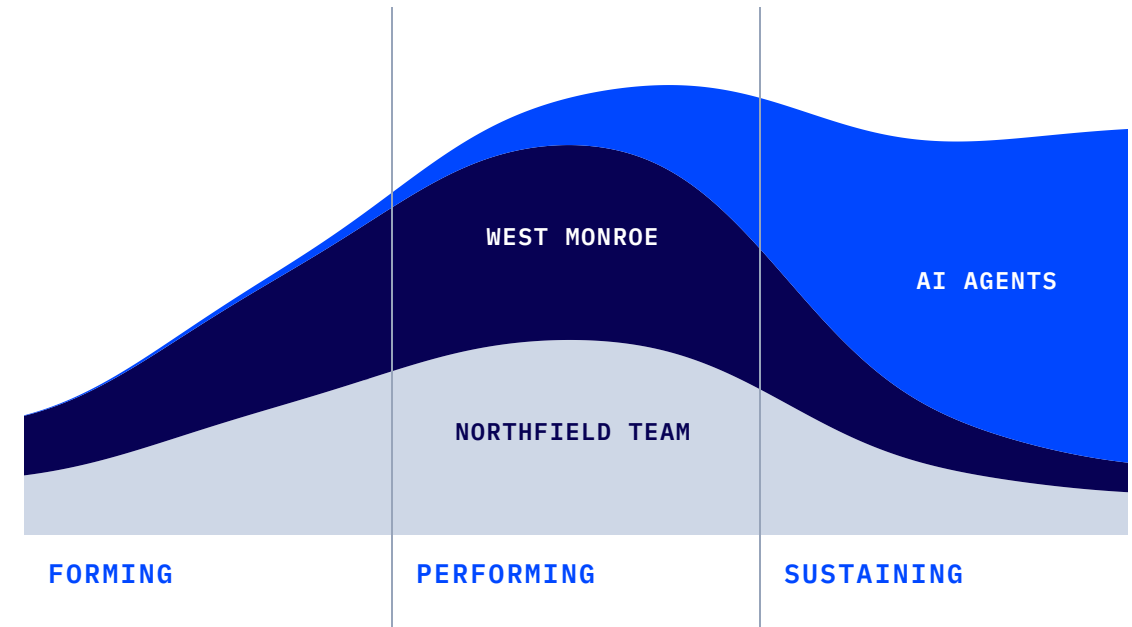
Traditional delivery

Humans only



Agent-enabled delivery

Humans plus agents



Same platform, same outcomes: agents take over maintenance, so both teams run lighter.

OUR APPROACH

Agents cut our team and Northfield's maintenance

NORTHFIELD TEAM

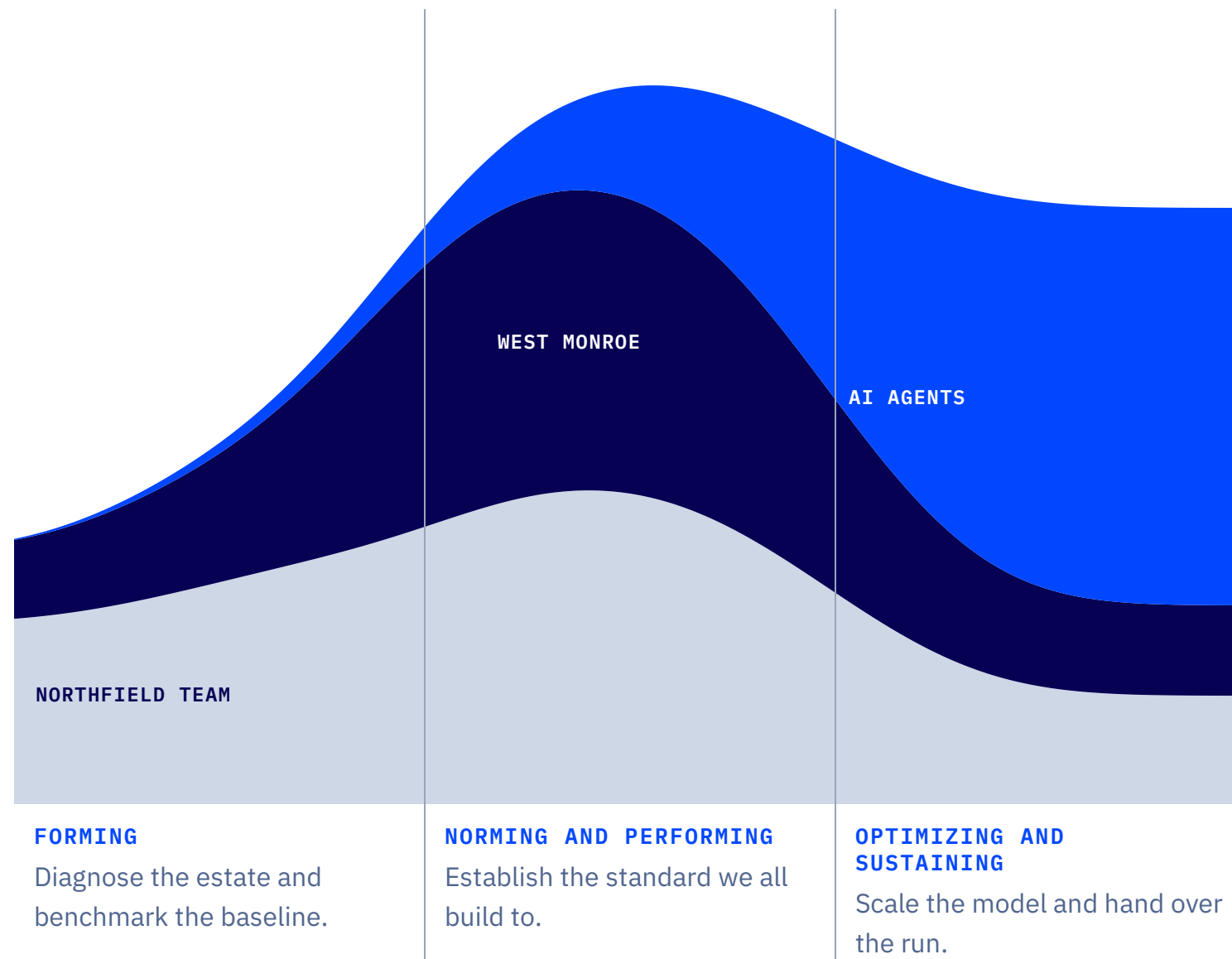
Domain experts who own the platform as we step back.

WEST MONROE

We set the engineering standard, then step back.

AI AGENTS

Agents run routine maintenance; humans decide exceptions.



Front-line sessions close out the plan in late October

STAKEHOLDER	TARGET	PURPOSE
Registration leads (3) Patient access	Oct 24	Front-desk workflow walk
Coding specialists (4) HIM	Oct 27	Query volume and gaps
Denials analysts (3) Revenue cycle	Oct 28	Appeal tooling and rework
Service desk lead IT	Oct 29	Incident trends by system
Unit clerks (3) Clinical ops	Oct 30	Charge capture steps
Billing leads (2) Finance	Oct 30	Statement exceptions

Wave 1 • Executives OCT 7 TO 16
Five sessions on priorities and funding.

Wave 2 • Leaders OCT 17 TO 23
Five sessions on process and tooling.

Wave 3 • Front line OCT 24 TO 30
Fifteen people in six group sessions.

Readout NOV 3
Findings to the steering committee.

STILL TO SCHEDULE

10

of 15 interviews have no date yet.

Source: Sample names for layout only; replace with engagement stakeholders before use.

If this happens, do this: contingencies for workshop 2

SEVERITY	IF THIS HAPPENS	EARLY SIGN	DO THIS	OWNER
P1	Decision-makers missing	Sponsor or area lead absent at 9:15	Mark decisions as recommendations; log an owner and a date; confirm by email within 24 hours.	Lead
P1	Video fails for remote table	No audio or picture for two minutes	Producer moves them to the phone bridge and shares a photo of the canvas; the scribe pastes notes live.	Producer
P2	Running long	Ten minutes behind at the break	Cut in this order: 1 playback to three minutes a table; 2 recap dot vote; 3 gap discussion, moved to follow-up. Never cut decisions.	Lead
P2	Dominant voice	One person holds over 40% of airtime	Round-robin on index cards; “Let us hear from tables not yet in”; side-chat at the break.	Area lead
P3	Off-topic deep dive	Five minutes on one system	Park it with an owner, offer a follow-up session, and return to the canvas.	Area lead
P3	Low energy	Quiet tables after the break	Stand-up pair share, switch to sticky notes, shorten the next timebox.	Co-lead

Two days to design how patients get access

OVERALL GOAL

Agree, test and commit to one future-state design for how patients schedule, register, get estimates and avoid denials.

DAY 1 • TUE NOV 4 • 9:00 TO 17:00

Discover the current state

- 9:00** Welcome, goals and ground rules
- 9:45** Current-state walk-through
- 11:15** Patient journey mapping
- 13:00** Pain points and root causes
- 16:30** Day 1 playback

OUTPUT BY END OF DAY

A validated current-state map and the top ten pain points, ranked by the room.

DAY 2 • WED NOV 5 • 9:00 TO 16:30

Design the future state

- 9:00** Recap and design principles
- 9:45** Future-state design by area
- 13:00** Cross-area hand-offs
- 14:45** Decisions and owners
- 16:00** Readout to sponsors

OUTPUT BY END OF DAY

A future-state design for each of the four areas, with a named owner and a decisions log.

Every cutover step has exactly one accountable role

CUTOVER STEP	CUTOVER LEAD	LEDGER OWNER	TECHNICAL LEAD	INTEGRATION LEAD	DATA LEAD	SUPPORT LEAD
Freeze legacy postings	A	C	R	I	I	I
Extract and reconcile balances	I	A	C	I	R	I
Load balances to the new ledger	I	A	R	I	C	I
Switch interfaces to the new ledger	I	I	C	A R	C	I
Hold the go or no-go decision	A R	C	C	C	C	I
Notify users and open hypercare	A	I	I	I	I	R

R Responsible
 A Accountable
 C Consulted
 I Informed

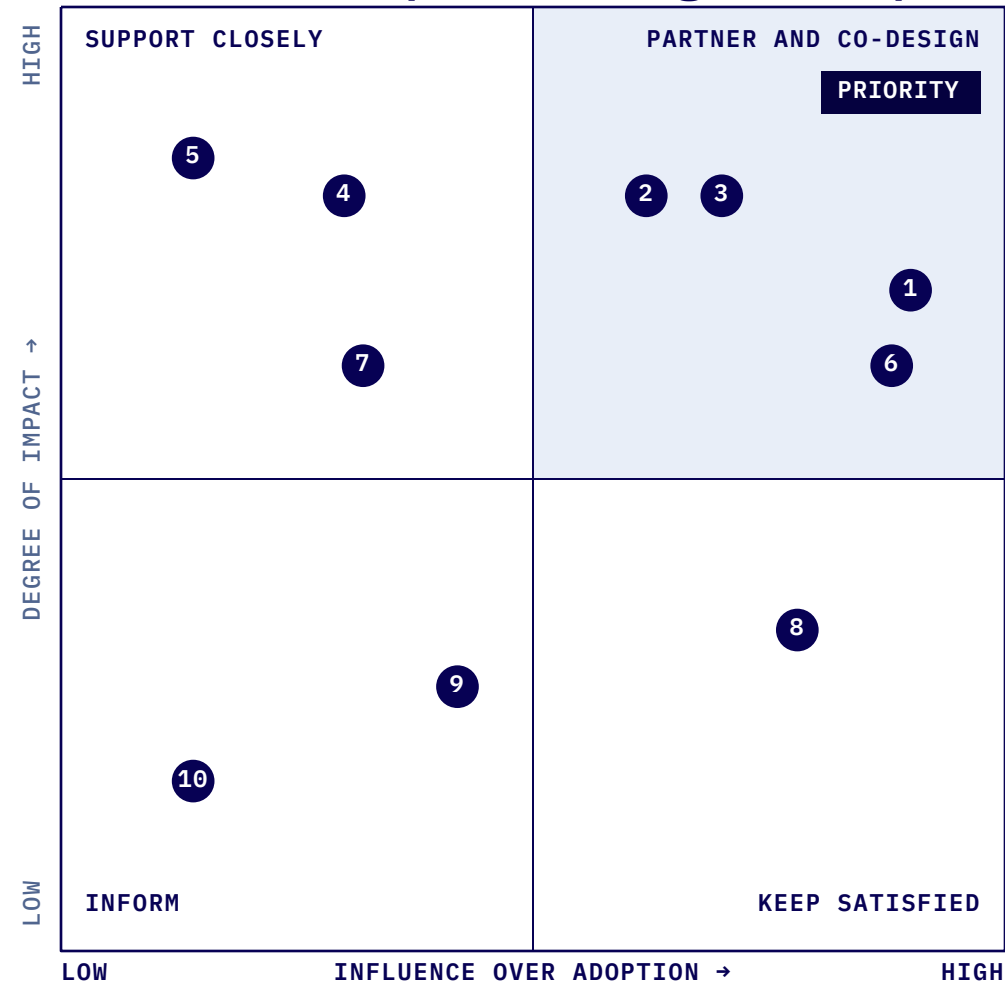
Authority: only the accountable role signs a step off. The cutover lead's go or no-go call overrides any other sign-off.

Engage the top-right quadrant first

- 1 Chief medical officer
- 2 Clinic nurse leads
- 3 Practice managers
- 4 Call-center agents
- 5 Front-desk schedulers
- 6 Physician group
- 7 IT service desk
- 8 Union representatives
- 9 Patient advisory council
- 10 Payer relations

Partner with 1, 2, 3 and 6; support 4, 5 and 7; inform 8 to 10 monthly.

Influence over adoption and degree of impact



Source: Northfield stakeholder interviews, Sep 2026.

Three ways to engage, one we recommend

OPTION A • FIXED FEE

Diagnose only

- ☒ Close diagnostic
- ☐ Target design and roadmap
- ☐ Wave 1 build

DURATION

4 weeks

\$288K

fixed fee

RECOMMENDED

OPTION B • HYBRID

Diagnose and build

- ☒ Close diagnostic
- ☒ Target design and roadmap
- ☒ Wave 1 build

DURATION

8 weeks fixed, then 16 weeks capacity

\$720K + \$210K/mo

fixed, then run-rate

OPTION C • CAPACITY

Embedded pod

- ☒ Close diagnostic
- ☒ Target design and roadmap
- ☒ Wave 1 build

DURATION

Rolling, 3-month minimum

\$240K/mo

pod run-rate

A fixed fee of \$720K, paid as milestones are accepted

MILESTONE	TIMING	% OF FEE	AMOUNT	CUMULATIVE
Mobilization complete	Week 1	10%	\$72K	\$72K
Diagnostic readout	Week 4	30%	\$216K	\$288K
Design sign-off	Week 7	40%	\$288K	\$576K
Final readout accepted	Week 8	20%	\$144K	\$720K
Total fixed fee		100%	\$720K	

FIXED FEE

\$720K

total fixed fee

Diagnose · weeks 1–4 **\$288K**Design and roadmap · weeks 5–8 **\$432K**

TERMS

- Invoiced on milestone acceptance
- Expenses capped at 8% of fees

Source: West Monroe estimate. Fees exclude expenses.

One team, one monthly run-rate of \$210K

CAPACITY

\$840K

estimated, four months

Team run-rate \$210K / mo

Duration 4 months

TERMS

- Invoiced monthly in arrears
- 30 days' notice to change the team
- Expenses capped at 8% of fees

ROLE	LEVEL	ALLOCATION
Engagement lead	Managing Director	25%
Process lead	Senior Manager	100%
Data architect	Manager	100%
Accountants	Consultant × 2	100%
Team monthly run-rate		\$210K

COMMERCIALS

One team, one monthly run-rate of \$210K

CAPACITY

\$840K

estimated, four months

Team run-rate \$210K / mo

Duration 4 months

TERMS

- Invoiced monthly in arrears
- 30 days' notice to change the team
- Expenses capped at 8% of fees

ROLE	LEVEL	ALLOCATION
Engagement lead	Managing Director	25%
Process lead	Senior Manager	100%
Data architect	Manager	100%
Integration lead	Manager	50%
Accountants	Consultant × 2	100%
Analysts	Analyst × 2	100%
Change lead	Senior Consultant	50%
Team monthly run-rate		\$210K

COMMERCIALS

Three ways to engage, one we recommend

OPTION A • FIXED FEE

Diagnose only

- ☒ Close diagnostic
- ☐ Target design and roadmap
- ☐ Wave 1 build

DURATION

4 weeks

\$288K

fixed fee

RECOMMENDED

OPTION B • HYBRID

Diagnose and build

- ☒ Close diagnostic
- ☒ Target design and roadmap
- ☒ Wave 1 build

DURATION

8 weeks fixed, then 16 weeks
capacity

\$720K + \$210K/mo

fixed, then run-rate

OPTION C • CAPACITY

Embedded pod

- ☒ Close diagnostic
- ☒ Target design and roadmap
- ☒ Wave 1 build

DURATION

Rolling, 3-month minimum

\$240K/mo

pod run-rate

Three tiers, each building on the one before

GOOD • FIXED FEE

Foundation

- ☒ Close diagnostic
- ☐ Target design
- ☐ Roadmap and business case
- ☐ Wave 1 build
- ☐ Training and hypercare

\$288K

fixed fee, 4 weeks

RECOMMENDED

BETTER • FIXED FEE

Accelerate

- ☒ Close diagnostic
- ☒ Target design
- ☒ Roadmap and business case
- ☒ Wave 1 build
- ☐ Training and hypercare

\$720K

fixed fee, 24 weeks

BEST • FIXED FEE

Transform

- ☒ Close diagnostic
- ☒ Target design
- ☒ Roadmap and business case
- ☒ Wave 1 build
- ☒ Training and hypercare

\$1.14M

fixed fee, 36 weeks

IN BOTH OPTIONS

Program management and weekly steering

Knowledge transfer to Northfield finance

A fixed-fee diagnostic and design

NOT IN EITHER

Licenses and infrastructure

Northfield staff time and expenses

COMMERCIALS

Two options for Northfield, one recommended

OPTION A

Design only

Prove the case, then decide on build.

DURATION

8 weeks

\$720K

fixed fee

OPTION B

Design and build

Carry one team through to Wave 1.

DURATION

8 weeks fixed, then 16 weeks capacity

\$720K + \$210K/mo

fixed, then run-rate

RECOMMENDED

A fixed base fee with four optional add-ons

BASE FEE

\$720K

fixed fee, 14 weeks

Diagnose and design

\$504K

Roadmap and handover

\$216K

TERMS

- Invoiced on phase acceptance
- Expenses capped at 8% of fees

OPTIONAL ADD-ONS, EACH A FIXED FEE

ADD-ON	WHAT IT ADDS	FEE
Wave 2 entity rollout	Five more entities	\$144K
Training academy	Role-based courses	\$96K
Hypercare, 90 days	Post-go-live support	\$72K
Benefits tracking	Quarterly value review	\$48K

Add-ons can be elected until design sign-off at the prices shown.

Five levers add up to \$2.6M of annual value

VALUE DRIVER	LEVER	METRIC, BASELINE TO TARGET	ANNUAL VALUE
Productivity	Close and reporting	Hours per close: 4,200 to 2,400	\$1.0M
	Rework and errors	Corrections a month: 310 to 120	\$0.3M
Revenue	Revenue cycle	Denial rate: 9.1% to 6.4%	\$0.8M
Cost	Procure-to-pay	Spend under contract: 54% to 72%	\$0.4M
Risk	Risk avoided	Open audit findings: 14 to 4	\$0.1M
Five levers, annual value at run-rate			\$2.6M a year

Source: Sample figures for layout only; replace with current program data before use.

From wave two, new value funds the next wave

WAVE 1 • MONTHS 1 TO 12

Foundation

Data platform, close automation in three entities

Investment \$2.4M



Value in the wave \$0.6M



NET IN THE WAVE

-\$1.8M

Budget-funded

WAVE 2 • MONTHS 13 TO 24

Scale

Revenue cycle and procure-to-pay across all entities

Investment \$1.3M



Value in the wave \$1.8M



NET IN THE WAVE

+\$0.5M

Self-funding

WAVE 3 • MONTHS 25 TO 36

Optimize

Controls, analytics and run handover

Investment \$0.5M



Value in the wave \$2.6M



NET IN THE WAVE

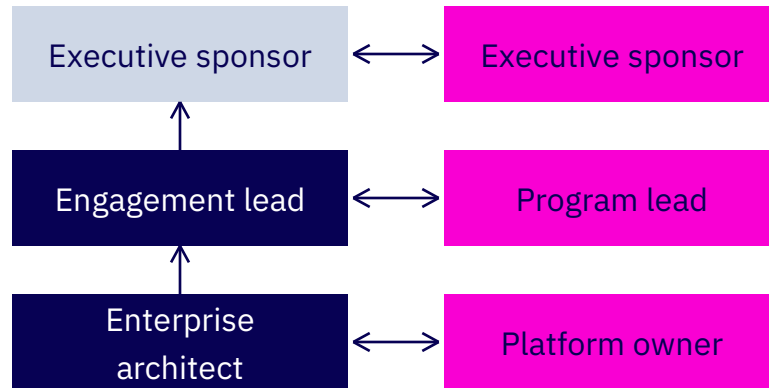
+\$2.1M

Self-funding

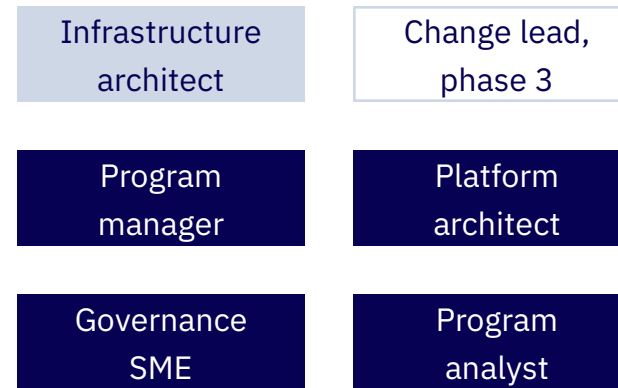
Source: Sample figures for layout only; replace with current program data before use.

Delivery pods scale up and down by workstream

LEADERSHIP PAIRS



WORKSTREAM LEADS



Northfield leadership roles

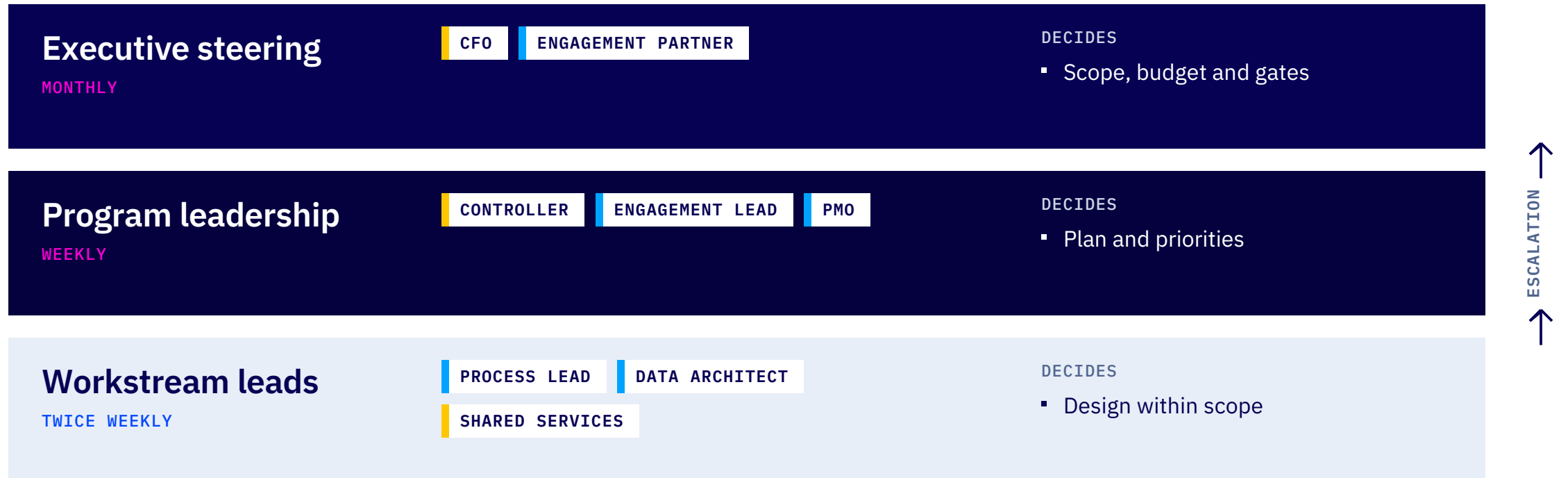
- **Executive sponsor (5%).** Sets direction, removes blockers, approves major decisions.
- **Program lead (50–75%).** Planning, coordination, dependencies, approvals and RAID.
- **Platform owner (50–75%).** Priorities, backlog, acceptance and operating-model calls.

DELIVERY PODS



- West Monroe full time
- West Monroe part time
- Northfield part time

Decisions stay close to the work, with a clear path up



■ West Monroe ■ Northfield



Jordan Ellis

Managing Director

CPA · Health care

ENGAGEMENT LEAD

Jordan Ellis leads the engagement

ROLE

Jordan sets direction, chairs the steering committee and is accountable for the five-day close.

BACKGROUND

Thirty years in health-system finance, most of them spent redesigning the close.

SELECTED EXPERIENCE

- 30+ close transformations
- 14-entity finance redesign
- Former health-system CFO

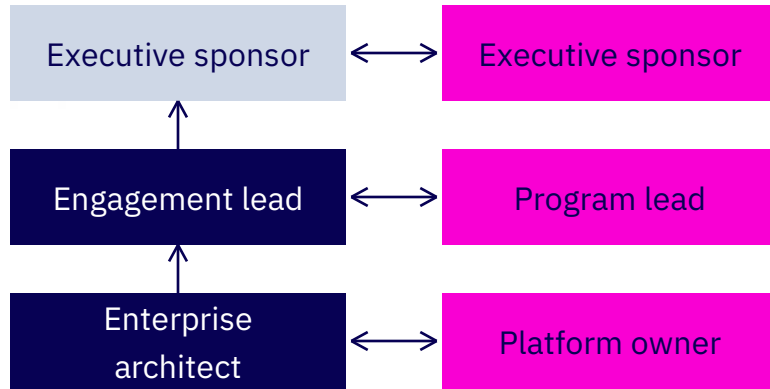
“

Our close became a calendar, not a fire drill.

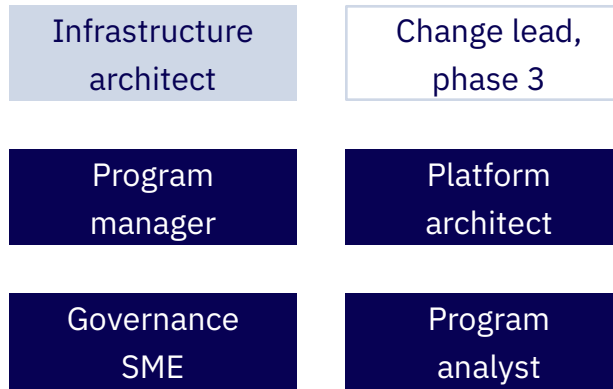
VP FINANCE · REGIONAL HEALTH PARTNER

Delivery pods scale up and down by workstream

LEADERSHIP PAIRS



WORKSTREAM LEADS



DELIVERY PODS



Northfield leadership roles

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■ West Monroe full time

□ West Monroe part time

■ Northfield part time



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Jordan Ellis • Managing Director

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VP FINANCE • REGIONAL HEALTH PARTNER

West Monroe at a glance

We are tech-forward advisors who draw on decades of industry experience to create value quickly and execute with precision.

WHAT WE DO

- Artificial intelligence
- Operations and technology
- Organization and people

WHERE WE DO IT

- Healthcare and life sciences
- Financial services and insurance
- Energy, industrial and high-tech

“

A team that excels at every phase of complex work.

CHIEF DATA OFFICER, REGIONAL
PAYER

People are at the core of everything we do

99%

of clients consider us a
strategic advisor

71%

of employees say their
work has real impact

10K+

pro-bono and volunteer
hours last year

Source: Sample figures for layout only; replace with current firm data before use.

ABOUT US

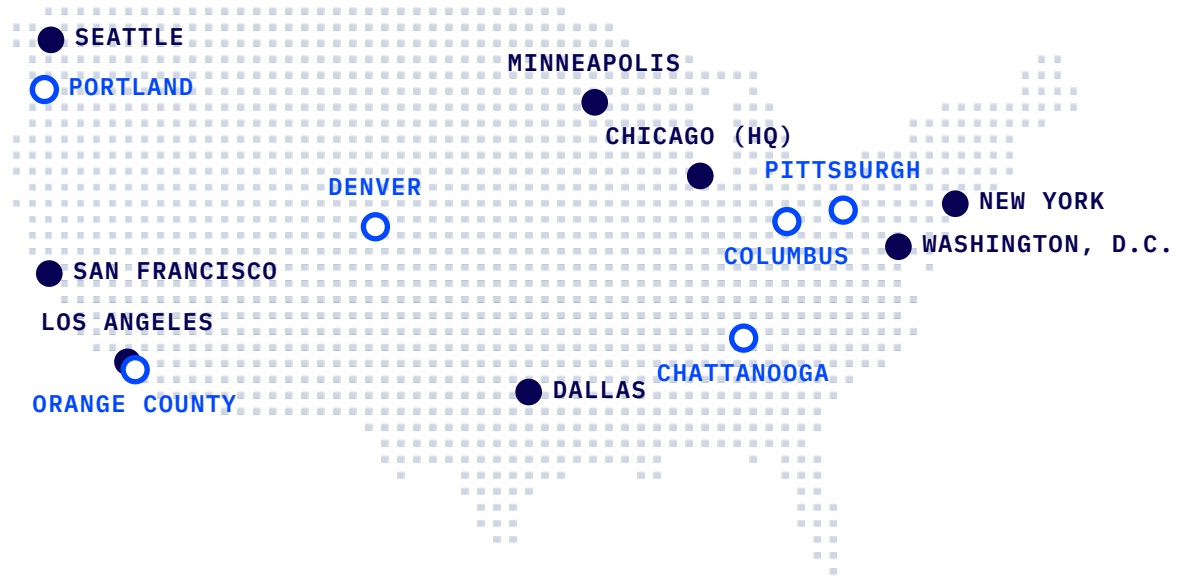
Eight offices and six talent hubs across the US, plus two abroad

INTERNATIONAL OFFICES

- London
- Costa Rica

TALENT HUBS

Teams of consultants who work from a metro area without a local office.



■ OFFICE

□ TALENT HUB

8

US offices

6

talent hubs

1,500+

people in all locations

Source: Sample figures for layout only; replace with current firm data before use.

ABOUT US

Ten offices and six talent hubs across three countries

- OFFICE
- TALENT HUB

10

offices in three countries

6

US talent hubs

1,500+

people in all locations

Talent hubs are consultant teams who work from a metro area without a local office.

Source: Sample figures for layout only; replace with current firm data before use.



Our data and cloud alliances multiply what we deliver

PARTNER LOGO

Alliance partner A

PREMIER PARTNER

86 52

joint engagements clients served

PARTNER LOGO

Alliance partner B

SELECT PARTNER

61 38

joint engagements clients served

PARTNER LOGO

Alliance partner C

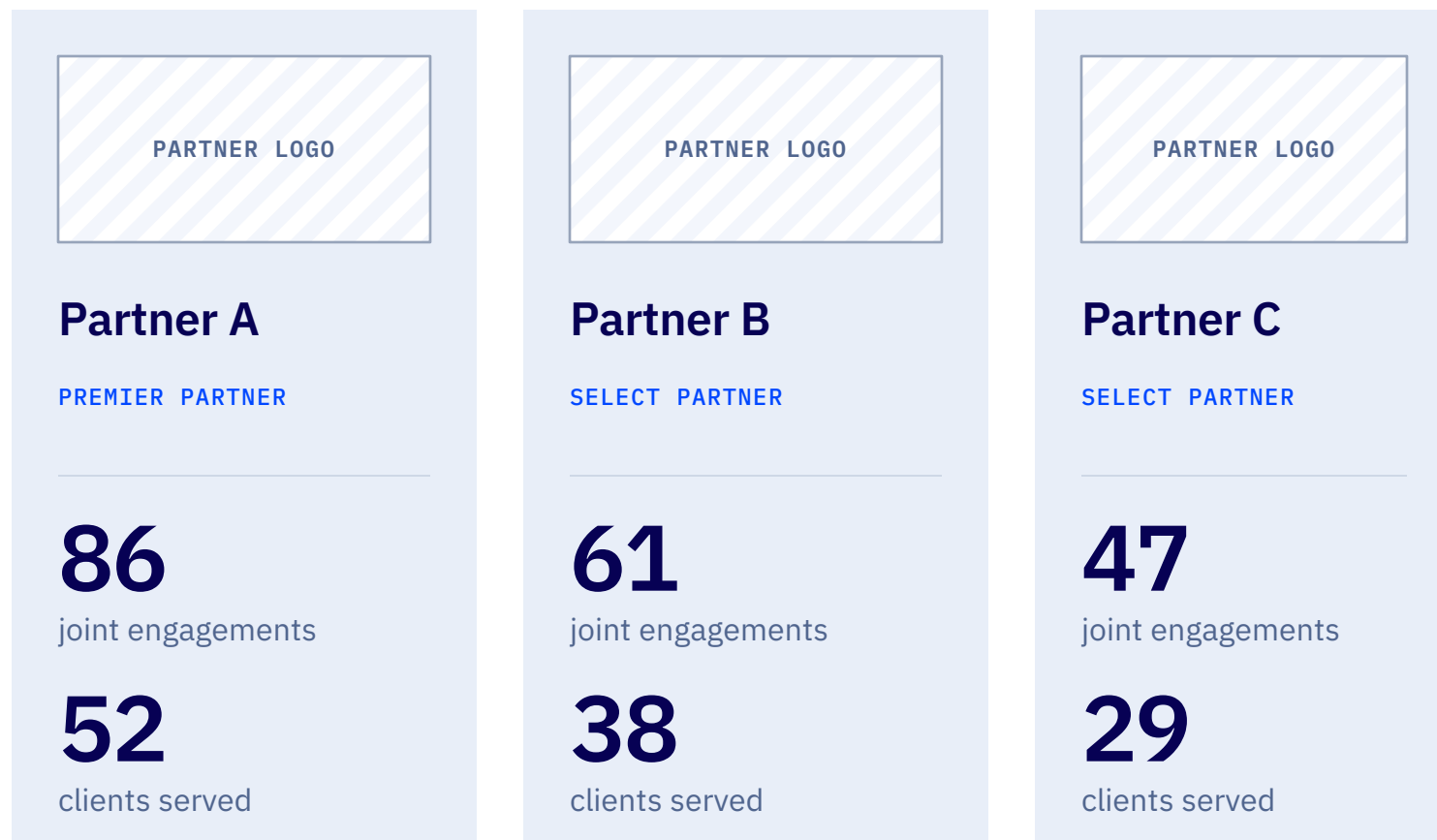
SELECT PARTNER

47 29

joint engagements clients served

Source: Sample figures for layout only; replace with current firm data before use.

Our alliances multiply what we deliver



Source: Sample figures for layout only; replace with current firm data before use.

WHY ALLIANCES MATTER

194

joint engagements with our top three alliance partners

Joint teams bring platform depth to our industry expertise, so clients get one plan and one delivery team.

Trusted by leaders across industries and technology

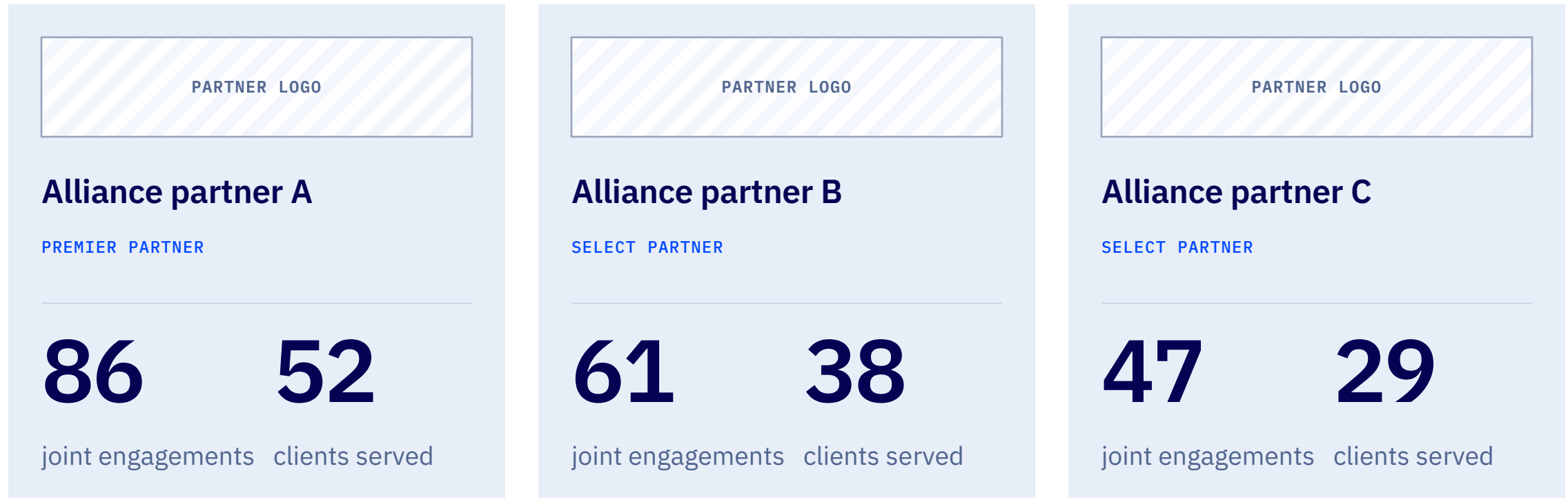
CLIENTS WE SERVE



OUR PARTNER ECOSYSTEM



Our data and cloud alliances multiply what we deliver



WHY IT MATTERS

Joint teams bring platform depth and delivery discipline, so clients adopt faster.

Source: Sample figures for layout only; replace with current firm data before use.

A 15-day close became a 6-day close



“

CHALLENGE

A 15-day close, with intercompany balances matched by email.

APPROACH

Automated reconciliations, rules at posting, review on day one.

6 days

to close, from 15

\$2.1M

annual savings

92%

of matches automated

We stopped finding problems on day nine.

CORPORATE CONTROLLER • REGIONAL HEALTH SYSTEM

Source: West Monroe engagement results, confirmed by the client.

Close in 6 days: rules, reconciliations and a daily cockpit

6 days

to close, from 15

\$2.1M

annual savings

92%

of matches automated

WHAT WE DID

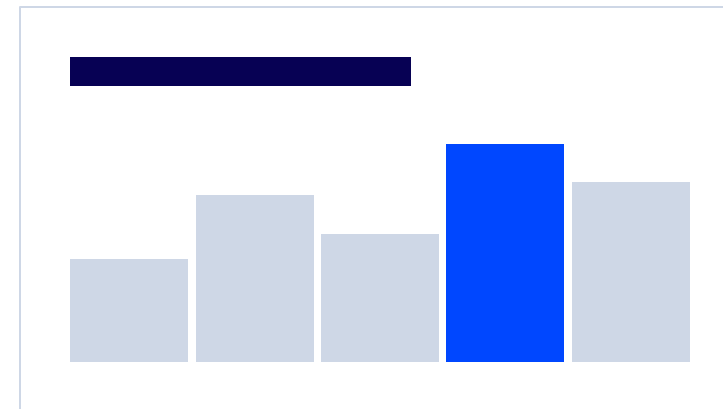
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- **Set rules** at posting.
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We stopped finding problems on day nine.

CORPORATE CONTROLLER



Close cockpit, week 12

DELIVERABLE

A daily view of every entity's close status, built with the finance team.

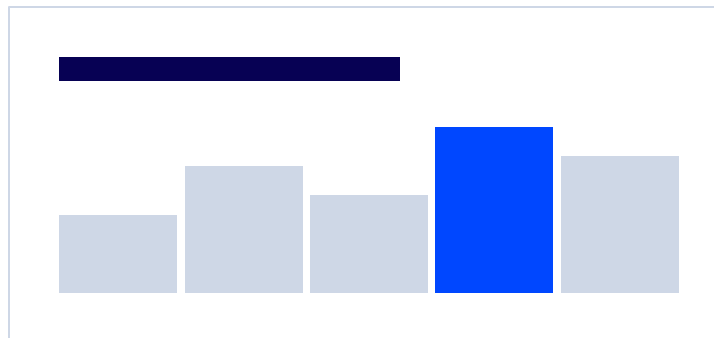
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Close in 6 days: rules, reconciliations and a daily cockpit

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Close cockpit, week 12

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CORPORATE CONTROLLER

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RESULT

6 days

to close, from 15

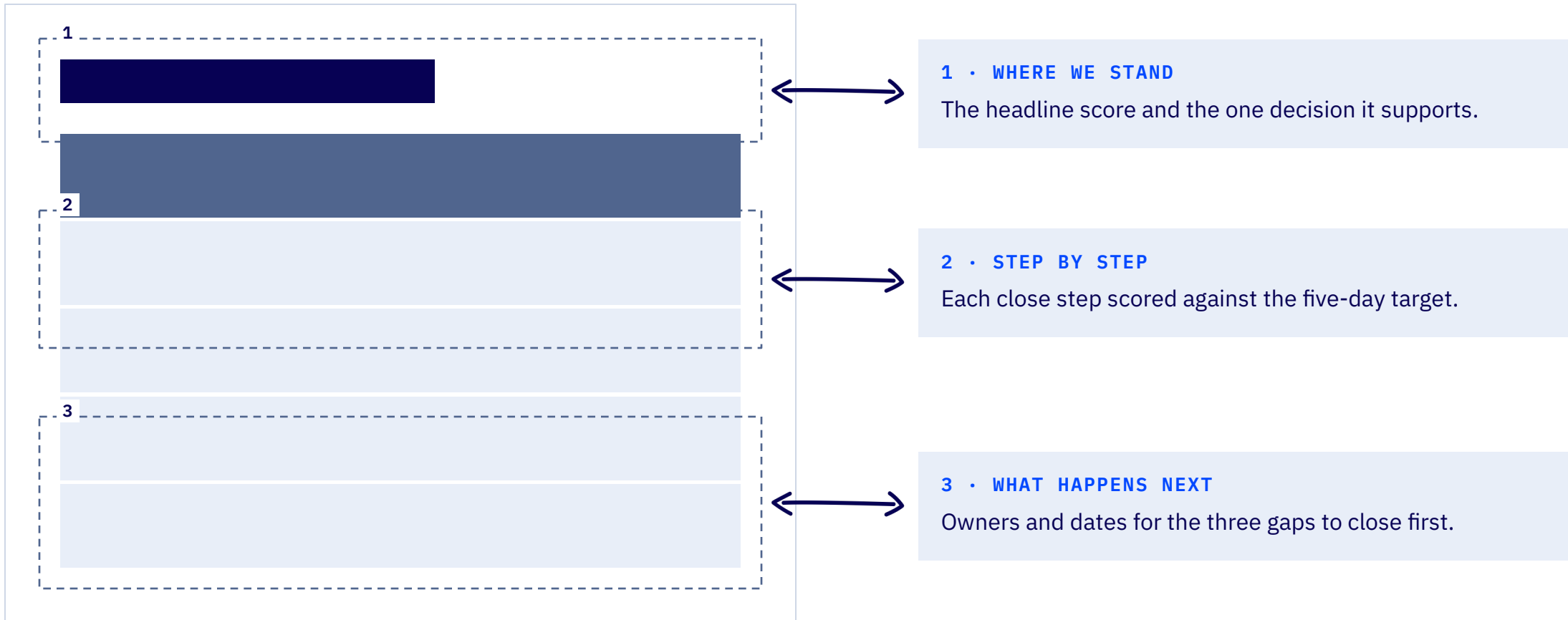
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Each zone of the readiness scorecard has a job



Source: Illustrative page. Final outputs are tailored to Northfield's data and confirmed priorities.

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